

Inventurus Knowl (IKS)

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Call: Feb 2025

February 12 , 2025

BSE Limited
The Listing Department
Phiroze Jeejeebhoy Towers
25th Floor, Dalal Street
Fort, Mumbai 400 001
Maharashtra, India

BSE Scrip Code: 544309
National Stock Exchange of India Limited
The Listing Department
Exchange Plaza, Plot No. C/1, G Block,
Bandra Kurla Complex
Bandra (East), Mumbai 400051
Maharashtra, India

NSE Symbol: IKS
Dear Sir/Ma'am,

Sub: Transcript of the earnings conference call for the quarter and nine -month period ended December 31, 2024

Pursuant to Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, please find enclosed the transcript of the earnings conference call held on Thursday , February 6, 2025 for the quarter and nine -month period ended December 31, 2024 .

The said transcript has also been uploaded on the Company's website and can be accessed through the following link:

<https://ikshealth.com/investor -relations/>

This is for your information and records.
Thanking you.

Yours sincerely,
Inventurus Knowledge Solutions Limited

Sameer Chavan
Company Secretary and Compliance Officer
Membership No. F7211

Encl: As above

"IKS Health Q3 FY '25 Earnings Conference Call "
February 06, 2025
MANAGEMENT : M R . S ACHIN G UPTA – F OUNDER AND C HIEF E XECUTIVE
O FFICER
M S . N ITHYA B ALASUBRAMANIAN – C HIEF F INANCIAL O FFICER
M R . S ARANSH M UNDRA – A SSISTANT V ICE P RESIDENT ,
I NVESTOR R ELATIONS
M ODERATOR : M S . R UCHI M UKHIJA – ICICI S ECURITIES
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Moderator: Ladies and gentlemen, good morning, and welcome to the IKS Health Q3 FY '25 Earnings Conference Call, hosted by ICICI Securities.

As a reminder , all participant lines will remain in the listen-only mode. And there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal the operator by pressing "*", then "0" on your touch tone telephone. Please note that this conference is being recorded.

I now hand the conference over to Ms. Ruchi Mukhija from ICICI Securities. Thank You and over to you.

Ruchi Mukhija: Thank you, Ryan. Good morning, ladies and gentlemen. Thank you for joining us today for the Q3 FY '25 earnings call of IKS Health. On behalf of ICICI Securities, I would like to thank the management of IKS Health for giving us this opportunity to host this earnings call.

Today, we have with us Mr. Sachin Gupta – Founder and CEO; Ms. Nithya Balasubramanian – Chief Financial Officer; Mr. Saransh Mundra – A VP (Investor Relations).

I will hand over to Saransh for the Safe Harbor statement and to take the proceedings forward.

Thank you and over to you, Saransh.

Saransh Mundra: Thank you, Ruchi. Good morning, everyone. Welcome to our First Ever Earnings Call for the quarter ended 31st December 2024. I am Saransh Mundra, I head Investor Relations at IKS.

We hope you have had an opportunity to review the "Earnings Release and the Presentations" that we have issued last night.

Before I hand over to Sachin and Nithya for their views on the result, let me just begin with the Safe Harbor statement.

As part of our remarks and during Q&A, we make certain statements which are forward-looking and may involve significant uncertainties. IKS does not take any responsibility to update such forward-looking statements and your discretion is warranted while making any investment decisions. Over to you, Sachin.

Sachin Gupta: Hi, good morning and good evening, everyone, depending on where you are. Thank you so much for joining our first earnings call as a public company for the quarter three fiscal year 2025.

Today, I wanted to start off with a few remarks and a little recap about our business model, because this is our first call and perhaps the last time I spoke with many of you was on the road show. So, we will start with a quick recap of the business model, then I will remark a little bit about the key themes that the business is strategically executing on. And then I will dive into some key themes that we are specifically focused on in Fiscal '25. And from there, I will dovetail into a high-level summary of our Q3 Fiscal '25 Results, after which I will turn it over to Nithya to talk through the details of the financials for you. And then we can dive into Q&A. So, as you all would recall, IKS is a Care Enablement Platform that operates in the physician segment of the US healthcare market. That physician segment consists of about 900,000-odd physicians driving \$1.5 trillion of revenue in what is a nearly \$5 trillion US healthcare market. That physician segment of the US healthcare market we think is the fulcrum and the most important point of care delivery within the continuum of care, simply because as the US healthcare system tries to achieve sustainability by solving for the cost, quality and access challenges that they are struggling with,

we believe the only way to really do that is through the transformation of care delivery from a reactive hospital-centric care delivery system to a

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proactive physician-led and patient-centric care delivery system, in which physicians reassume their role as being quarterbacks of care truly configuring care attached to patients' value system.

And so, given that this physician market is becoming the new fulcrum of care delivery, what we saw when we started our business and started building this Care Enablement Platform is that the physician market would rapidly consolidate as people would realize that it is the new fulcrum of care delivery. And along with that consolidation would come a maturity that would lead them to actually rediscover what we call their core versus chores equation, and start to really focus on the core tasks around patient care and patient experience. And identify all of these chores that are taking them away from the core task of patient care. And then start to delegate or outsource these chores to some entity or platform that could do these chores better, cheaper, faster at scale.

Remember, the most important entity in care delivery, the physician, was traditionally spending as much as 50% of their time on non-patient care non value add, non-discretionary tasks. And through the deployment of our platform, not only are they able to direct a lot of that 50% of their time back to patient care which then improves cost of care and quality of care, but also then fundamentally changes their economics from a revenue and cost perspective. Just to put in perspective, physicians are spending nearly 15% of their revenue on these chore tasks, which is about \$225 billion. And the physician market is growing at about 8%.

And so we are talking about \$225 billion TAM growing at 8%, which is nearly the size of the IT services industry in India today, that \$225 billion. And what's interesting is that only \$30-odd billion of that \$225 billion has been outsourced to date to platforms like us. And that outsourced TAM now recognizing the value that this outsource can create is actually growing at 12%. So, while the industry is growing at 8%, the outsourced TAM within that \$225 billion,

which is at about \$30 billion, is really growing at about 12%.

And within that ecosystem what IKS has done is, has created a tech-led human in the loop Care Enablement Platform. And really the only platform in the world that truly delegates all of these chore tasks that have been discovered so far through the right combination of the proprietary technology that we have built and the global human capital that we have developed over these last 18-odd years.

And I say the only platform because given that these chores, there's about 16-odd chores that one has identified across the continuum of the physician setting, what we have discovered really is that a lot of companies have been built over the years that are focused on one, two, three, maybe five of these 16 tasks. But buyers are starting to discover in the physician enterprise world that they should not be in the business of buying individual point solutions and then trying to integrate them. Imagine buyers having to buy 10, 12, 14, 15 point solutions, then integrating them and holding the bag on consolidating and creating value from those point solutions.

And so I think buyers are discovering that the value of the whole is much greater than some of the individual parts. And as they discover that and buying

behavior shifts from sort of best in

class point solutions to the overall platform, I think, IKS continues to be perhaps the only company in the world that truly provides them that alternative.

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Over these last 18 years IKS has built a business model where 95% plus of our revenue is recurring in nature. We have very low client attrition, it's a sticky business model. Our top ten clients and our top five clients are all having average vintages of five plus years. We have grown to a workforce of about 13,150 employees as of December 31st, 2024. Of which nearly 500 employees are technologists that are building our proprietary technology, and nearly 2,500 employees of those 13,150 are clinically trained. And it's through the combination of our proprietary technology and our human capital that we are being able to delegate these tasks to them better, cheaper, faster and at scale.

Let me just comment quickly on the key themes by which we are executing on this business through which we intend to continue to retain and consolidate our leadership position as the Care Enablement Platform of choice in this space:

- The first theme really is to continue to further enhance our journey from where we were human-led and tech in the loop for certain of these chores, to now becoming more and more tech-led and human in the loop. That affects both the scalability of our tasks as well as obviously the pricing and the profitability at which we are able to deliver these.

- Second, the idea, because the buying behavior is still in transition from point solution, best in class buying to now starting to buy more and more of the platform manifest, one of our strategic execution predicaments continues to be that we need to be number one, two or three in each of the features, each of the 16 features in the platform, even as we are the only company in the world that has the full breadth of the platform.

- And number three, obviously, continue to proliferate the full breadth of the platform into our captive customer base. Which as you imagine, as you know now, our captive customer base went from about 50-odd large enterprise scale customers, which has traditionally been our area of focus at IKS. Because we believe in a market that's consolidating you want to work with the large consolidators versus the ones getting consolidated. So, traditionally we had 50-odd large enterprise scale customers. But through the acquisition of AQuity Inc., we expanded that customer base to north of 850-odd customers, of which really 500-odd tend to be that enterprise scale consolidator customers.

And so, across these 500-odd enterprise scale customers that we intend to really focus on going forward, there's nearly 150,000 physicians that are employed by them. Which, as you can imagine, is nearly 18% of the overall physician market in the US. And the full manifest of the platform being cross sold to that entire installed base of 150,000 physicians, it makes for a massive growth runway over the next couple of decades. And that's really the journey that we are on.

So, those are the three big execution themes at a strategic level. And then specifically from an FY '25 perspective, just to remind everybody the themes that we have been executing on is:

- First, effectively integrate the AQuity acquisition into making IKS one integrated platform. As a part of that, one of our key themes is that, because our traditional focus has been and will continue to be on large enterprise scale customers that are consolidators, with the additional AQu

ity we have grown to about 850-plus
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customers together. And we will actually through FY '25 and perhaps the early part of FY '26, continue to cut some of that tail and really consolidate into 500-odd large customers. And so, one of the big themes of FY '25 was to really start to cut that tail and bring the customer base to those large customers that we want to focus on.

● The second big theme that we have been executing on is, as you know, AQuity traditionally did not have too much proprietary technology, nor were they using the global human capital model that effectively. And so our second big execution theme for FY '25 really was to transform the AQuity operating model with the IKS operating model. And in that process, take our blended margins that had dropped from 24% pro forma after AQuity's consolidation into IKS back towards that early to mid-30s which we think is where the steady state business should settle down at. So, that was our second big theme for FY '25.

● And our third big theme was to really activate the most strategic aspect of the acquisition, which is the cross sell, where we will be able to cross sell the full breadth of our platform across these large 450-plus enterprise scale AQuity customers that have come to us through this acquisition.

So, those are the three big themes that we have been executing on through FY '25.

With that, I will now actually dive a little bit and provide a high level intro to our "Q3 FY '25 Results":

I am very happy to note that actually we have been able to execute better than we had imagined across all of the three themes for FY '25. As I had mentioned earlier, because we are cutting the tail for some of the smaller customers, as well as because we are transforming the AQuity delivery model by deploying the IKS technology and global human capital, that has a dampening effect on our revenue growth in FY '25.

● One, because of course we are reducing our customer base which as you see has gone from 850-plus to now about 750 customers by December 31st, 2024, that has a revenue dampening effect. And the second, because we are giving the traditional legacy AQuity customers incentives in terms of discounts to transform their delivery model leveraging the IKS technology, that has a little bit of a dampening effect on the revenue.

So, as we have been going around, we have been mentioning that FY '25 will be somewhat muted from a revenue growth perspective. Of course, over a period of time, we believe that our growth will continue to be well north of that 12% that the outsourced TAM is growing at. Happy to note that in Fiscal '25 Q3, in spite of the dampening effect of those two strategic endeavors, we have actually been able to grow the business 16% year-on-year from FY '24 Q3. And in constant currency terms, that's about 12% year-on-year growth. So, in spite of the dampening effect, that was very deliberate in our FY '25 revenues, we have been able to grow 16% year-on-year in revenue terms to a revenue of about Rs. 657.2 crores. Second, we have actually been able to accelerate our earnings significantly faster than we had imagined. As we were going around in our roadshows, we had mentioned that we believe we will get to the early to mid-30s consolidated EBITDA margins perhaps over the next 18 to 24 months. Very happy to report that we have actually clocked nearly a 31% EBITDA for the quarter end

ing December 31st, 2024, which is for us well ahead of the glide path that we had
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in mind to get to the sort of early to mid-30s, which is where we will stabilize. That reflects a 24% year-on-year growth in EBITDA margins and a 28% growth in PAT margins between Q3 Fiscal '24 and Q3 Fiscal '25.

Obviously, a lot of this has been achieved by actually instead of increasing headcount, decreasing headcount. So, we have actually taken headcount from Q3 Fiscal '24, which is about 13,250 people to 13,150 by Q3 Fiscal '25. Which means for a 16% year-on-year growth, we have actually reduced headcount by about 100 people instead of growing headcount. And this is in spite of actually increasing SG&A headcount, because we continue to invest significantly in sales and marketing to activate the cross sell motion as well as in R&D to continue to drive more and more transformation from human-led tech in the loop to tech-led human in the loop. So, that continued sort of non-linearity between revenue growth and people growth continues to be exhibited in the actual headcount. And then, of course, in the margins themselves that are exceeding our expectations.

And one other thing that I want to mention is that, because we were able to activate some of the cross sell faster than we had imagined, we are still being able to demonstrate faster than the

TAM growth of 12% to 16% year-on-year growth in revenue, in spite of the damping effect. So, we feel like we have been able to activate the cross sell relatively faster than we had imagined, which is leading to this revenue growth and keeping it relatively robust. That's a broad summary of sort of what our fiscal performance has been. Nithya will obviously dive into the details of some of this financial performance as we get along here. One thing that I will mention is, even as we are publishing year-on-year growth and quarter -on-quarter growth, it's really important to note that in our business year-on-year is really what is relevant, because there is significant seasonality that happens from one quarter to the other .

For example, Q3 and Q4 are traditionally weaker quarters than Q1 and Q2 because of the seasonality that sets in patient volumes due to the holiday season in Q3 and due to the severe winter weather in Q4. And so, I would encourage everybody to think of our business much more on year-on-year terms so that we are doing a little bit of a like-on-like comparison versus a Q-on-Q terms, even though we are publishing the Q-on-Q numbers for your benefits itself. So, that's some high level remarks on the financials.

Moving on, some of our revenue momentum is actually being driven by certainly some of the cross sell that we have been able to activate in customers like Louisiana Children's Medical Center and several others. But I think the revenue momentum is also being driven by the fact that we are actually being able to execute on our strategic thesis of driving more and more of the full platform penetration.

So, really happy to report that there have been three very significant customer wins. These are new customer wins over Q3. First, \$1 billion health system called Palomar Health, as you must have seen in the press release, have committed to the full platform manifest of IKS for their employed physician group in a 15 year path breaking deal for which they have committed to our platform. In this deal, Palomar will deploy the full breadth of IKS'

platform across our

feature clusters of revenue optimization, clinical support and value based care in their entire physician installed base. And from that deployment we are expecting some very significant upsides.

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The other thing that makes this deal unique is we have actually , based on our own modeling of what upsides we will create, we have actually guaranteed an upfronted a significant amount of those upsides, about \$16.5 million to Palomar in this process. And in a unique arrangement, the way this will happen is, as those upsides start accruing now that we have gone live with Palomar , up to the \$16.5 million will accrue directly to IKS. And then even beyond that \$16.5 million over the years, there's a significant gain share arrangement that IKS will have with Palomar which will continue to add non-linearity to our margins and yet create significant value for Palomar .

Similarly , we are very proud to announce a deal with Radiology Partners, which is the largest consolidation of radiology physicians across the U.S. They are 10x the size of any other consolidated entity in the radiology market in the U.S., nearly \$3.5 billion in revenue and 3,000-plus radiologists. Partnering with them, we are creating a very unique virtual radiology assistant model that through a combination of technology and global human capital has the potential to improve radiologists' productivity on the same fixed cost by as much as 25% to 30%. So, imagine the impact of a fully manifested VRA model on a 3,000 radiologists group like Radiology Partners with \$3.5 billion in revenue, the numbers start to get very, very significant at north of \$600 million, \$700 million a year in value creation.

And the way we have created this model with Radiology Partners is, once the model is manifested at a significant scale, this will turn into a JV where Radiology Partners and IKS will together then take this unique radiology enablement platform to the market, to the entire radiology market, based on the credibility and the learnings and the IP that we would have developed through that relationship with Radiology Partners.

Last but not the least, I would like to comment a little bit on Western Washington Medical Group, which is one of the largest independent medical groups in the Pacific Northwest. While the initial relationship that we have struck with them is on the revenue cycle side, that relationship is rapidly maturing into other manifests of the IKS platform. So, strong deal momentum, both on new customer acquisition, but faster -than-anticipated momentum on the AQuity legacy installed base cross-sell.

With that, I would like to just turn attention to a couple of product-related exciting launches as well. Super excited to announce the launch of Scribble Now , which is our fully autonomous GenAI and NLP-enabled offering for clinical documentation, ambient clinical documentation. With Scribble Now , today , IKS is now the most comprehensive suite of clinical documentation offering to the U.S. healthcare industry . By that I mean that depending on different care

delivery settings and different types of specialties, there is a need for different type of clinical documentation solutions in U.S. healthcare.

All the way from settings like radiology and pathology, where there is no conversation between the physician and the patient, there you still need technology-enabled transcription-type solution, which we enable through our Scribble Transcribe product; to live virtual scribing for which we have our Scribble Live product; to Scribble Pro which is really important for

complex encounters that often primary care-type physicians have, where there are multiple paths of diagnosis, there we have our GenAI-enabled and clinician-supervised Scribble Pro product. And then we also have our Scribble Swift product, where the GenAI is supervised by a human to eliminate any hallucinations.

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So, through this breadth of clinical documentation offering, which I think is unique in the world, we are now able to meet the various physicians and their settings, where they are in their journey of clinical documentation needs. And I think this should turn out to be a very significant competitive advantage for us over a period of time.

And then, given that we are living in this world of so much hype and attention to GenAI, I thought I will spend just a couple of moments talking at a high level about our AI strategy. So, our approach to automation enabled by technology really began sort of in 2017-'18, first with RPA, non-cognitive RPA, and then with cognitive RPA, as you must have seen in the presentation that was shared. And not to oversimplify, but if you really think about RPA, what it does is it's a technology that follows specific instructions and commands through some rule-based automation.

Over the last couple of years we graduated that use of cognitive RPA to really intelligent GenAI-embedded automation. We have seven or eight use cases, just like Scribble Now, across the 16 features of IKS, where we are actually embedding GenAI in those features. And as you can imagine, the value from GenAI really is that it helps create complex content based on prompt engineering that really leads to significant enhancement in productivity and efficiency. So, one way to think about it is, while cognitive RPA maybe leads to 5% to 15% automation, GenAI could lead to 20% to 35% automation; and in some cases, like clinical documentation, even 70% to 85% automation. And really as we are now starting to see the benefits of GenAI, we are excited to launch our journey towards Agentic AI, where the actual Agentic AI agents understand the goal of the task and are autonomously able to determine and execute the next steps to achieve those tasks.

So, we are super excited to sort of embark on this journey, empowered by the success of the automation that we have seen that is reflected both in our scalability and our continued margin improvement. In fact, happy to note that based on all the success we have seen, we are actually launching a GenAI center of excellence in the U.S. That will be working across all of the use cases that we have developed over these years and that have the potential to not only leverage GenAI, but now move towards the Agentic AI world.

So, with that, I will quickly summarize our performance for Q3 again. Solid execution across all our macro business themes, as well as the specific themes for FY '25. Ahead of glide path execution on margins, relatively rapid activation of the cross-sell into the AQuity installed base. And appropriate optimization of the customer base so that we can really focus on growing the customers that will create value over a period of time.

With that, I will turn it over to you, Nithya, to dive into some details of the financials.

Nithya Balasubramanian: Thank you, Sachin. Good morning and good afternoon to everyone on the call. This is obviously a very special call, not just because this is our First Earnings Call, but also because not too long ago I used to be on the other side of the call.

So, diving into the numbers, if you're following the slide deck I am on Slide #10:

Our revenues, we reported Rs. 657.2 crores in Q3 FY '25, which was a very, very healthy 16% year-on-year growth. Looking at

EBITDA, again, we are very happy to report significant margin expansion. Some of you will remember that in FY '24 our pro forma EBITDA, which is 12 months of AQuity and IKS combined, came in at 24%. And from there, we have been able to achieve a significant 650 bps expansion to 30.5% or 31% almost EBITDA in Q3 FY

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'25. We continue to believe that as we continue to transform AQuity's delivery model, these numbers will keep inching up in the future as well. So, the growth in EBITDA came in at 24% year-on-year.

Looking at profit after tax – again a very healthy 20% is what we were able to report in Q3 FY '25, which is again a significant expansion even if you look at quarter -on-quarter , we have been able to expand these numbers by more than 200 bps. And you will note that PAT growth actually came in at 28%. Sachin mentioned this before, the growth in PAT is actually higher than the growth in EBITDA, because our interest expense has come down, which has been enabled by the strong cash generation and therefore our ability to pay down the debt that we had assumed when we acquired AQuity .

Looking at our adjusted PAT, where we are adjusting out amortization of intangible assets which we had recognized during the acquisition, which, as you appreciate, is a non-cash expense. Looking at adjusted PAT, it came in at 22% in Q3 FY '25. And we saw a growth in adjusted P AT of 31% year -on-year .

Moving on to other operating metrics:

Again, EPS came in at a very healthy , almost Rs. 8 per share in Q3 FY '25. And ROE continues to be on a healthy expansion trend. Of course, prior to the acquisition, our ROE metrics in FY '23, if you look at the full year, we were at 36%. Our current numbers continue to see a healthy expansion as we are able to achieve the AQuity transformation. So, ROE in the quarter was at 33.2%.

I did mention healthy cash generation before, as you can see on the slide deck in slide number 12, you will see that our operating cash flow came in at Rs. 154 crores, our free cash flow at Rs. 109.5 crores. And looking at net debt, had it been business as usual, our net debt position would have actually landed at Rs. 503 crores. I will just take a moment to explain these numbers.

So, if you remember , in FY '24 our net debt position was Rs.850 crores. Through very healthy cash generation, we were able to pare it down to Rs.553 crores by the first half. And business as usual, this cash generation that we had done would have brought down our net debt position to Rs. 503 crores.

There are two exceptional items in the quarter . One of which is almost Rs. 25 crores IPO-related expense, which was advanced by the company on behalf of the selling shareholders. And this has already been recovered from them in the January month. So, this is a one-of f non-recurring item. And the other item is the Rs. 139 crores, or the \$16.5 million, that we have advanced to Palomar , which is an upfront guarantee of the net economic value add we will be able to deliver to them through our platform. This again, Sachin explained in detail during his remarks.

So, our net debt position, had it been business as usual, would have been Rs.503 crores. And what you will see going forward is this number continue to inch down as we continue to generate cash in the business.

Looking at our Summary Financials:

I will just call out some additional items which I have not covered on the previous slides. So, revenue came in at Rs. 657 crores. Our other income, excluding interest income, which is non-operating, the rest is all operating other income, came in at an additional Rs. 20 crores.

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Our adjusted EBITDA, where I am adjusting ESOP cost, adjusted EBITDA came in at Rs. 207 crores, which translates into a margin of about 31.5%. Including ESOP cost, our EBITDA was Rs. 201 crores and EBITDA margin was at 30.5%.

Our finance cost, I already mentioned that we have been already able to pare down our debt fairly significantly , which is why you see that number is coming down and it will continue to come down in the future as well. Our finance cost was about Rs. 20 crores in the quarter . Our tax expense was about Rs. 30 crores. Our ETR in the quarter was about 19%, and therefore the profit for the period, or PAT, came in at Rs. 130 crores, or a PAT margin of about 20%.

Adjusted profit, adjusting for amortization, came in at Rs. 145 crores and a margin of 22%.

With that, I will conclude my remarks. We are, of course, very, very excited by the strong and healthy trends that we are seeing across the board. Thank you for your attention, and we will now open the floor for questions.

Moderator: Thank you. Ladies and gentlemen, we will now begin the question-and-answer session. The first question comes from the line of Seema Nayak from ICICI Securities. Please go ahead.

Seema Nayak: Hello. Yes, congratulations on the first quarter after listing, and thanks for taking my question.

My first question is, out of your service offerings such as RCM, coding, Scribble, etc., which one is getting the maximum traction? And what is the revenue breakup of service offerings?

And my second question is regarding AQuity , what kind of cross-selling traction are we seeing with the newly acquired large set of clients?

Sachin Gupta: Thank you for the question, Seema. I appreciate it. So, as it relates to your question around which of the 16 features are seeing the most traction, I am happy to note that actually we have sort of stratified these 16 features into three feature clusters; revenue optimization, clinical support, and value-based care, based on the type of value they create for the customer . And at

this point I can tell you that we are seeing a pretty secular growth trend across those features. Also, given that our go-to-market now is really not feature-wise but is really much more platform, I had mentioned earlier that our go-to-market now has pivoted from you can buy certain features to either you buy the clinical bundle or you buy the administrative bundle. And the clinical bundle consists of about nine of the 16 features, and the administrative bundle consists of about seven of those 16 features.

And so, given that we are going to market in that manner where either the customer buys the entire clinical bundle and then they can pick and choose from the administrative bundle, or they buy the entire administrative bundle and they can pick and choose from the clinical bundle, we do not really report numbers based on individual features. But I am happy to note that the growth is secular across the three feature clusters or the two bundles, depending on how you look at it.

Your second question was really around the AQuity cross-sell. Look, like I would mentioned earlier, we actually had anticipated that the cross-sell will take a little longer to actually put into motion, because we had to create an overlay consultative sales engine that would take some of the more transactional point solution-based relationships that AQuity traditional

ly had

and elevate them to a platform sale. So, we actually created that overlay cross-sell engine in the first six months of the fiscal. But happy to note that in Q3 itself we were able to start to see traction in the cross-sell motion. There are several deals that we were able to consummate in Q3. The one that I am able to publicly announce is the Louisiana Children's Medical Center ,

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which is a very significant health system in the New Orleans area. But there are several other marquee systems that are already starting to consume other manifests of the IKS platform beyond what they traditionally had in the AQuity relationship.

So, that's one of the reasons why we have been able to deliver a 16% year-on-year growth for Q3 in spite of the muted effects of cutting the AQuity customer tail as well as offering customer discounts and incentives in order to allow us to transform their operating model that is now then showing up on the bottom line.

Seema Nayak: Thank you, and all the best.

Moderator: Thank you. The next question comes from the line of Abhishek Kumar from JM Financial.

Please go ahead.

Abhishek Kumar: Hi. Good morning, Sachin. Always good to hear from you. A couple of questions from my side. First, as we look at 12% dollar revenue growth year-over-year this quarter, can we break this down between the growth in the heritage IKS business and in the AQuity? The reason I am asking is, as you pointed out, there is some dampening impact of tail cutting and offshoring. So, just wanted to appreciate the strength in the heritage IKS business from a year-over-year perspective.

Sachin Gupta: Great question, Abhishek. Thank you. And good to hear from you. So, look, obviously, because the dampening effect of the revenue is largely in the AQuity installed base, obviously the IKS legacy business and installed base is actually growing significantly faster than the 12%. While we do not want to give specific numbers because we really are now operating like one company and one platform, I will tell you that the growth in the legacy IKS business has been robust. Also, there's been some tremendous new logo addition, like I called out in deals like Palomar and Western Washington and Radiology Partners, which are all reflecting in the IKS numbers.

And so yes, the IKS business is back to its historical growth rates, which you have evidenced FY '22 to '24. The legacy AQuity business obviously is going through a dampening effect. But again, as the AQuity cross-sell motion starts to truly get activated and kick in, we hope to be able to overcome that. And then obviously the blended growth rates will continue to be far superior than the 12% that the industry is going at.

Abhishek Kumar: Okay. Maybe a related question. When I look at the standalone numbers, on a Y-o-Y basis they seem to have kind of remained flat or a slight decline actually. So, I was wondering if that standalone number is a reflection of heritage IKS business or there are some subsidiaries which are excluded and therefore my reading is wrong.

Nithya Balasubramanian: Sachin, I can take that. So, the standalone numbers are not reflective of legacy IKS business. We have other subsidiaries in the U.S. where the rest of the revenues have been booked. So, I think you should refer to Sachin's comments on the growth related to the legacy IKS business.

Abhishek Kumar: Okay. One last question from my side. Palomar deal, we have mentioned that we have paid upfront guarantee of Rs. 139 crores and Sachin, you indicated that it's a 15-year deal. So, first, I wanted to just understand the structure and does this Rs. 139 crores, or \$16.5 million, represent the saving that we will do for them over the 15 years that the tenure of this deal is for. Any c

olor on how we structure and then how overall what kind of ROI, etc., we make on this upfront investment? Thank you.

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Sachin Gupta: Great question, Abhishek. Thank you. So, the way the Palomar deal is structured is like I was saying it's a 15-year no out for convenience deal and what's most unique about it is that they are committed to deploying the full 16-feature manifest of the IKS platform from the get-go. At the deployment of this full 16-feature platform, we actually anticipate that the value created, the net economic value that we will create for Palomar at the confluence of the costs that we will save for them and the revenue upsides that we will generate for them will be many multiples of that \$16.5 million.

I cannot reveal exactly how much, but it will be many multiples of the \$16.5 million and so hence we were comfortable advancing that \$16.5 million to them, and the way the deal is structured is the first \$16.5 million of the net economic value add that will be created will come to IKS. After that, up to a certain threshold, the net economic value add is shared 50-50 between IKS and Palomar, and then another certain threshold, a larger amount of the benefit accrues to Palomar.

So, a very unique deal structure. Very exciting for us, because traditionally our deals, even though our customer stickiness is longer, tend to be four to five year-duration, this is 15 years, no out for convenience, full manifest to the platform. And based on our estimates of the value that will be created which is many multiples of the \$16.5 million over the 15 years, not only will we expect to claw back the \$16.5 million relatively rapidly, but then start to add significant non-linearity in our margins beyond the traditional gross margins that we expect from our platform.

Abhishek Kumar: Great. Thank you, and all the best.

Moderator: Thank you. The next question comes from the line of Srivathsan from Aventus Spark. Please go ahead.

Srivathsan Ramachandran: Yes. Hi, just two quick questions. One, does the last year sale numbers factor in the full three quarters of AQuity? That's one. Second, just wanted to get your sense on this Palomar deal, is this a first deal of this nature and is there any existing revenue from this client that's sitting in the financials already? Thank you.

Sachin Gupta: So, this is absolutely our first deal with this customer and that makes it even more exciting that now large customers are trusting us with the full manifest of the platform in such a long-term deal construct. So, there is no other existing revenue from Palomar prior to this deal, which makes it even more exciting. As to your earlier question about the nine months, I am sorry, I did not quite catch the question. Maybe Nithya, if you did, you can answer it or if you can please repeat your question.

Nithya Balasubramanian: Yes, I did. So, to your question, in Q3 FY '24, there is two months of AQuity baked in.

Srivathsan Ramachandran: Okay, thank you.

Moderator: Thank you. The next question comes from the line of Gaurav, an investor. Please go ahead.

Gaurav: Yes. Thanks to the IKS team for this and congratulations on the phenomenal results. One of my questions is, as we talk about integration and amalgamation of AQuity into the IKS structure, are we also looking at any other similar acquisitions now or in the near future?

Sachin Gupta: Thank you for the question, Gaurav. Look, I think the AQuity acquisition, as you know, was the first significant acquisition

on that IKS did after nearly 16 years of existence. And we did this through a combination of the cash we had on the balance sheet and a very conservative amount of debt as a multiple of our EBITDA. Through this, as you know, we have created a massive

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customer base that we are actually trying to right size so that we could really focus on those 500-odd enterprise scale customers. And I think, as I mentioned before, those 500-odd enterprise scale customers employ 150,000-odd physicians, which is 18% of the entire U.S. physician market.

If we were to be able to successfully cross-sell our platform across those 500-odd customers, we are talking about a 90x growth opportunity from our current scale. So, our preferred growth path from here is really going to be organic in nature, given the massive customer base that we develop. Our inorganic strategy will be much more focused on perhaps bleeding-edge technology-type acquisitions, where we will be able to bring the power of our massive customer base and the access to data and context that we have, by which we can mature that technology rapidly. So, the inorganic strategy will be much more focused on tuck-in tech acquisitions.

As a gene pool, we are not a company that relies on acquisitions for growth. In fact, even before we did AQuity, as you know, we were growing at nearly 30% year-on-year CAGR. So,

AQuity was a unique strategic merger, really, that we embarked on, and I think we have a lot of good work to do ahead of us to capitalize on the benefits of that. Inorganic activities will be much more focused on technology to accelerate our journey from human-led tech-in-the-loop to tech-led human-in-the-loop.

Gaurav: Got it. Thank you, Sachin.

Moderator: Thank you. The next question comes from the line of Sagar Dhawan from ValueQuest. Please go ahead.

Sagar Dhawan: Yes. Thank you for the opportunity, and congratulations on a good set of numbers. Just a question on the Slide 14 that we have in the presentation, revenue from top 10 customers, which is there Rs. 272 crores. Does this include any customer which has gotten consolidated in this top 10 into the acquisition of AQuity, or these are only the top 10 customers of the IKS ex of AQuity?

Nithya Balasubramanian: If you are referring to the Q3 FY '25 number, this includes top 10 customers of the consolidated entity. But if you are comparing it to Q3 FY '24, there was only two months of AQuity, as I mentioned before.

Sagar Dhawan: Right. So, could you please provide the revenue from the top 10 customers on a Y-o-Y basis, ex of AQuity, just from the top 10 customers of IKS, ex of AQuity?

Nithya Balasubramanian: We can take this offline. Somebody from our team will be in touch.

Sagar Dhawan: Sure. And there's just another question on the cross-sell opportunity. Just wanted to understand whether, basically what kind of revenue per client potential do we see in the AQuity's customer base for the cross-sell, if you can just provide some guidance on that.

Sachin Gupta: So, that's a good question. Look, I think one way to think about that is that our opportunity set in a customer tends to be, if they are spending about 15% of their revenue on these tasks, at the full manifest of our platform, they will probably end up paying us, depending on their specialty mix, payer mix, something in the range of 10% to 12% of revenue, right? And so when you start to think about 18% of America's physician market existing in these 500-odd large enterprise scale customers that we will want to retain and cross-sell to, you can start to do the math.

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Typically, the 500-odd customers that we are really going to focus on, at the full manifest of IKS, should all be able to generate nearly \$100 million a year of revenue to IKS. And that's the type of customer base that we are retaining. And that's sort of the ACV potential at the full manifest of the platform. That's also why the Palomar deal is so exciting for us, because it starts to set a paradigm for customers embarking on journeys that apply the full manifest to the platform. So, that's sort of a high-level way of thinking about what the ACV potential will be is that we are really focused on retaining those 500 customers across the legacy IKS and legacy AQuity customer base that at full manifest could be \$100 million ACV customers.

Sagar Dhawan: Understood. Thank you and all the best. That's it from my side.

Moderator: Thank you. The next question comes from the line of Nilesh Jain from Astute Investment Management. Please go ahead.

Nilesh Jain: Hi, congratulations on a great set of numbers. Firstly, I had a question on AQuity. At the time of acquisition, when we had acquired, it was a 9% margin. Where are we now in terms of journey to presently, at what margins will be AQuity operating? That's my first question. And maybe I can take other questions.

Sachin Gupta: So, thank you for the kind remarks. I will say that, like I was saying earlier, we are now operating like one platform. And so, we really would not like to reveal individual margin profiles of AQuity and IKS, but it'll suffice to say that there's obviously been significant margin transformation in the legacy AQuity business. And our estimate of being able to get to sort of the mid-30s on a blended basis was based on that journey, except that we have been able to accelerate that journey significantly faster than we had thought. And so, we have gotten to nearly a 31% EBITDA margin and a 32-odd percent adjusted EBITDA margin in this quarter itself. But I think you can safely assume that the AQuity margins are well on their way to transformation and there is still a significant runway ahead of us.

One of the math that I would share with many people in the roadshows was that, for every human that we are able to transform or replace through a combination of our technology and some supervision through a global human capital, we are able to create about, give or take, \$20,000 a year of additional gross margin through every human that we replace. And when we started out, there was nearly 2,500-odd humans in the U.S. So, depending on what we are able to take that down to, based on our customer agreements and customer comfort, that will eventually signal sort of the end of that runway for AQuity margin transformation. I will say that we are probably not even at the halfway mark yet.

Nilesh Jain: All right. All right. My second question is, you mentioned you have around 16 tasks. So, just wanted to understand, are there any other features or tasks where you are not present and which are not part of your offering. And what would be those and if you are looking to add

those as well?

Sachin Gupta: Absolutely . Great question. Thank you. So, look, I think at any given point of time, we have at least two or three new features or tasks that are in co-development with customers that obviously continue to then increase the TAM further than what

it is available today . And so

happy to note that we have at least two or three in co-development right now .

One such example, for example, would be several tasks within the value-based care feature cluster of our platform. That value-based care phenomenon in the U.S. is going through a major shift. Initially a lot of the focus was on risk and quality optimization, whereas now the

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focus is in total cost of care management. And so when you start to think of care coordination, transition of care management, risk stratification of populations, utilization management, at least the back of five of the utilization management, those would all be new features.

Also, there are some other features within the physician's workflow that are applicable both in fee-for-service and value that are under development. For example, one example for that would be order entry , both synchronous and asynchronous order entry . So, yes, constant development of new features. I would not be surprised if we would reveal three to four new features over the next 12 to 18 months.

Nilesh Jain: Good to hear. And then last question is, I mean, we have seen that there's been a lot of M&A activity recently by private equity on the RCM side. So, where do you see competition on your revenue optimizing side of offerings, which you have? How do you see them and how do you compete?

Sachin Gupta: It's a good question. Look, I think that's a very interesting market, and it's really changing rapidly . If you think about it, one way to think about it is, traditionally , there were sort of two genres of companies in the web cycle space, maybe three genres of companies. One, U.S.-based companies that were directly contracting with healthcare providers and leveraging a combination of their U.S.-based human capital, their U.S.-based technology , and subcontracting a bunch of offshore labor to deliver the RCM outcomes to customers. So, companies in that genre would be companies like R1 RCM, which was formerly called Accretive, or Ensemble Health or Conifer . Those would be that type of RCM companies. The second type of RCM companies were the ones that grew up and traditionally as Indian task vendors, those are the ones that I think have been a lot in the news in the Indian ecosystem. You heard of Access Healthcare and GeBBS and Omega. Their legacy was where they were subcontractors of tasks for U.S. RCM companies, but they did such a good job of executing and got to so much scale that now they are actually going upstream and trying to acquire customers directly in order to be able to deliver those efficiencies to the customers. Might that create some channel conflict? Maybe, but we have to give them credit for the type of scale they've driven.

And then the third genre perhaps is a company like us that was neither a U.S. company nor an Indian company , but deliberately created a global execution model with proprietary technology and always had direct customers in the U.S. where we took accountability for their outcomes. And so I think those would be the three genres of the companies. I feel like all those three genres today are converging into that globally enabled tech-driven RCM solution. Some of us perhaps started like that. Others are converging to that. So, it will be very interesting to see how the winners emerge in this space.

But again, remember , our competitive positioning is that RCM is just one part of the overall value chain of chores that physicians are straddled with. And so our competitive positioning really and our right to win often has been that not only are we a best-in-class RCM, but we are actually able to drive a lot more improvement in the physician productivity through our clinical support solutions. And

then also beyond the fee-for-service patient panels that are affected by RCM, we are actually able to help physicians with their value-based care patient panel with our VBC feature cluster .

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Nilesh Jain: Thank you for that detail. Just a last bookkeeping question. You generated around Rs. 500 crores to Rs. 600 crores of operating cash flows. By when do you think you will be paying off your entire debt or you will become debt free? That's it. Thank you.

Sachin Gupta: So, all things remaining equal, remember , we have operated for 16 years with no debt and actually significant amount of cash. So, all things remaining equal, we should be able to get debt free sometime next fiscal. But obviously , there might be other uses of capital, including

tuck-in tech acquisitions, more innovative arrangements with customers where we align the value that we create with their outcomes. And so depending on that, there might be some detours to that. But all things remaining equal and us not embarking on any such endeavors, there's no reason why we wouldn't be debt free next year .

Nilesh Jain: Thank you. And all the best.

Moderator: Thank you. The next question comes from the line of Siddharth Mishra from Fidelity International. Please go ahead.

Siddharth Mishra: Hi, Sachin and Nithya. Good to hear from you. I wanted to ask a couple of questions. One is, firstly , I appreciate the very long-term opportunity which is there and congrats on the progress which you have done in the AQuity acquisition. I just had a question on the deals which you won in the last quarter . What is the ramp timelines of these deals and when do they convert into revenues?

And secondly , Q4 will be, I think, the quarter where you will have a base where you have the full AQuity company in the base. So, do you expect to grow as strong as we have seen in this quarter , even in Q4, considering the ramp-up of the deals which you won in Q3?

And secondly , this is more of a philosophical question. So, you are at 750 customers as of end of December 2024 and you want to get to that 500 customers with AQuity . So, just wanted to understand, how aggressive will you be in cutting the AQuity clients versus having also a good overall growth, top line growth?

Will you kind of go in a pace which will still keep our overall growth good in the double-digit category? Or do you think you will first be very aggressive in cutting AQuity clients irrespective of what impact it might have on revenues? And then how will it impact your profitability growth as well? So, these are two questions.

Sachin Gupta: Great. Thank you. Thank you, Siddharth. So, look, first of all, just to sort of recap, our theme for FY '25 was to cut the lowest hanging fruit of the tail that we definitely do not want to play with going forward, which I think we have executed on for the large part, and really get the blended margins of the business to the place that they need to be in.

And so as you have seen, we have really been executing to that. And I am fairly confident that we will be able to continue to execute to those themes in Q4 and beyond. The one way to think about it is, even in spite of all of the rationalization of customers as well as transformation of margin-related discounts, we have delivered a 16% year-on-year growth. And this is before the cross-sell has gotten activated. And this is before, like you noted, some of the ramp effect of the IKS new customers come into play .

So, I have maintained through the roadshows, and I will still maintain that the opportunity that we

have created for ourselves is to execute far faster than the 12% TAM growth over the next 10, 15 years, not just over the next few quarters. So, I have no reason to believe going into Q4

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and well into the next several years, we have any reason to slow down that growth to any lower than much faster than the 12%. So, that's one.

Yes, we will continue to optimize margins. We have been a little lucky in that we have been able to accelerate that faster than what we had originally thought. And I think as long as we stay on that path, maybe we get to the steady-state margins towards the mid-30s instead of the next two years within a year. So, yes, I do believe that we will be able to deliver significantly faster than the 12% growth and continue to run towards that steady-state EBITDA margin profile of getting to the mid-30s over the next year .

As it relates to your specific question about when these deals will ramp, all of these three deals actually have gone live as we talk and they will ramp through Q4 of Fiscal '25 and Q1 of Fiscal '26. So, the full effect will be felt over the next two quarters and that will continue to reflect in even faster year -on-year growth than perhaps we have been able to witness in Q3.

Siddharth Mishra: Yes, thanks for that. So, just to clarify , the 16% year-on-year growth which is shown in this quarter is over a base which had only two months of AQuity , right? And in Q4, you are saying that even with the full impact of AQuity acquisition in the last year, you still expect to grow very strongly . Is that a fair understanding or --

Sachin Gupta: While I am not giving formal guidance, I think that will be safe to assume.

Siddharth Mishra: Okay . Got it. Thanks. Thanks a lot.

Moderator: Thank you. The next question comes from the line of Chirag Kachhadiya from Ashika Institutional Equities. Please go ahead.

Chirag Kachhadiya: Hi, am I audible?

Moderator: Yes, please go ahead.

Chirag Kachhadiya: Yes. So, I have one question. So, every year, what average price increase took place in for the 16 available features on platform for the existing legacy clients? Yes.

Sachin Gupta: So, good question. Thank you. Look, first of all, let's all recognize that we are operating in an

environment where it's significantly intensely competitive and customers are looking for more and more value from their partnerships. Having said that, I think a reflection of the strength of our platform has been that we have cost of living adjustment clauses in our contracts with most of our customers that are indexed to U.S. inflation.

And we are working hard towards not only retaining those clauses, but then also creating further non-linearity opportunity in our contracts through conservative, but unique value-sharing arrangements like we have done in the case of Palomar and several others. So, generally, we have COLA indexed to U.S. inflation and beyond that, we are coming up with value-sharing constructs that will give us further protection and enhancement of margins.

Chirag Kachhadiya: Okay. In addition to my question, so in the growth which we are anticipating north of 5%, that is industry growth, what is COLA led and net new growth will be there on incremental basis?

Yes.

Sachin Gupta: Yes, I think the bulk of the growth is actually incremental growth, I would just say that the bulk of the growth is actually incremental growth, because legacy AQuity really did not have much COLA in their contracts at all. And so I would say, the bulk of the future anticipated growth should be attributed to real growth in existing customer volume

s or new customer additions.

Chirag Kachhadiya: Okay, thank you.

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Moderator: Thank you. The next question comes from the line of Ruchi Mukhija from ICICI Securities.

Please go ahead.

Ruchi Mukhija: Hi. I have two questions. We have announced three large deal wins. Sachin, can you define for us what constitutes large deal for IKS?

Sachin Gupta: Yes, that benchmark continues to change, Ruchi, over a period of time. But I will say that in today's world, a large deal for IKS still is one that is able to contribute north of \$10 million in annual contract value per year.

Ruchi Mukhija: Got it.

Sachin Gupta: Now, many of these deals will take some time to ramp to that. But as long as they have that type of a commitment and potential embedded in them, they will qualify for the large deal.

Ruchi Mukhija: Noted. Secondly, we have given aging of our top 10 and top five clients. The aging reported for three quarters implies that they have been shown in our top five and 10 accounts. Is that the right understanding? And the reason for that?

The second part of this question is, what business attribute by you, as a management team, look to track when you are looking at a client aging? Is the client vintage, which stresses the deep client relationship? In that case, this number should show consistent and gradual increase? Or is it a churn, which has been the characteristic of this metric when you look at the past history?

Nithya Balasubramanian: Hi. Let me take that, Ruchi. Thank you for the question. So, what do you see on the slide in terms of vintage of top five and top 10, the numbers are not really comparable. If you look at IKS legacy, our vintage has only improved. There are some clients after AQuity acquisition who got into our top 10 and top 5 who had slightly lower vintage. So, that's the reason you have seen, let's say, if you are looking at Q3 FY '24, our top five client vintage, the numbers might look like it's come down from six to five, it's only because the mix changed with AQuity acquisition.

Ruchi Mukhija: So, going forward, what I am trying to understand is the churn is a constant feature of this metric, or should we expect that the large client stay in this top position and this vintage keep on increasing?

Nithya Balasubramanian: No. This vintage will keep on increasing. I think at the outset, Sachin had mentioned that 95% of our revenues are from repeat clients. And I can share that even amongst the AQuity clients, since our acquisition, we have not really lost any top clients that we are focusing on. And of course, IKS clients as well remain steady with us.

Ruchi Mukhija: Got it. Thank you and all the best.

Moderator: Thank you. The next question comes from the line of Devesh Mehta from Preeti Investment Managers. Please go ahead. Ladies and gentlemen, Devesh has left the question queue. We move on to our next question, which is from the line of Hemendra Kumar, an investor. Please go ahead.

Hemendra Kumar: Good morning. Thanks for the opportunity. I really appreciate the business, the model of what IKS operates. My question is, as you have a platform helping admin jobs and taking care of physicians' tasks, etc., what is the company's perspective on continuum of care to patients? Maybe I am not aware of U.S. policies about patient care.

And are you doing anything in that aspect where you can devise a model where you can give a dietary management or li

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lifestyle diseases where you can focus? Because you have a very large human capital and maybe you can operate on this model also. But again, as a pilot project, can you do the same for patients in India so that as a pilot batch, you can generate the same across the U.S.? Thank you.

Sachin Gupta: So, Nithya, do you want to take that? I had trouble following the question.

Nithya Balasubramanian: I did too. I am sorry. If you do not mind repeating the question.

Hemendra Kumar: Yes. My question was basically on continuum of care to patients where typically you are operating a model which is of physicians taking care of admin tasks, physicians, etc., joining the hospital and exiting the hospital, etc.. But is there any model from your side of continuum of care to patients who are leaving the hospital and leading a life where they are having lifestyle diseases, chronic conditions, and are we doing anything to manage such people? Is there a policy in the U.S. where you can contact patients and give continuum of care because you have a large human capital? And can you do the same in India also?

Sachin Gupta: It's a great question. Actually two questions there. So, yes, like as I was referencing when the question was asked around some of the new features we are creating, our entire offering that enables total cost of care management in fully delegated risk contracts actually involves exactly that, where we are building care coordination, transition of care management models, enabling effective home-based care for patients, community-based care enablement, all of that is really aimed at managing the patient across their journey within the continuum of care, including their home.

And so yes, from our total cost of care offering, sir, we will actually start to take baby steps in the realm that you are talking about because the reality is that outside of end-of-life care, as much as 60% to 70% of all cost of care is driven by chronic lifestyle disease. And to manage those patients, one needs to sort of follow them and meet them wherever they are in their care journey.

So, yes, we will start to see significant expansion into managing our customers' customers, which is our customers' patients across the continuum of care. As it relates to India, we feel right now we are still focused in the U.S. The Indian market to us seems a little immature. For example, a lot of the ambulatory physician office-based care isn't even covered by insurance. So, as the Indian market matures a little bit, I am sure there will be learnings from our journey in the U.S. that we will be able to bring to India, but that's not an immediate-term focus.

Hemendra Kumar: Yes. Thanks for that answer, but I just want to update you that from 2008 or '09 onwards, the workshop done by a U.S.-based company has invested in continuing of care for the diabetes management molecule in India and I was a part of that program which was successful and I have seen reduction in HbA1c, cardiovascular management, patient satisfaction, etc., dietary management. It was a wonderful, wonderful program. So, you can take feedback from such companies and implement and maybe guide your team on management practices, etc. Thank you.

Sachin Gupta: Thank you for the suggestion.

Moderator: Thank you. Ladies and gentlemen, as there are no further questions, I now hand the conference over to Mr. Saransh

Mundra for his closing comments.

Saransh Mundra: Hi. Thank you, everyone, for joining our first earnings call. It was a really insightful session. I think many more participants came in than we expected. For any further questions, please feel
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free to reach out to me or anyone in the company. We will be very happy to answer you. Thank you, everyone.

Moderator: Thank you.

Sachin Gupta: And again, I will just end by saying that we are literally at the start of this, what I think is a multi-decadal secular journey to drive sustainability in the U.S. healthcare market and we are super excited to have you on this journey with us.

Moderator: Thank you. On behalf of ICICI Securities, that concludes this conference. Thank you for joining us and you may now disconnect your lines.

Please note that this transcript has been edited for readability.

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Call: May 2025

May 22, 2025

BSE Limited
The Listing Department
Phiroze Jeejeebhoy Towers
25th Floor, Dalal Street
Fort, Mumbai 400 001
Maharashtra, India

BSE Scrip Code: 544309
National Stock Exchange of India Limited
The Listing Department
Exchange Plaza, Plot No. C/1, G Block,
Bandra Kurla Complex
Bandra (East), Mumbai 400051
Maharashtra, India

NSE Symbol: IKS
Dear Sir/Ma'am,

Sub: Transcript of the Q4 FY25 Earnings Conference Call

Pursuant to Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, please find enclosed the transcript of the earnings conference call held on Friday, May 16, 2025, for the Company's financial results for the quarter and year ended March 31, 2025.

The said transcript has also been uploaded on the Company's website and can be accessed through the following link:

<https://ikshealth.com/ir/2025/q4/transcript-q4.pdf>

This is for your information and records.

Thanking you.

Yours sincerely,
For Inventurus Knowledge Solutions Limited

Sameer Chavan
Company Secretary and Compliance Officer
Membership No. F7211
Encl: As above

"IKS Health
Q4 FY25 Earnings Conference Call"
May 16, 2025
MANAGEMENT : MR. SACHIN GUPTA – FOUNDER AND CHIEF EXECUTIVE
OFFICER
MR. NITHYABALASUBRAMANIAN – CHIEF FINANCIAL OFFICER
MR. SARANSH MUNDRA – AVP INVESTOR RELATIONS
MODERATOR : MR. RUCHI MUKHIJA – ICICI SECURITIES LIMITED
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May 16, 2025

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As a reminder, all participant lines will remain in the listen-only mode and there will be an

opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal the operator by pressing star then zero on your touchtone telephone. Please note that this conference is being recorded.

I now hand the conference over to Ms. Ruchi Mukhija from ICICI Securities Limited. Thank you and over to you.

Ruchi Mukhija: Good morning, ladies and gentlemen. Thank you for joining us today on the Q4 FY25 earnings call of IKS Health. On behalf of ICICI Securities, I would like to thank the management of IKS Health for giving us the opportunity to host this call. Today we have with us Mr. Sachin Gupta, founder and CEO, Ms. Nithya Balasubramanian, CFO and Mr. Saransh Mundra, AVP Investor Relations. I will turn over to Saransh for safe harbor and to take the proceedings forward. Thank you and over to you, Saransh.

Saransh Mundra: Hi, everyone. Welcome to our second earnings call at the public company. I would like to begin with a safe harbor statement before I hand over to Sachin and Nithya for their remarks. As part of the prepared statements and during Q&A, we may make certain statements which are forward-looking and may involve significant uncertainty. We do not take any responsibility to update such forward-looking statements and your discretion is advised by making any investment decisions. Over to you, Sachin.

Sachin Gupta: Hi. Thank you, Saransh. Good morning, good evening, everyone, depending on where you are. Excited to discuss our performance for Q4 fiscal 25 as well as the full year fiscal 25. On today's call, I have with me, of course, in addition to Saransh, who leads Investor Relations, Nithya Balasubramanian to get the name right, our CFO and I will kick it off with some remarks that I have on the overall health of the business and the performance at a high level, the financial performance and then I will invite Nithya to dive into the financial performance in a little bit more detail, after which we will open it up for some questions.

So, with that I wanted to start off again at the risk of a little bit of repetition, with a recap of our business. It's only our second earnings call. Some of you might still be new shareholders.

So, I'll just give a quick recap of the business and our strategy and then talk through how we're executing on that strategy and the resulting financial performance from that strategy. So, just to – and given, obviously, we're a niche business, I think that'll be best for everyone.

So, just as you know, we're operating in the U.S. healthcare provider market and specifically given its cost, quality and access challenges, one of the things that we have seen that is

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happening is this transformation from reactive, hospital-centric medicine to physician-led, patient-centric and proactive medicine. And so, we believe that the physician setting becomes a new fulcrum of care delivery as a result of that.

That physician setting in the U.S., as you will remember is about a \$1.5 trillion market and that has under gone a tremendous amount of consolidation over the years and hence it has created this enterprise-scale market segment that IKS operates in. Our fundamental thesis is that when a cottage industry like the original physician market was it was highly fragmented in the U.S. with 900 thousand physicians, most of which existed in groups of five or less.

When a market like that consolidates into enterprise-scale and needs to become the new fulcrum of care delivery, they go through some fundamental transformations and one such transformation is what we call this transformation from where they rediscover their core versus chore equation within their task continuum and the smart enterprises figure out that they should focus all their energies on the few core tasks where they create the most differentiated value and then a lot of the chores that are important to run the business, but might not be where they create differentiated value, typically get delegated to some platform or entity that can do these chores better, cheaper, faster at scale. Now, obviously, given the size of the U.S. healthcare market and specifically the physician market itself being \$1.5 trillion, the chores that we have identified, which cut across the entire spectrum from the exam room where the physician sees the patient all the way to the back of fice, those chores are almost 15%.

The cost is almost 15% of their revenue, which makes for a \$225 billion dam just for the chores that we have identified and what we saw over the last 15 years with the business is that this phenomenon of consolidation and hence the delegation of chores really started to pay out and hence about 35 billion of that 225 billion has already been delegated and so this \$225 billion-odd dam is actually growing at nearly 8%, but the outsourced TAM, which is around \$35 billion right now is growing at 12% and that's really that outsourced TAM or the concentrated or the consolidated segment of that outsourced TAM is really what we are addressing. One other point, obviously, to remember is our right to win. Now, in a market which is as large as \$225 billion growing at 8% or 34 billion growing at 12%, obviously a lot of players come in because they want to share that pie.

Now, what's happened, though, unfortunately is because the size of the dam is so large, a lot of players that have come in are really point solution vendors, as I call them. So they've decided to focus on one or two or maybe, in some cases, four or five of these chores and our thinking

was that as the market matures, buyers will realize that they can't be in the business of buying 10, 12, 14 point solutions, integrating them.

And so our vision was we will create a platform that has the full breadth of these 16-odd chores and eventually buying behavior will graduate more and more to that platform buying behavior, and so having said that because that buying behavior is a sort of graduation curve, today, we have to live in the duality of certain buyers that still are buying more

point solution

and then early innovators that are starting to demonstrate the platform buying behavior and that becomes a very important strategic execution predicament for us.

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Just in a couple of minutes, I'll lay out five key strategic execution pillars for ourselves. One of those pillars, as a result of this dynamic of buying behavior, where some are buying point solutions and expanding, whereas others are now starting to buy platform, we have to execute in a way where we are leaders in each of the features of the platform, so we can win in the point solution buying behavior as well as we are, of course, the only company in the world, perhaps, today, that has the full breadth of the platform.

So, if you can go back to the previous slide, that's a high-level overview of the marketplace.

Let me now talk a little bit about the five key strategic pillars with which we are executing in this business. And so the first and foremost pillar is that we are moving to an AI-led, AI-native manifest of this platform.

And so I've always spoken in the past about we started in a human-led, tech-in-the-loop model and we've gone from human-led, tech-in-the-loop, now starting to go to tech-led, human-in-the-loop, but with the pragmatic use of GenAI, we now have another set of evolution happening rapidly, which is going from tech-led and human-in-the-loop to fully autonomous technology for certain features.

So, that's going from human-led, tech-in-the-loop, in some cases to tech-led, human-in-the-loop, but in many cases to fully autonomous GenAI and the execution of features is one of the most important strategic execution pillars for our strategy going forward. That's number one. Number two, as many of you will remember, we acquired a significant-sized business called AQuity in the third quarter of FY24 and when we acquired AQuity, we had to execute on that acquisition in four key dimensions.

One, of course, given the size of their business, which is almost similar to our business at that time, we had to integrate this into one core organization that came across as a platform in the market. So, that, I'm happy to note, we executed really well on that, mostly through FY25. That was one of the big focus areas of FY25.

The second piece was, as you remember, AQuity was a very human-led, US-based headcount execution model and hence it presented a huge margin optimization opportunity and one of our key vectors of execution within the AQuity acquisition was getting their margin. If you remember, our pro forma margins in FY24 when we acquired AQuity had gone to about 24% and the idea was that as we transform AQuity's operating model, we'll be able to get our blended margins to the early 30s over a period of time.

So, that was something that we would execute on through most of FY25 and then, of course, from going into FY26 as well. The third key vector was AQuity's 450-plus large enterprise-scale clients. There was a big opportunity to cross-sell the various features of the IKS platform into the AQuity install base. Remember, we had 16 features in our platform AQuity barely had about three.

So, it created a whole cross-sell opportunity and that obviously, that execution had to be started late FY25 and really would start manifesting most of FY26 onwards. And then, last, but not

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the least another important vector was in addition to 450 clients that AQuity had, they had another nearly 250 clients that were not those consolidated, enterprise-scale clients. And as you know, the cost of servicing small clients is almost as much as servicing large clients and they don't yield as good margins.

So, the idea was through FY25 and FY26, we would cut a lot of that scale of AQuity customers, which I'm happy to report also is something that we've executed fairly well on in FY25. From the time of our acquisition, where we had 850-plus customers, now we are down to, as of March 31st, 2025, about 700-odd customers, still some ways to go in FY26, but that's a vector that we have executed well on in FY25. So, that's the strategic pillar number two.

Pillar number one, take the platform from human-led to autonomous in some cases and tech-led in some cases. Pillar two, get all the elements of the AQuity acquisition correct. Pillar three, execute on this thesis of being number one or two or three in each of our 16 features,

even as we are executing on building the platform. That requires us to establish leadership in each of the features and prove that leadership through analyst rankings.

We are typically focused on KLAS, and Black Book, Everest. And so, I think we've made significant progress along those lines in FY25 and I will talk to that third pillar. The fourth pillar, obviously, is the growth strategy. On the growth strategy, I just want to clarify, there are essentially three market segments that we are operating in. Large health systems, mid-size health systems and independent physician groups that could be private equity-owned or sometimes they are public companies as well.

And in those three, we have a differentiated growth market strategy. Large health systems, the buying nature is still very, very rigid, on-point solutions. So, there are strategies we will continue to orchestrate, a land and expand strategy. We will establish leadership in our features and sell those features as a point of entry to those large health systems and then expand from there.

In mid-size health systems, I think we're already starting to see a graduation from point solution buying to platform buying. So, there our strategy will be, the market will be much more platform-oriented. And then, very interestingly, in the PE-led platforms or even publicly traded platforms, we're starting to see more and more appetite for the platform buying. So, just keep this in mind, two of the three market segments, mid-size health systems and independent position groups, we will see more and more of a platform driven growth market.

In large health systems, we'll still have a more land and expand type of strategy. That's our fourth key strategic pillar, which is our growth strategy. And the fifth one is that slowly, but surely, we will continue to move more and more to an outcome company. Now, this is a really important area because most of the healthcare IT ecosystem that exists in the U.S. has been in the world of, we will provide you a piece of technology and leave you with the challenge of figuring out how to deliver economic or clinical value for it.

We believe that one of our goals going forward, based on the history of our company over the last 18 years and the genetics that we have built, with a little bit of fine-tuning, we w

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in the business of taking more and more accountability for driving outcomes for our platform, rather than just providing them the widgets. And that is a gene pool that we are now starting to hone and build. And that becomes the fifth key strategic pillar of our execution approach.

And so what I wanted to lay this out for you all, because every earnings call, I will now refer to these five pillars of execution, so that we can relate to them and understand what is working in it. And if we have to pivot on any of them, we will talk about that. So with that said, let me just quickly talk about our execution across these five pillars and put in the context of numbers very quickly.

First, of course, let's just go to our feature cluster platform view. And I'm happy to note that we've actually made significant progress in that transformation from both human-led to tech-led, but in some cases, tech-led, autonomous, GenAI enabled. If you look at the chart that Saransh is flashing, you'll see Scribble is now, we have a variant of Scribble called Scribble Now, which is a fully autonomous, ambient, listening Scribble technology built on GPT-4 and in the market now.

With Scribble Now, we now have the most comprehensive range of clinical documentation solutions for physicians in the U.S., meeting physicians where they are in their journey of using clinical documentation, all the way from transcription, dictation-oriented transcription solutions to fully autonomous ambient listening technology. We've also made significant GenAI enabled progress in our coding feature, which is a very large feature in our platform. We already have the autonomous coding piece figured out for at least two very significant medical specialties and we'll continue to progress that rapidly, having figured out how that technology works and having the access to mature that technology based on all the data and our involvement. So, significant progress towards autonomy in coding, massive progress towards autonomy and another very important feature, which is patient credential clearance, which is essentially built around prior authorization.

Many of you must have heard prior authorization for care that patients need is a very big challenge in the U.S. Often, lack of prior authorization leads to care not being allowed for patients, critical care not being allowed in some cases for patients. And so, it's a huge burden for physicians to prove that the care that they are about to provide to the patient or recommend to the patients is actually needed for their condition.

So, we've actually built a very exciting autonomous technology with EVE that actually addresses that problem. And then, there's been a whole bunch of progress from a GenAI perspective in several features of our revenue cycle feature cluster within our platform, where a lot of progress has been made in the denial prediction and prevention dimension because

revenue cycle is garbage in, garbage out.

The more you prevent at the front end, the less work you have to do at the back end. And our denial prevention strategies are a very, very important part of that. And then, of course, we've also been able to make some progress from our technology perspective and GenAI perspective

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in predicting patient behavior and differentially nudging patients, both to improve clinical outcomes and financial outcomes.

So, that's been some of the progress on that key vector of moving from human-led to both tech-led and autonomous-enabled through GenAI. What does this do for us? It does three things. It fundamentally improves the scalability of our solution. It improves the velocity at which we would scale, because imagine a platform deal that we would do. In a pure human-oriented model, it might take us six months to implement the platform.

In an AI-oriented model, we might be able to implement the platform in three months, across all the features. So, scalability, velocity and then, of course, margins continue to improve as a result of more and more tech deployments. So, tremendous progress in that strategic vector. I will talk about our progress in the other four strategic vectors more in the context of our financial numbers.

So, let me provide a high-level overview of our financial performance for the fiscal quarter 4, for fiscal year FY25. I have to say that I think we've had a tremendous performance quarter that gives us a lot of excitement going into the future. For fiscal quarter 4, we're happy to report a 17% year-on-year growth in revenue, coming in at about INR724 crores. In addition to this performance, it's important to note that this 17% growth is in spite of the headwind of cutting the AQuity customer base.

Remember, we've taken that from 850 total customers to now 700 odd. Also, it withstands the loss in revenue per customer that happens if we transform the AQuity margins because, obviously, we incentivize customers to transform the model from a pure US-based tech model to a tech-led, in some cases a GenAI autonomous tech model supplemented by offshore human capital in India.

So, in spite of those headwinds in revenue, we have 17% year-on-year revenue growth and absolutely tremendous execution of margins. Our EBITDA came at INR226 crores for Q4 fiscal 2025, which represents a 68% year-on-year growth from Q4 of FY24. So, tremendous execution on the margins.

I will say, and Nithya will perhaps provide a bit more details -- there were some one-offs in the FYQ4-24 numbers that were deflating the margins. So, that has partly contributed to the 68%, but irrespective of that, there is tremendous growth. And then, of course, that's resulted in a massive PAT growth of about 133%.

So, tremendous execution across the top line and the bottom line in Q4, leading to a successful quarter. EBITDA margins have gone from the 24% pro forma that we had in FY24 to now Q4 FY25 has come in a little off of 31% EBITDA. So, I think that's been a tremendous progress in this quarter as well.

And we stay on track for the margin optimization endeavors that we had spoken about when we started introducing the business to you. So, this starts to speak to that second vector, which

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is the AQuity acquisition, right? The second strategic vector. We're fully integrated as we talk on AQuity.

The margin optimization is in full flow, which has led to this 31% plus EBITDA in FY25-Q4.

And of course, there's a little bit more to come in FY26. I will talk about the cross-sell. I'm happy to report that we've been able to cross-sell features of the IKS platform to four significantly large health system customers in the AQuity install base.

We're not at liberty to reveal their names, but super excited to see that early green shoots of cross-sell play out. And then I already said the right sizing of their customers has happened from total customer base of 850 to 700. So, that second strategic vector is execution strongly reflected in our numbers.

One other thing I want to say about our Q4 numbers is, I said this on our last call as well, in our business, due to quarterly seasonality typically it's not necessarily a good idea to look at Q-on-Q both. But I realize that you guys are used to looking at Q-on-Q growth in addition to year-on-year growth.

So, we put those numbers here as well, if you look at the graphs around quickly. And the Q-on-Q growth on a constant, on a collective quarter basis for FY25 is 10% Q-on-Q revenue, nearly 13% Q-on-Q EBITDA, and 14% Q-on-Q PAT margin. So, not only a strong quarter of execution from a year-on-year perspective, but a very strong order of execution from a

consecutive quarter basis as well.

All of this, if you look at the bottom line, is essentially being enabled with strong transformation of margins in the legacy AQuity customer base, but then also some tremendous customer additions that we've had, that we are happy to announce. And I'll put them in perspective of our market segments.

The first one I'd like to talk about is Sky Lakes. This falls into that mid-size health system marketplace. When I spoke about our growth strategy pillar, I spoke about the market segment, and I said mid-size health systems. Mid-size health systems are showing more and more of a proclivity to the platform construct.

And so, happy to report that Sky Lakes has actually committed to the entire IKS Care enablement platform for their employed physician base. Super exciting. It's a mid-size system in the Pacific Northwest. Our definition of a mid-size health system is one that has revenues south of about a billion dollars.

So, \$250-\$300 million to billion-dollar health system range is how we think about these mid-size health systems. What's also exciting about Sky Lakes is, if you remember, we have 16 features that enable the physician business.

But we were also being pulled by the health system, the hospital ownership for these physician businesses in the market segment of the health system, saying, why don't you apply the same

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core versus true discipline to our hospital business as well? And we were just sort of dabbling in it.

But, you know, the way IKS typically works is when we want to really launch in a particular space, we like to take on ownership of the function end-to-end. So, in Sky Lakes, what makes it super exciting is they've also given us ownership of their entire hospital revenue cycle.

Now, you'll see that in the hospital RCM space, there are many vendors that do point solutions, even within RCM, where they'll just be doing AR follow-up or patient AR. With Sky Lakes, we have responsibility for their entire hospital revenue cycle, in addition to the manifest to the full care enablement platform in their employed physician base.

Important to note that if they're able to manifest and grow this business effectively in the hospital RCM space, that actually expands that \$225 billion TAM by another maybe \$50-odd billion, which is the hospital RCM TAM. So, super excited about Sky Lakes, reflects that strategy that we have around mid-size health systems for clarity to buy more and more platform use.

Second exciting deal, Western Washington Medical Group, already an RCM, customer of ours, independent medical group in the Pacific Northwest, also has embraced the full manifest of the IKS Care enablement platform.

Again, right in sync with our strategy, two segments of the market, showing proclivity to platform constructs, mid-size health systems, and independent medical groups. They could be private equity or they could be publicly traded. Western Washington, fine example of that.

Third, super exciting win, Platinum Derm, another private equity-owned dermatology platform, maybe the number two largest platform for dermatology in America, also was an RCM customer initially, and now they've embraced the full breadth of the IKS platform that we implemented there.

Another very important win, OrthoNY, that really starts to put us in the orthopedic specialty, which is also going through massive consolidation. And they've embraced our revenue cycle platform and are also doing some innovative things with us in the patient engagement hub future of our platform.

And then last but not the least, a massive, massive win for us with GI Alliance. This is a \$2.5 billion-plus company that employs 1,000-plus digestive health physicians. Again, \$2.5 billion-plus in revenue, the world's largest digestive health physician roll-up. They were recently acquired by Cardinal Health, which is a Fortune 50 company, for a cash consideration of maybe \$3.9 billion.

With that acquisition, Cardinal has also announced that they will be expanding GI Alliance's capabilities into other specialties like urology. And the idea there really is for us to become the de facto RCM execution platform for all of their endeavors across all of their specialties.

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So a really, really marquee and exciting set of wins that, again, talk to the strategy that we have for growth, where mid-size systems and independent groups are showing more and more proclivity to buy platforms. Large health systems still continue to show more point-solution behavior.

Last but not the least, my remarks is, if you would go to the execution that we're having around

being a leader in each of the features versus building the whole platform, and that – I'm really happy to announce that Black Book has recognized us. Next graph slide.

Black Book has recognized us as the number one player in AI-driven RCM Solutions, critical documentation, and medical coding. And then in addition to that, CLASP continues to recognize us as the number one player in transcription, but they've also indicated that they would collapse, all of the variants of critical documentation into one category . And if they did that, I think we emerge naturally as one of the leaders in that space. And then, of course, from a hygiene perspective, we continue to advance all our security-related controls to the HITRUST r2 certification.

So all in all, a very, very exciting and successful execution order that I think encapsulates all the hard work that the teams have been doing. At a very high level, as we touch upon FY25, before I turn it over to Nithya, FY25 – if you go to that graph slide for FY25 – a year of strong execution, 47% year-on-year growth in revenue that has led to nearly a 50% year-on-year growth in EBITDA.

Revenue came in at INR2,664 crores, resulting in a 791-odd crores EBITDA that then created a INR486 crores PAT, which was a nearly 31.2% year-on-year growth for FY25. Obviously , the PAT growth is muted relative to the EBITDA growth because of two reasons. One is the interest that came on the debts that we had taken on to acquire AQuity .

And second is the amortization of the intangible assets that came with the AQuity transaction. So, again, to sum it up, a strong order of execution, FY24-FY25 Q4, resulting in a pretty strong year, good execution across each of those five strategic pillars that we will keep talking about. With that, I will turn it over to Nithya.

I have one other remark to make after Nithya finishes right at the end. But with that, Nithya, over to you.

Nithya Balasubramanian: Thank you, Sachin. Saransh, if we can flash slide 15. I think Sachin has provided you enough color on revenue, so I'll talk a little bit about the expenses. If you look at slide 15– I'll start with Q4 commentary and then give you a bit more color on the year as well. If you look at employee benefit expenses, which tends to be our largest expense item, You will actually realize that we have been able to reduce that significantly both year-on-year as well as quarter -on-quarter . If you compare it to Q3, this number was 57.3% as a percentage of

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revenue. From there, we have been able to bring it down to 51.8%. And if you compare it to Q4 of last year , that number was 60.8%.

We have actually been able to grow revenues despite the reduction in headcount. Headcount, you will note that we were at 13,241 at the end of last year. From there, in Q4 FY25, we ended the year at 12,661.

If you look at adjusted EBITDA, which excludes ESOP cost, that's still at 32.8% for the quarter . EBITDA, as Sachin had mentioned, still at 31.2% compared to 30.5% in Q3, a 70 bps margin expansion. As you compare it to Q4 of last year, that number was 21.7%. So significant expansion compared to last year .

It's been driven by two, three different factors. One was, of course, there was a little bit of a one-off in the base, which was towards the integration and acquisition costs when we were acquiring AQuity last year. But even excluding that, margins have expanded very significantly driven by one realization of cost synergies in the administrative expenses.

But more importantly , the transformation of AQuity's business model, where we have been able to do both a combination of deployment of IKS's tech as well as changing the mix from onshore to offshore. And of course, there's been a ramp-up of the new customers as well. Looking at finance cost, we had about INR20 crores of finance cost in the quarter , and D&A was at INR28 crores. Year-on-year , you'll actually see a meaningful reduction in finance cost, and that has been driven by our strong cash generation and therefore our ability to pay down debt.

If you look at ETR for the quarter , it was about 18%, but for the full year, it was about 19%. This took back to INR148 crores or 20.4% in terms of margins. The ETR for the full year, as we mentioned before, was 19%, but please expect ETR for the next year to be around 21% to 22%, because we will be losing some tax breaks on one of our SEZ units.

A few more commentary on the full year numbers. The full year revenue growth, as you will note, is 46.5%, but do remember that the base FY24 includes five months of AQuity . We ended the year with an EBITDA of 29.7%, and you will note that the pro forma EBITDA of IKS and AQuity combined for 12 months in FY24 was 24%.

So a significant margin expansion compared to FY24. Finance cost has increased to INR89 crores for the full year, but this is due to the full year impact of the debt that we had assumed when we acquired AQuity . The full year PAT was 18.2%, as against the 20.4% last year. Please note that the pro forma PAT was actually 12%, so again, comparing like-to-like, PAT has also expanded quite significantly .

Saransh if you can go to the cash flow slide. So our cash flow remains very, very healthy . If you look at the operating cash flow, we ended the year at INR434 crores. If you look at free cash flow, it ended at INR275 crores, both of which represent very, very strong growth

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year-on-year . This is despite a INR139 crores performance guarantee that we had extended to Palomar Health, one of our marquee clients.

We had discussed this with you in the last call. If you look at our net debt position, again, we have been able to bear it down quite significantly . If you look at FY24, we ended the year at INR860 crores. From there, we have been able to bring it down to INR560 crores. Again, despite the performance guarantee we had extended.

If you can go back one slide up. So, EPS for the full year was INR29 and return on equity remains healthy at 27.2%. The GAAP between FY24 and FY20 is 25, largely explained by the fact that some of our strategic investments were revalued at a higher number . Therefore, the equity expanded without any commensurate expansion in profits.

Now I will stop my remarks here and hand it over to Sachin.

Sachin Gupta: Thank you, Nithya. I appreciate all the details. So, as you saw, a really strong quarter of execution and really a strong fiscal year of execution that has now resulted in us being one platform that is ready to execute in this mission of creating, pretty much, what is a new market. And Saransh you can move to slide 5 for my last set of comments. I want to end by saying that it's really important to also recognize what's happening in the market. In addition to talking through the numbers, my other two objectives from this call was to lay out the five pillars of our execution strategy for you.

So that we can continue to refer to those pillars and build a consistency in our dialogue. And the second was this whole conversation around making sure we calibrate on what's happening in our market from a competitive perspective. Just to put it in perspective, there's been \$45 billion of equity investment from 2021 to 2024 in this larger healthcare IT landscape.

This \$222 billion TAM and now the \$275 billion TAM with Acute RCM coming in, as well as the \$35 to \$40 billion TAM that's already outsourced that's growing at 12%. Important to note, what are the key characteristics of this investment of \$45 billion?

Most of it has gone towards what I call point solution companies. A lot of it specifically within point solutions, revenue cycle point solutions. So, whether you look at the big boy R1 that was taken private from being public by CD&R and TowerBrook for a valuation of \$9 billion, even when they were barely making any net income, \$3 billion net income.

Or when you look at some of the marquee private equity arms, Indian arms of marquee global private equity firms investing in more India-based tasks, RCM vendors that are trying to graduate and go upstream in working directly with healthcare providers in the US.

For example, the GeBBS, the Accesses and the Infinixes of the world or the AGSs of the world. Or then you start looking at the US-based venture capital type leading players like you start to think about the money raised by an abridge, which as you know , is actually one of our strategic investments as well.

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And we had invested just about 18 months ago, perhaps, at a pre-money valuation of nearly \$100 million. And they just raised \$400 million at a valuation of \$2.75 billion. So, within 18 months, the valuation has gone from \$100 million to \$2.75 billion. Of course, they don't yet make money and they're predominantly a point solution when they're specifically in the ambient Gen-AI critical documentation space.

Or Suki, for that matter , where Zoom made a strategic investment and they're also a point solution for critical documentation. Or for that matter , Innovaccer , where they recently raised a big chunk of capital at a valuation of \$3.5 billion. All this to say that there's a lot of investment happening in this space. And in my mind, that investment signals three things.

First, it is an imminent recognition of the massive opportunity in this space. So, that is really important. The type of players that are investing and the quantum of capital that is being invested is massive.

Second, naturally , that's going to create competitive intensity . And so, yes, we should expect significant competitive intensity backed by large pools of capital in these spaces. But what's exciting for us is really , if you think about it, most of these companies are still very point solution oriented.

Especially after some of our wins of this quarter and last quarter , we feel like our strategy of where there will be recognition by the buyers of the value of the whole being greater than the sum of the individual parts and the graduation towards the platform approach, I think is going to be a very , very important cornerstone of our strategy .

And hopefully, if we can translate that to also really making sure we deliver outcomes from the platform, it starts to create a mode that in spite of all this capital going into these point solution companies. We will be able to withstand the competitive intensity that is coming our way without compromising margins.

And then last but not the least, humbly, I want us to think a little bit about the capital efficiency with which we are building the business. As you can see, so much capital is going into each of these point solution companies. And here we are building the full breadth of the platform, essentially through the internal accruals and the power of our organic balance sheet.

So I wanted to bring that to a conclusion. In conclusion of my remarks, again, a strong quarter of execution. Hopefully through this dialogue, we'll continue to calibrate more and more on the strategic execution pillars and continue to report our performance against them.

One last remark, obviously, given that this is still a market that is being discovered at the platform level, I do want to highlight that we're not yet in our market at the level of maturity where you can model out exactly what the growth curve will look like.

And hence, I've humbly been saying, look, I believe we will grow faster than the TAM is growing. The TAM is growing, the outsourced TAM is growing at 12%. I do believe

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fundamentally, if you model us out in the long term, we will continue to grow faster than the 12% and our earnings growth will continue to be faster than the earlier growth.

But it's impossible for me to provide any guidance as it relates to what will happen next quarter and next quarter after that. So I know many of you are anxious for that. Many of you are wondering if it was INR225 crores of EBITDA and Q4 FY '25, how much can we lock in from that for FY '26?

I just want to highlight that it's not a mature enough market where I can give you that type of guidance, which is why I will continue to refrain from giving guidance about future performance. And if you could please keep that in mind as you ask questions. With that, thank you for your time and attention. I will turn it back to the operator for questions.

Moderator: Thank you. The first question comes from the line of Ruchi Mukhija from ICICI Securities Limited. Please go ahead. Ruchi, if you can please unmute your line and ask your question.

Since there is no response, we move on to our next question, which is from the line of Sameer Shah from ValueQuest. Please go ahead.

Sameer Shah: Yes, hi. Congrats, great numbers. First question is if you would like to address the UnitedHealthcare news flow and the impact that it would have on our company. And second, you know these large deals that we have signed, typically when do they, when do they -- what is the kind of ramp up time that they would take?

Sachin Gupta: All right. Thank you for the question and your compliments. Appreciate it. First, UnitedHealthcare. Look guys, it's obviously a difficult time for them. They have been hammered with three macro issues that have hit them sort of back to back, right? One of course is the HCC Version 28 that is compressing their margins on the Medicare Advantage contracts that they have, which is a big part of their business.

Second, as you know, the CEO of their insurance business got assassinated just a few months ago. And third, they have this big security issue with Change Healthcare. So I will tell you that there are not many other companies in the world that can withstand three such back to back events that come at them in such dramatic fashion.

Having said that, they are still the world's largest commercial health insurer. They I think have something like 60 million American lives insured, just to put that in perspective. And you know, the reality is IKS's relationship with UnitedHealthcare is through OptumHealth, which is about \$110 billion subsidiary of UnitedHealthcare, which employs 50,000 plus physicians, the world's largest physician employer.

And we obviously are building our care enablement platform for OptumHealth. And remember, our care enablement platform works on the non-discretionary spend, on the opex. And so our platform is all about creating more financial value from the opex that they're already incurring.

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So if you look at what's happening with us over the last 18 months, where they've been hit by these three dramatic events, I think our business with OptumHealth has grown at least 25%-30% if not more over that duration. So without going into any speculative thinking about what might happen, the CEO has changed.

Look, the business isn't going anywhere. It's the world's largest commercial health insurer. It has too many lives where anything's going to go wrong. It's not like they don't make money anymore. They make money, but they make less money than they used to because of industry

headwinds that they are facing. And so they're having a big valuation decline. And in a country like America, big valuation decline means CEOs have to take the hit. And so that's really what is happening.

Our relationship has only grown over this period. And we have no reason to believe, based on the nature of our relationship, that these headwinds that they are facing create anything but tailwinds for us. Now, again, I don't want to be speculative, but I will tell you that from everything I'm able to see rationally, taking the emotion out, the OptumHealth relationship looks very, very strong for us going forward.

The second question you have, Sameer, is as it relates to the ramp-up of the new strategic customer that we assigned. I will say, Sameer, typically what happens is full-platform execution takes somewhere between 4 to 7 months, give or take, depending on the size of the customer. And if it is one feature that we are implementing, typically that can come through in about 3 to 4 months.

So that is the way to think about how these ramp-ups happen. And obviously, depending on which feature it is, how much tech intervention there is or not, some of this timeline can be accelerated or not. But generally, those are sort of good benchmarks to keep in mind as it relates to ramp times due to implementations.

Sameer Shah: Super. Thanks. I'll rejoin the queue.

Moderator: Thank you. The next question comes from the line of Srivathsan Ramachandran from Avendus Spark. Please go ahead.

S. Ramachandran: Hi. I wanted to get your thoughts on two things. You mentioned about the changes that are happening in the industry with some of the private equity capital and changes, right? This coupled with AI, how do you see it? Because the biggest difference from a business model point of view means the outcome-slash-output-based pricing model you had.

Do you see customers or competition kind of changing this, offering other options which were in the benefits of technology and technology improvements within clients? Any big market changes you've seen because of these two changes that's happening in the market.

Sachin Gupta: So, thank you for your question. I appreciate it. Look, as I was saying, when there is this much capital being put behind point solutions and there is a democratization and acceleration of the development cycles due to something like AI, obviously at a point solution level, the

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competitive intensity is going to go up, which is why if you remember one of our key pillars of our strategy is that we want to continue to strive to be number one or two or three in each of our features, while we are the only company in the world building the full breadth of the platform. And I think that really is addressing that issue, right?

The reality is that Gen AI is commoditizing the rapid advancement of these features is a reality. Our alpha from that perspective is deep customer relationships that give us access to data and context to mature the Gen AI faster than some of these new competitors, even no matter how much capital they have. That continues to be an advantage for us is the deep, like think about it right now, GI Alliance, it's the world's largest installation of GI doctors by a long way.

Like this number two player is probably not more than 300 or 350 doctors. These guys are more than 1000 doctors. Now think about our ability to build the care enablement platform for GI better than no matter anybody else that comes in with a large amount of capital, we naturally have an advantage, whether it's at a feature level for GI specialty or at a platform level.

So I think, absolutely, we should expect more competitive intensity, we should expect more players at a point solution level. Our strategy to win at the point solution level is the deep customer relationships we already have that allow us to mature the data. And our strategy, obviously larger strategy to win against point solution vendors is this whole graduation towards platform buying behavior.

Having said that, I think we live in a very, very disruptive world. And so one of our biggest opportunities is to stay extremely alert from a competitive perspective and track the implications of competition in the marketplace day to day.

S. Ramachandran: Sure, thanks. I just have one quick question. The runoff of revenues on the Aquity revenue, you're more or less done close to the bottom of it. I just wanted to get your thoughts.

Sachin Gupta: No, like I said, when I was articulating the sub vectors of that Aquity strategic pillar, that -- that tail cutting will continue to FY '25 and the larger part of FY '26. And so it is not done yet.

And we will see, because remember, I said that we went from 850 total customers to 700.

Our end objective is to be ending up somewhere between the 550 to 600 range. And so there's still some work to be done and there's still some customers left to be transformed from a U.S. based headcount operating model to a tech-led offshore headcount enabled model. So those two things will continue to FY26.

S. Ramachandran: Sure, thank you. I will come back later.

Moderator: Thank you. The next question comes from the line of Nilesh Jain from Astute Investment Management. Please go ahead.
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Nilesh Jain: Hi, thank you for the opportunity . Great set of numbers. My first question is, I wanted to understand your strategy on your top 10 clients. So if you look at your top 5 clients they do average revenue of around \$15 million. You want to understand the potential for those top 5 clients. And what is our strategy to further cross sell or increase our wallet share there? And then on the next top 5 clients, wherein we do around \$8 million average revenue. How do we plan to grow these clients?

Sachin Gupta: Okay , great. Thank you for your question, Nilesh. So the reality , Nilesh, if you take our top 5 clients or our top 10 clients, all of our top 10 clients have a wallet possibility of at least \$100 million a year ACV with us. So for none of our top 10 clients today , we can say that we are anywhere close to the full wallet potential that they have.

Our largest customer , the full wallet potential is North of \$300 million. So the simple math, whether you look at top 5 or top 10 is to continue to manifest the land and expand play that we have with them. And the fact that the top 5 and the top 10 are growing is actually demonstrating the fact that that land and expand play is continuing to work out.

Now , the other thing that's happening in that mix is we are now starting because we are getting midsize customer and independent medical rooms that are manifesting the full platform, that list of top 5 or top 10 is rapidly changing. And that's why you see some of those vintages in those customers changing in the deck is that there are some new customers coming in with a full platform manifest that day one become a top 10.

And so I think this top 5, top 10 is still a little bit immature. The way to think about it is we are nowhere close to full wallet share in any of our top 10 customers. Some we have more headroom, some we have lesser headroom, but there's headroom across all and then the top 5, top 10 list, hopefully , if we execute well on our strategic vectors is actually likely going to change over the next, say , two, three years.

Because if we keep landing more and more platform customers, they naturally graduate to the top 10 faster than some of the other customers.

Nilesh Jain: Okay , given it's dynamic, if you acquire, you might like last year you acquired Palomar type deal. So that would change the top 10 clients given the so how should we look at the growth on that side? So your top 10 clients and then obviously , under the rest of the clients, obviously , depending how you cross sell them?

Sachin Gupta: Nilesh it is very hard for me to give you a way to model that, the way what I would model is every year 80% plus of our growth will come through expansion of existing customers and under 20% will come to the addition of new customers that I can tell you, but it's very hard for me right now in this relatively immature marketplace, where there are so many dynamics graduation from one solution behavior to platform behavior in certain market segments.

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Full adoption of features by large health systems that we traditionally not seen, it's very hard for me to give you how to model our top 5, top 10. I will just say 80% of the retail growth, 80% plus from existing customer expansion and under 20% from new customer acquisition.

Nilesh Jain: Okay , my second question is we have seen a good amount of reduction in your employee count by almost 450 employees from the previous quarter . So what is driving this efficiency and how do we see the employee count grow for the next FY26?

Sachin Gupta: So Nilesh the way to think about is Nithya has this statistic, I'll ask her to tell the specifics of it where for FY20 to 24, our revenue grew significantly faster than our headcount. That non-linearity demonstrates the power of our technology led disruption. And there are two vectors in our technology led disruption, where we go human led to tech led, which is incremental reduction of headcount for the same solution.

And then there's human led to fully autonomous, where the disruption of headcount is significant. And that is also being manifested predominantly in the AQuity customer base where we've dramatically transformed the AQuity margins from where we acquired them. So that's the way to think about it, we will continue to drive non-linearity .

We have remember now I think about 460 odd technologists that are writing a lot of this proprietary technology . We have a GenAI center of excellence that is producing a lot of this technology . And so that trend is going to continue after Nithya if you have that statistic.

Nithya Balasubramanian: So the employee count reduction you're seeing in Nilesh is predominantly driven by the fact that we have been able to deploy both IKS as technology as well as the transition between onshore and offshore. If you look at legacy IKS, what we have been able to demonstrate in the past is non-linearity between revenue and employee count growth. If you go back and look at

FY20 through 24, our revenues grew about 25% and in the same time frame, our employee count grew only 10%.

So as the tech matures and I think Sachin has mentioned several times before, it remains a very high priority for us and we continue investing there and therefore we hope to do as much or better in terms of our ability to drive faster revenue growth with lower employee count growth in the future as well.

Nilesh Jain: All right. Just a last question or broader question on the industry . Like you mentioned, majority of the private equity players have been acquiring point solution based companies and focusing on the RCM side. So and these companies have not been able to make a profit as you know IKS has been able to generate.

So what do you think why these companies have been able to - - have been facing such challenges given you are the leader making top 30% margins and they have been not able to even make double digit. What do you think is the challenge they are facing and what we are not facing?

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Sachin Gupta: So Nilesh, I will try to refrain from commenting on what they are not doing right. I can tell you what we are trying to do and what we are trying to do is -- IKS built a business model where our pricing was outcome based. We get paid as a percentage of the customer's revenue and so that's when you combine that with our ability demonstrated ability to constantly drive non-linearity by reducing the headcount required for a particular task at the unit task level that naturally creates a margin accretive business model in which the customer certainly wins because we are actually collecting more and more for them and we win because we are able to collect that at a lower cost and since your question was RCM only I am focusing on that.

So it's based on the inherent structure of the business model that we built from the get-go that perhaps we are able to drive margin superiority and that is also one of the other reasons why I said that we will continue to make that outcomes orientation one of the five strategic pillars that we will execute on now not just for RCM, but for the entire platform because I actually fundamentally believe when I talk to buyers across the country .

I'm finding that there is a fatigue emerging in buyers of buying the next best AI point solution and then figuring out whether it delivers value or not and if they find models where their outcomes are aligned to the vendor outcomes I think there's a different level of proclivity to that construct.

So again I've been refraining from commenting on sort of what they are not doing right because I'm sure they're very smart companies and they'll figure it out, but my best guess is that the fundamental structure of our business model and our execution driven around it is what has driven our margin superiority .

Nilesh Jain: Okay . Thank you so much and all the best. I'll join back in the queue.

Sachin Gupta: Thank you.

Moderator: Thank you. The next question comes from the line of Seema Nayak from ICICI Securities Limited. Please go ahead.

Seema Nayak: Thanks for taking my question. My question is more towards the sector . So what percentage of our revenue is from Medicaid and how does the reduction in the Medicaid spending impact the provider ecosystem?

Sachin Gupta: Great question. Thank you. So please understand that our customers tend to have a fairly healthy payer mix depending on specialty . Some of them have no Medicaid at all and depending on specialty they do have Medicaid and so I don't know that I can give you the Medicaid percentage of our revenue across our entire customer base.

If I were to hazard a guess it's probably less than 10% across our entire customer base. Having said that again I keep reiterating this look we are in the non-discretionary opex business, so when reimbursement per unit of care is cut which could be first of all to understand the Medicaid policy they are not cutting the let's say that today the government reimburses \$100 per unit of care for Medicaid. That \$100 grows every year based on a certain rate of inflation.

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What the government is saying is if it was growing at 4% a year, they're going to reduce that growth from 4% to 2%. So first of all on an absolute basis the reimbursement per unit of care is still going to grow . The growth will contract and second when the reimbursement per unit of care is not growing as fast and their costs are still growing dramatically it actually puts more pressure on the customers to adopt our model further .

So from my perspective I don't look at the Medicaid growth contraction not Medicaid contraction. But the Medicaid growth contraction, I don't look at it as a headwind for us and of course it's a small percentage of our overall payer mix but nevertheless, tomorrow the same

phenomena is being faced in Medicare physician fee schedules as well, and in my opinion that's an imminent tailwind.

Seema Nayak: Yes, thanks and one more question recently there is a news that Aetna is leaving the US healthcare space so does that affect us in any way?

Sachin Gupta: I'm sorry, I couldn't catch that question ma'am. Can you repeat that slowly sorry?

Seema Nayak: So, Aetna is going to leave the US healthcare insurance space so does that affect us?

Sachin Gupta: No not really.

Seema Nayak: Okay, thanks. I'll get back in the queue.

Moderator: Thank you. The next question comes from the line of Chetan Shah from Jeet Capital. Please go ahead.

Chetan Shah: Hi, thank you Sachin and team and congratulations for a great set of performance over the years. Just two quick questions in your opening remark you mentioned about intensifying competitive scenario, and also in one of the questions, you said that it is still a premature stage, because the competition is in an investment mode and they are not exactly providing the full-fledged service the way what we are doing the business.

So my question is how do you see this, this thing evolving over a period of time and in terms of business model and margin, how will this impact us? That is the first part of question.

And second part of question is the kind of cash flow what we generate do we have any plan apart from reducing our debt from the books and in terms of any kind of inorganic small or large opportunities something what we did in last 6 to 8 quarters? These are the two broad questions. Thank you so much and once again congratulations.

Sachin Gupta: Thank you for your kind remarks and thank you for the question. See look in terms of what the future might hold in terms of growth of revenue and margins, like I was saying earlier because of the relative immaturity of this market remember only \$35-odd billion of now over the \$270 billion TAM is outsourced so far so it's highly immature it's very fragmented competitive intensity is increasing.

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So, it's very hard for me to hazard a guess by which you can model, but I understand you guys have to model so which is why I've always maintained look if the outsource TAM is growing at 12% my point is if I'm growing faster than 12% I'm gaining market share, if I'm growing slower than 12% I'm losing market share all I'm able to say is if the outsource TAM is growing at 12% I think we will grow faster than that outsource that outsource TAM, now that growth accelerates to 15% and we gain market share it will be faster than 15%.

If that growth de-accelerates which I don't see de-accelerating it might be lower and then the other thing I'm able to tell you is based on everything I am seeing, I have confidence that for the next several years our margin growth will be faster than that revenue growth that I'm talking about.

So our focus is on gaining market share versus trying to predict the exact trajectory of growth and our focus is on continuing to prove the superiority of our model by having industry leading margins that continue to grow faster than the revenue growth, so we're trying to tell you that it's very hard to tell you what the revenue growth and profit growth will exactly look like.

That's number one.

On cash flows a great question look I think, if you see our 18 years of history we are not a natural acquirer type of company we are very organic growth based company we did a very significant acquisition in Aquity we have taken a good 18 months to digest integrate that's we've laid out the four key vectors within acuity that we are executing on, we have perhaps complete execution on two of the four vectors, two are still ongoing.

I think our stated strategy is not to acquire our stated strategy is to grow organically. We might have some uses of capital periodically in these type of unique outcome orientation deals where we might be able to participate in the outcomes that we create for our customers thereby demonstrating skills again and driving that type of platform behavior.

So those might be smaller uses of cash than a typically large acquisition. Also just genetically our leadership team is not very comfortable with debt even today our debt is I don't know 0.6x something like that EBITDA and we would like to operate conservatively in a manner where eventually there is little to no leverage in the business. So stated strategy not acquisition oriented never say never but there might be some uses of capital in outcome oriented deals with customers.

Chetan Shah: Thank you so much Sachin for this, I'll come back in the queue. Thank you so much.

Moderator: Thank you. The next question is from the line on Nilesh Jain from Astute Investment Management. Please go ahead.

Nilesh Jain: So, like last year we did Palomar type of deal wherein we paid an upfront amount. Are we looking for any such more type of deals for FY '26 or in FY '27 as well?

Sachin Gupta: It was a great question, thank you for the question. See if you think about the Palomar deal shortly after the Palomar deal we did another deal that I just announced called Sky Lakes

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which is in that mid-size health system segment, and that Palomar deal by upfront the incentive we created a precedent of a large mid-size health system embracing the full platform which has already paid dividends not only in Palomar but now paying dividends in the construct of Sky Lakes, as well because they got the confidence that another peer health system was able to you know embrace the full platform successfully .

And of course in Sky Lakes we did not have to incentivize them with any sort of outcome orientation. So, Nilesh, I think the short answer is in each of these market segments smartly and strategically we might do one or two example deals that might play out over FY '26 and FY '27. But it is not the way we are going to continue to operate.

So, yes it's possible that there might be two three more deals for different market segments that might emerge over the next 12 to 18 months, but we feel very confident about our strategy and the other thing I want to lay out is we are very disciplined about tracking our return on capital in scenarios like that and so not only will those deals obviously create traditional margins by the full manifest of our platform.

But we are very clear that to the extent there has been capital deployed there to incentivise those deals, or the creation of those deals, we will be religiously tracking our return on capital on that as well. So in the end our simple thesis is either we produce superior return on capital for our shareholders or we give them the capital back.

Nilesh Jain All right. Just a bookkeeping question for Nithya, in the balance sheet the other financial asset has gone up from INR21 crores to almost INR11 crores what exactly would be that?

Nithya Balasubramanian: Other financial assets the Palomar upfront guarantee that we had paid out that is booked in other financial assets.

Nilesh Jain: Okay , sure. Thank you.

Moderator: Thank you. We have the next question from the line of Siddharth Misra from Fidelity International. Please go ahead.

Siddharth Misra: Yes. Hi I had just one question. Could you talk about your pipeline and the details around the pipeline?

Sachin Gupta: Can you hear us Siddharth?

Siddharth Misra: Yes I can hear you. Can you hear me?

Sachin Gupta: Yes. thank you for the question.

Traditionally we haven't published our pipeline or the details but I can say confidently that when we look at all of these three market segments that we are focused on the mid-size health systems, the large health systems and the independent medical groups that might be single specialty or multi-specialty might be publicly traded or private equity owned.

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We are obviously tracking pipeline by each of those market segments and the pipeline seems to suggest that we are at an all-time high in terms of where the pipeline stands. Now I will say that the buying cycles are still a little immature, so I'm not in a position to predict the conversion rate.

The reason I'm not publishing the pipeline. Siddharth, is because I see so much immaturity still in the buying behaviour in the conversion cycles and so as we start to get data that I think starts to make a little bit more sense that can allow you to model conversion timelines and dates, we'll be able to talk a little bit more about it. But I will say that activity for us right now is at an absolute all time high.

Siddharth Misra: Okay thank you.

Moderator: Thank you. Ladies and gentlemen with that we conclude the question and answer session. I now hand the conference over to Saransh Mundra A VP Investor Relations. Please go ahead.

Saransh Mundra: Thank you everyone. Thank you for joining the call. In case of any further questions please feel free to reach out to us. My email id and the Investor Relations email id are there in the press release and the link for the call. Thank you.

Moderator: Thank you. On behalf of ICICI Securities Limited that concludes this conference. Thank you for joining us, and you may now disconnect your lines.

Please note that this transcript has been edited for readability .

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Call: Jul 2025

July 9, 2025

BSE Limited
The Listing Department
Phiroze Jeejeebhoy Towers
25th Floor, Dalal Street
Fort, Mumbai 400 001
Maharashtra, India

BSE Scrip Code: 544309
National Stock Exchange of India Limited
The Listing Department
Exchange Plaza, Plot No. C/1, G Block,
Bandra Kurla Complex
Bandra (East), Mumbai 400051
Maharashtra, India

NSE Symbol: IKS
Dear Sir/Ma'am,

Sub: Transcript of Conference Call – “IKS - Partnership with and Investment in WWMG” .

This is in reference to our letter dated June 30, 2025, regarding the schedule of the Conference Call, and our letter dated July 3, 2025, regarding the submission of the presentation and audio recording of the said Conference Call.

Pursuant to Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, please find enclosed the transcript of the c onference call held on Thursday , July 3, 2025, for the “IKS - Partnership with and Investment in WWMG” .

The said transcript has also been uploaded on the Company's website and can be accessed through the following link:

<https://ikshealth.com/ir/sebi-disclosures/conference-call-transcript-IKS-Partnership-with-and-Investment-in-WWMG-Jul03-2025.pdf>

This is for your information and records.
Thanking you.

Yours sincerely,
For Inventurus Knowledge Solutions Limited

Sameer Chavan
Company Secretary and Compliance Officer
Membership No. F7211

Encl: As above

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“Inventurus Knowledge Solutions Limited
Conference Call”
July 03, 2025

MANAGEMENT : MR. SACHIN GUPTA – FOUNDER AND CHIEF EXECUTIVE
OFFICER
MS. NITHYA BALASUBRAMANIAN – CHIEF FINANCIAL
OFFICER
MR. SARANSH MUNDRA – HEAD OF INVESTOR RELATIONS

Moderator: Ladies and gentlemen, good day and welcome to the Inventurus Knowledge Solutions Limited Conference Call. As a reminder, all participant lines will be in the listen -only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during this conference, please signal the operator by pressing star and then zero on your touch phone.

Please note that this conference is being recorded. I now hand the conference over to Mr. Saransh Mundra, Head of Investor Relations. Thank you and over to you, sir.

Saransh Mundra: Thank you. Thank you so much. Welcome everyone to this special conference that we have organized to explain our rationale behind the investment in an MSO with Western Washington Medical Group.

I am Saransh Mundra, Head of Investor Relations. Before we proceed, I would like to start with a safe harbor statement as always. As part of our prepared remarks and during Q&A, we may make certain statements which are forward -looking and may involve uncertainty. IKS does not take any responsibility to update such statements and your discretion is warranted while making any investment decisions. With that, I will hand it over to Sachin. Sachin, over to you.

Sachin Gupta: Hi, thank you, Saransh. Hello, good morning, good evening, everyone, depending on wherever you are joining from. Excited to chat with you today about what I think is going to be a very pivotal and exciting milestone in our journey at IKS of enabling better , safer, more efficient care. I will dive into the specifics of the opportunity in a minute. The way I would like to conduct this conversation is I just want to set a little bit of the backdrop of the environment that led to the strategy that created this opportunity. Then we will talk a little bit about the specifics of the opportunity to the extent that we can, and then in about 15 -odd minutes, I should have finished all of that, and then we will turn it over to you all for specific questions so that we leave enough time for questions.

So with that, just to sort of reset a little bit on the backdrop, I am going to talk about two aspects of the backdrop, one, the macro US healthcare backdrop, and the second, what was IKS doing while this backdrop was playing out, and then we will take it from there. So look, as you have known, as I have said multiple times, this is as difficult a time to be a US healthcare provider as there has ever been. Why do I say that?

Because I think it is one of the only industries in the world where we see that reimbursement when adjusted for inflation is declining. If you look from 2001 to 2024, actually Medicare reimbursement, which becomes the benchmark for driving all other reimbursement for physicians, has grown only nearly 10%, whereas costs have grown by 54%. So when you adjust for inflation, what you will see over these 20 years is that reimbursement has declined.

And this is evidence across all specialties, so it is not that some specialties are alien from this macro trend, which obviously puts a lot of operating pressure on these provider groups, and that is obviously one of the most fundamental pieces of our business model, that the industry that is

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most important in solving for the cost and quality predicament, cost quality and access predicament in healthcare, is also the one under the most economic pressure.

And that is where our platform turns out to be one of the major solutions that can drive sustainability. And obviously in this backdrop of declining reimbursement when adjusted for inflation, one of the things that we observed over the last few years is there are three macro trends that are emerging, that are becoming more and more evident as we talk. So Saransh, if you

can move to the next slide, please.

What are these three macro trends? First, as you know, over the last 15 odd years, there were two or three manifests of physician aggregation that played out in the industry. And the reality is, physician aggregation was not created just to create economic value for the physician, but it was created so that it could solve the fundamental problems in US healthcare.

What were those problems? Cost of care, quality of care, access to care, and hopefully as one would aggregate, it would have an impact on operating cost as well. And so we just took a step back and evaluated, even as 69% of America's physicians today are employed in some shape or form, either by hospital systems or corporations, how has that played out in terms of solving for the fundamental challenge of cost, quality, and care?

And the reality is, the metrics suggest that we haven't necessarily, these physician aggregations haven't been able to dent cost of care in the right direction, i.e. overall cost of care hasn't

necessarily come down. Quality of care hasn't improved dramatically. Access certainly hasn't improved.

And operating costs, which was the most natural improvement that you would see as these guys would aggregate, hasn't really played out. It's been more or less flat. So the motives of physician aggregation haven't really delivered on their promise, even as physician aggregation has played out to the extent of 69%.

Second, the other holy grail that we were all counting on in the US healthcare market to deliver reduction in cost and improvement in quality of care was when reimbursement would shift from quantity-based fee-for-service reimbursement to value-based care where reimbursements get attached to the relative cost and relative quality of care delivered.

And the fact of the matter is, after a lot of huffing and puffing and many of us saying that value-based care is the answer to all the ills in US healthcare, the reality is it hasn't quite played out. Medicare Advantage, which is the most popular manifest of value-based care, has not been able to quite deliver to its promise.

And there are two levels of reducing cost and improving quality in MA, or in two levels of economic success in Medicare Advantage. One is the premium that you get for lives that you take risk on can be optimized through better differential diagnosis capture, which is also known as risk coding or HCC coding. And with the latest regulation from CMS, which is now about 18 months old, I guess, version 28, the reality is there is no more opportunity to create better premiums on risk lives through better risk capture.

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That opportunity is somewhat limited because we're no longer risk-adjusting several diagnosis conditions. And the second opportunity was to build total cost of care infrastructure to improve quality of care and reduce cost of care. And most aggregators didn't really invest in building this total cost of care infrastructure because everybody was just focused on improving the premium and producing MLR from those lives based on that.

And so really, as version 28 kicks in and premiums take a hit to the extent of 12% to 18% once it's fully manifested, I think value-based care isn't really quite playing out the way it was supposed to. And then the third macro trend I think that's really important is, in general, why does healthcare IT exist? It exists to serve the healthcare provider universe. And yet, I have never seen a situation where there is so much dissonance between the value creation that has happened between a community that exists to serve another community. Basically, what I'm saying is healthcare IT has ended up creating a lot more value for itself than for the healthcare providers for whom it actually exists.

You know, and so why do I say that? One, if you look at health

care provider margins for whom

healthcare IT exists to serve them, healthcare provider margins are low single digits, if that. You look at healthcare IT margins, of course, when it's run right, can be well north of 20% -25% EBITDA, right?

We look at valuations about \$45 billion of equity has been invested in healthcare IT over the last three-odd years. And the average multiple at which that has been invested is a 60 PE multiple.

And for when you look at provider entities, again, for whom healthcare IT exists, you know, their multiples are 12-13 times earnings.

So there is a great level of dissonance. And then when you survey the value that healthcare IT has delivered, most buyers in the provider organizations will say that it hasn't quite lived up to its promise. So there are these three macro trends that were playing out as we were thinking about how does IKS continue to position itself for success going forward in what is still a very, very large opportunity from a care enablement platform perspective to delegate the chores so that they can focus on their core.

Now, juxtapose these three macro trends. Saransh, you can move forward to a couple of more slides. With what was going on in IKS, if you can move to the next slide. What IKS has been doing over the last sort of 18 years, if you think about it, we've been humbly building a platform that in the backdrop of those three macro trends that I just described, our platform actually humbly has been chugging away, plugging away at delivering real value to the healthcare provider aggregations.

And that value is captured broadly across four dimensions. We're able to reduce the cost of operations. We're enabling better quality of care because the physician-patient relationship improves and the engagement between the physician and the patient improves. We are constantly expanding capacity and access to care. And we are slowly but surely starting to dent the total cost of care, especially through our value-based care features.

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So when you look at it in the backdrop of several challenges in physician aggregation and healthcare IT, here's a company that is humbly plugging away and actually moving the needle in these very important dimensions of healthcare improvement. And oh, by the way, has been doing all these things in a way where the financial outcomes of these healthcare providers improve very, very dramatically.

Over the years, what we have evidenced, if you go to the next slide, is that at the confluence of implementing all the 16 features of our platform, we're able to improve, give or take, if these features are employed sort of incrementally over a period of time, we're able to improve the economics of our provider clients by, give or take 900 basis points as a percentage of their revenue.

Again, I'm talking about EBITDA expansion of 900 basis points after paying for the cost of IKS's platform. And that is achieved through these three dimensions of we're helping them capture revenue properly on the care that they're delivering. We're helping reduce operating costs and to improve clinician efficiency and delegating all these chores, we're creating time that actually improves clinician productivity and converts that into revenue.

So across these three dimensions and after paying for the platform, they're evidencing somewhere between 850 and 900 bps of value creation. And remember, for an industry that barely even has 850 to 900 bps of operating margin in the first place, that can start to be very attractive.

And then what we've discovered in through platform deals is when our customers are implementing our platform in a more holistic way, where they're not incrementally implementing our features, but are taking the platform as a whole from the get-go, that value creation actually compounds further due to synergies and the compounding effect of one feature

on another to the extent of 1,100 to 1,200 bps.

So, in an industry where healthcare IT is failing to deliver to its promise, while creating a lot of value for itself, where provider margins are under so much pressure, because the relative decline in reimbursement, here is a platform that is fundamentally transforming the economics of these businesses while making an impact on the Holy Grail goals of reducing cost of care, improving quality of care, improving patient engagement and improving access to care.

So, when we include the operating environment, what started to become clear to us is that we should perhaps take a step back and really figure out how to accelerate IKS's proliferation of our platform into these provider aggregations. And to do that, we took another view at, so what has IKS's journey looked like over these last 17 or 18 years? And if you take a step back, we started off as an RCM company back in 2007.

By about 2012 -13, even as we were delivering transformative RCM value on the back office, it became evident to us that we should swim upstream and take on chores, not just from the back office, but all the way from the front office and into the exam room. That's when we created a more comprehensive care enablement platform that started happening in 2013 -14 timeframe.

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So if you call the RCM back office as IKS 1.0, and the emergence of our care enablement platform that delegates all the features, all the chore features from the physician environment, let's say we call that IKS 2.0, at some point over the last couple of years, it started becoming evident to us that we should start taking baby steps towards IKS 3.0 to drive true adoption of the platform and long-term sustainability of healthcare.

And that's when we conceived this idea of IKS 3.0, which is we will still at its core be the platform implementation company that brings the full breadth of this platform to the providers. But now we would venture into start taking ownership, taking baby steps towards ownership of the outcomes that we are promising for providers. Because one of the biggest challenges providers have reported is that healthcare IT vendors come and give us a set of widgets.

Those widgets, then we are left to our devices to create value from those widgets. And in order to help solve that, we said if we also started building transformation capabilities where we don't just leave our customer with the platform and the 16 features, but we participate with them in actually creating value from the features, then suddenly there is an opportunity here to really start becoming a unique transformation enabler for healthcare providers.

And it's with that thought process that we started defining IKS 3.0. Then the question became what would be the manifest of engagement mechanisms by which we would actually co-own the outcomes that we need to create for the providers, for them to realize the value of this 900 to call it 1,100 bps as a percentage of their revenue, which as you can imagine is highly transformative

for an industry like this. And as we came up with trying to figure out what those engagement models are, at a high level, we basically came up with three models.

One, where for absolutely leading provider aggregations within their specialties, we would contemplate structures where if they adopted our entire platform, so think about some of our large clients like a radiology partner, which is probably six or seven times greater than any other radiology aggregation in the world. Or think of another recently announced client like a GI Alliance, where they are the largest, absolutely largest aggregator of GI in the world, certainly in the US, but in the world, and are now owned by Cardinal Health.

So for such market leaders, if they commit to the full platform, not only would we be

ring the

benefits of the platform to their P&L, but we would consider JV type structures that will allow for us to then convert or leverage the success of our platform in their very large install base, to then bring that as a proof point to the remainder of that specialty in the industry, and together co-own some of the outcomes or some of the profits that we would create from that industry.

So that was one of the manifests of the strategy of allowing our very large customers to help us co-create the GI manifest of the Care Enabling Platform or the radiology manifest of the Care Enabling Platform, and then participate in the spoils of that creation as we bring that platform to the radiology industry or the GI industry or what have you, right? So that's one type of strategic upstream alignment with our customers.

The second was the type of alignment that you saw in a deal that we announced a few months ago, which so far seems to be going well, is Palomar Health, where we said to our customers, Inventurus Knowledge Solutions Limited

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we have so much confidence in the value that our platform can create that we will actually underwrite our performance for you. And in some cases like Palomar, we actually advanced some of that underwriting to them.

And of course, if you're going to underwrite some of our performance, we'd also create non-linearity for ourselves where we share a much bigger part of the upsides that we deliver beyond the threshold that we are underwriting. And then last but not the least, which is today's conversation, is in some cases, selectively, we will even consider investing in actually growing the provider aggregation where we are implementing our Care Enabling Platform.

Because if you think about it, if our platform achieves its promise of creating those type of differential economics, alleviating physician administrative burdens, getting physicians and their clinical teams to practice at the top of their training, reinstating the sanctity of the physician-patient relationship and driving better patient engagement, then it actually starts to become a huge differentiator to enable growth of that provider aggregation that our platform is enabling.

And so, in a few cases, we'll selectively pick the right cultural groups where we might make investments from our balance sheet into that provider aggregation, thereby creating a long-term perpetual relationship with them in which we can actually grow that group because now it becomes highly attractive to others in the market. And as we grow, we actually now participate in two pools of economics.

One, the basic pool of economics that gets created in our P&L through the implementation of the platform in that provider group. And two, obviously, the economics that are now getting created in the provider group as it differentially aggregates successfully in a market where aggregations haven't necessarily played out to their potential.

So those are the three models of aligning our outcomes with our customer outcomes and how that could become a differentiated strategy going forward that still proliferates IKS 2.0 and the implementation of a platform, but actually locks that into very long-term relationships and creates another pool of value that we can start to participate in going upstream with our customers. So that was the thought process behind IKS 3.0.

And after having done one or two manifests each of the other constructs, we are now embarking into this relationship with this medical group called Western Washington Medical Group, which is a group in the Pacific Northwest in Washington State, just outside of Seattle. It's a little less than \$100 million in revenue.

They have about 100-odd clinicians in addition to APPs, predominantly fee-for-service, but baby steps in the world of value/risk. And what we have essentially done is we have created an upstream

relationship with them at a pre-money valuation of about \$18.4 million, where IKS is investing about \$17 million in an entity that we are going to co-own called the MSO, which is a Managed Services Organization that will have a perpetual contract to provide all the services that the medical group needs to exist and operate outside of actually delivering care.

So the delivery of care by the physicians is still the ownership of the Western Washington

payroll, finance, all of the clinical support needed through medical assistance, nurses, front office staff in the clinics, the end-to-end infrastructure needed to support these clinics will be owned by the MSO entity. And so that was the structure that we created through which this investment would be made.

And like I was saying, with a pre-money valuation of 18.4 million, we're investing about \$17 million and will own about 48% of this MSO. The Western Washington Medical Group and its physician shareholders, who are the only shareholders of this group, will own about 52%. And the way the revenue of the medical group will be split is that about 37.5% of the revenue from the top goes to the Western Washington Medical Group doctors in order to pay for their compensation for delivering care.

And about 63%, give or take, of the revenue share comes to the MSO. And this 63% is spent largely in all the costs that are needed to be incurred to support the practice, part of which becomes the IKS Care Enablement Platform. So part of this is also a deal between the MSO, which is a newly formed entity in which we've invested, and IKS, an arm's length relation between those two, where IKS's entire Care Enablement Platform is manifested and is provided to the MSO so that the MSO can enable the physicians.

And through our initial modeling, working with the doctors over these last 6 to 12 months, it became evident to us that such a structure enabled by the IKS Care Enablement Platform could produce very attractive growth and economics from that growth for the MSO entity that we would be investors in. So now we're basically through this structure creating two pools of economic value.

One pool of economic value is the traditional margins that IKS gets on its P&L from the implementation of this Care Enablement Platform, which continue to grow as the doctors grow in the medical group, which is where all of this money is being invested, by the way, towards physician growth, physician compensation guarantees.

We are not investing any capital to cash out physicians. One of the things that's very different here in our environment is that a lot of these provider aggregations work in a way where when private equity comes in, they cash out the providers based on a certain valuation upfront, or they income scrape from their future earnings, create a valuation from that income scrape and cash out part of that valuation. We were very clear that in order to drive alignment, no party is getting cashed out.

We are going to invest all of this capital in growing physicians. And growth in physicians will also lead to better concentration for our ability to take risk. And so we will also actually embark on risk-based contracts over a period of time.

There's about 70,000 plus lives, senior lives without counting agents that are available in that geography. So we hope to take on intelligent and thoughtful manifests of value-based care risk that will also create economics in the MSO that we will get to participate in. So that's the fundamental structure that we have created.

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And our simple discipline was one, let's not cash out physicians. Second, have upstream alignment with the physicians.

And then through the MSO entity. The MSO is the perpetual provider of all of the infrastructure needed to provide support to the physicians. The Care Enablement Platform is necessarily manifested as a part of the MSO. So the legacy IKS care enablement platform goal is achieved.

And then both parties together grow this business significantly over a period of time. And our other discipline was that we're obviously traditionally used to a healthy return on equity type of economics from all the investments that we made. And so we were very clear that our Proforma should enable us to create very healthy ROEs from this investment that we are doing in the physician group. And we feel like based on our modeling, we should be able to achieve that.

And as a testament to that possibility, we also wanted to align my remarks by sharing humbly a set of data that talks to some of the strategic investments that we have made over the years, and how those ROEs have played out for us. First of all, first and foremost, each and every one of these investments was strategic in nature, just like this Western Washington Medical Group investment is strategic in nature. Each of these were, why were they strategic? Because they allowed us to take a feature where we hadn't yet developed our technology, white label that

feature from the company that we were investing in and embedded in our platform so that our platform remains complete, even as we build a feature out on our own. And there's been three such investments that we've made over the years. One is Lightbeam Health, which is a population health technology.

The other is Sift Medical, where it was a denial prevention and prioritization technology. And the third is the more popular company Abridge, which is a clinical documentation ambient AI technology. All three of these, of these three, we've already replaced two of these entirely with our own technology that we've built.

In Lightbeam's case, we have not yet built that full technology, and we're in the process of building that technology. But nevertheless, we wanted to at least put out for you all how those three investments have performed. And these investments give us the humble courage of conviction that as we play out this Western Washington thesis, you know, and by the way, I just want to clarify, the ROE numbers in these are certainly not indicative of the type of ROE we are expecting from Western Washington.

These are just to indicate that so far our track record has been half decent when we've made strategic investments, first and foremost, as it relates to using those technologies in our platform, and then secondarily, also the financial value that they've created over the years.

So again, I'll wrap up quickly by saying, this is a very exciting and strategic pivot for IKS where we are now solving for a real issue in the industry that is stifling physician aggregation by partnering with them upstream in creating a differential set of economics to not just by providing our platform to them, but actually owning the change that is needed from the platform to produce the outcome that it can deliver.

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You know, I think this pivot makes us even more attractive and even more stickier in the value that we can create for our provider aggregations going forward. And it really makes us now a strategic transformation partner in the industry from being a potential enabler of strategic transformations.

And so again, very excited to report this development and really looking forward to continuing to talk about how this performs over a period of time. With that, I'll pause for a second and turn it over to people that have questions.

Moderator: Thank you very much.

Management: Before we open it up for questions, I just have this one request that we keep the questions strictly related to the deal. We will not be

able to take any questions regarding the prior quarter and we will talk to you about it when we report our results. We can open it up for questions now.

Moderator: Certainly. Thank you very much. We will now begin the question and answer session. Our first question comes from the line of Ruchi Mukhija from ICICI Securities. Please go ahead.

Ruchi Mukhija: Congratulations on the JV. I have two questions. So IKS has been a capital prudent entity as it has evolved over the years. Now, over the last nine months with the Palomar deal, \$16.5 billion, \$17 million here, we have put capital to use. Sir can you talk us through how these transactions changes the capital construct for the business? That's question number one.

Second, what kind of payback period and profitability do we have or do we expect from the JV?

Sachin Gupta: Okay. Thank you, Ruchi, for the question. I appreciate it. So our core has always been conservative from a capital deployment perspective. And I think relative to the type of capital deployments that are happening in healthcare IT today, like I said \$45 billion over the last four years, you will find that we will continue to be fairly capital conservative over a period of time. A couple of principles that I tend to use as we think about capital is you will not find us, I won't say ever, but very easily resorting to a structure where we are deploying capital in a manner where it's equity dilutive to our shareholders. So that's one of our principles is we don't like to get ourselves in a position where we are leveraging shareholder equity to generate capital that we then want to deploy to bulletproof our growth. So that's one.

Second, I think in terms of even debt as an instrument, as you've seen, we've been very conservative even on our debt, even for the Aquity deal. At its peak, we had about \$117 million of net debt when we had taken that on. As you've already seen from the last quarter's numbers, that debt is down to nearly half.

And if you look at just our last quarter's EBITDA as a percentage of that EBITDA, we're hovering somewhere around 0.5, 0.6 times EBITDA. So I think we generally tend to be relatively conservative. We will take strategic opportunities like these where, for example, think about it, this creates a deal structure where we are locked in into perpetuity, into a position aggregation that is going to be growing fairly rapidly.

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So it's a 30 -year contract between the MSO and the physician group to start with, with five -year renewable terms post the 30 years. So when you think about the longevity and the predictability of revenue and margins that this creates from our platform, it actually makes the investment a little bit of a no -brainer.

And that is a good detour to the second question you had is what are our expectations on returns from this? That was also one of the reasons why we put out some of the ROE metrics in the past. But our general thinking is that, one these constructs should generate greater growth and margins in our traditional care enablement platform business.

And in addition to enabling that, the economics that it creates purely on the investment should always be in the range of our historical ROE that we've delivered. And that number historically has been, in fact, even north of 30%. But my thinking is that we definitely want those ROEs to be North of 20%. So that's generally our guiding principle. Don't get over -leveraged, don't dilute equity. Invest in constructs that provide more predictability, faster growth to our core business at the traditional margins. And the ROEs from the investments are comparable to sort of what the overall business has delivered in the past.

Ruchi Mukhija: Got it. Sachin, also, if you could talk about the strategic continuity. We saw like two such large transactions with evolving capital structure

in nine months. How would you think about the pace

of this kind of transaction? Would you now take a pause and consummate these two large ones before we prepare or chase more such opportunities in the market?

Sachin Gupta: You know, Ruchi, the way to think about it is this. These opportunities are generally not easy to create and they take a lot of time to construct. So we obviously created these two opportunities pretty much over the last 15 months or 18 months and now they've come to fruition.

The simple answer is there are a whole bunch of criteria that we go through before we feel like we want to do one of these deals. As long as those criteria are being met, we will be open to such structures because I think the value creation is dramatic. Our first lens will be, can we execute to create the value.

That is always the first lens that we put these structures through. So my first lens will still be, if we do another deal, can we execute the two that we've already executed? Some of the other large customers that we have signed up over the last six to nine months that we've announced publicly and others and still take on one or two more.

If we feel that we can execute successfully to them, then we'll put it through the other lenses of, are we still staying prudent on our capital structure? We're not diluting equity. We're not taking leverage that over leverages us. And our definition of leverage is very traditional relative to perhaps some other equations that I've seen.

And so first, can we execute? Second, does it still stay within the constraints of the availability of capital that we might have? And third, does it still promise to generate the type of ROEs that we are comfortable with? And if a deal meets all of those three criteria, then obviously we'll selectively play out on those. So I don't want to give future guidance around it, but we'll continue to be strategic in which ones we take on over a period of time.

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Ruchi Mukhija: And last bit here, it's a new, I would say, area of intervention with MSOs. Now, could you help us what kind of protection or hedge we have in case things doesn't work out the way we planned?

Sachin Gupta: Yes, that's a good question. Actually, the beauty of this perhaps is that it's not a new area. Think about what we've been doing. We have been enabling other MSOs with our platform to deliver the value. The challenge that a lot of these customers have had is they often don't have the capabilities in -house to effectively leverage the value, effectively create the value from the platform.

So we're just going one step up and saying, now we will not just provide the platform, but we'll actually help you implement the platform effectively from a change management perspective. So it's actually not necessarily a brand new area that we are venturing into. It's really an upstream extension of what we were already doing. And in the process, participating in the economic value that the platform imminently creates.

So I think, of course, there will be some new unknowns that will come our way and we will learn from those. But I wouldn't say that this is a whole new area that we are necessarily venturing into. It's really a more upstream pivot to participate in the upstream value and align our outcomes with our customers.

Ruchi Mukhija: Thank you and all the best for the JV.

Moderator: Thank you. Our next question is from the line of Srinath V from Bellwether Capital. Please go

ahead.

Srinath V: Hi Sachin. Assuming 60 or 70 million top line for the MSO, what kind of revenues would IKS make for our care enablement platform that we provide to the MSO, just to get our understanding of the size of the opportunity, not as a shareholder, but as a platform provider?

Sachin Gupta: Yes. So thank you for the question first of all. I think while we won't be able to re-

veal the

specifics of the numbers. One way to think about it is at the full manifest of the platform from any customer, like multi-specialty medical group customer, the IKS revenues tend to be somewhere between the say 10% to 12% range of the customer's revenue.

That's the way to calibrate on the customer's total top line revenue. In this case, the top line revenue is whatever it is close to 100 million. So that's the way to think about it. Now, why a range of 10% to 12%? Because it also depends on the specialty mix, the nuances of some of the features being implemented or not being implemented. As we go from upside only risk to full risk, more features get implemented.

But generally, I would say the full wallet potential is somewhere between 10% to 12% of the customer's revenue for our platform, for any customer. Obviously, we probably can't reveal the specifics of the numbers for this. But suffice to say that we structured the deal in a way where the full platform is manifest and obviously at a pricing between IKS and the MSO that allows for the realization of traditional IKS margins on P&L.

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Srinath V: Perfect, perfect. Also, would it be fair to assume since we own the MSO that all the 90 physicians of Western Washington would use our whole suite of products from get-go? Because in our previous relationships, there is a scale-up period as we impress upon the physicians to use our suite of products. Would it be fair to assume that since we own the MSO, that shouldn't be the case?

Sachin Gupta: That is a fair assumption. In fact, that's a very important construct for the deal, right? And so all applicable features, and I say applicable features because one of the features of the 16 for value-based care, which entails success in full risk, that feature might take some time to implement because they don't have any full risk contracts in value-based care yet.

So as we build out full risk contracts, that feature might play out over a period of time. But the idea here is as fast as we can implement the entire platform, obviously, the doctors will be using that full platform.

Srinath V: Perfect. And last, to get a broader understanding, our pricing has largely been outcome-based.

And therefore, in this particular construct, as we own the MSO, so if outcomes are significantly better than what was agreed upon, would we be entitled to some bonus or some revenue structures as a platform provider, or that would largely only flow as a shareholder of the MSO? How would that work out if we actually deliver better than expected?

Sachin Gupta: Yes. So, as you said, a large part of those upside economics would flow into the MSO where we own 48% and potentially might end up owning more in the future. A large part of those upside economics will flow into the MSO, but there'll still be some upside economics being captured directly into IKS.

Srinath V: Got it. And that would get dividend out to us, or how do we kind of get to those cash flows just to get a broader understanding?

Sachin Gupta: Yes, that would be the typical mechanism where the excess earnings of the MSO are, after factoring in for some reserves that you want to build in the MSO, get dividend out over the years. Because obviously, that creates upside for the physician shareholders as well.

Srinath V: Perfect, perfect. Just last one on Palomar, especially from the PPT. You've said that if the complete suite of platform is put in, 12% of business is a kind of upside potential for customers.

So, as we've seen this being implemented in Palomar, are you seeing these kinds of benefits for the physician group there, given that probably we've been implementing it for 6 months or 3 months? I just wanted your broad views on that. And thank you.

Sachin Gupta: At Palomar, we are in the process of still implementing certain features. A

large part has been

implemented. Some are still being implemented. But from the features that have been implemented, I can tell you that the performance so far is turning out to be better than we had modeled. So, we continue to remain optimistic about the value creation opportunity at Palomar.

Srinath V: Thank you. I'll get back to the queue.

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Moderator: Thank you. Our next question comes from the line of Srivathsan from Avendus Spark. Please go ahead.

Srivathsan: Yes, hi. I had two questions. One is as a part of this, you would kind of expand into areas which so far the platform is not taking care of. So, any specific areas you would do, any specific areas you would use a plug-and-play partner, just want to get your thoughts on that. Second, an extension of this is the incremental development or capabilities you build, you are free to cross sell, upsell at IKS level, or is it has to be sold only from the MSO level? I have a follow-up on this and then I'll come to it.

Sachin Gupta: Yes. I'm not sure I totally understood the second part of your question. So, maybe I'll request you to repeat it. But in the first part of your question, yes, I mean, right, so far, we haven't necessarily contemplated any additional features that would need to be implemented outside of our platform.

And so, we don't need -- we haven't yet identified the need of any subcontracting or other technologies that need to be white labelled into the platform. But as the need arises, we obviously have the flexibility of doing that as operators of the MSO, and as obviously owners of the IKS platform, we will do that on a need based basis.

But right now, our first step is really to manifest the full features of the platform and generate the pretty large cost savings and revenue upsides that we have modeled in our proforma. That'll be our first mention. But yes, we have the flexibility where needed to use any subcontracted technologies or services that will be necessary. But we haven't quite envisaged any yet. Sorry, can you repeat the second part of your question?

Srivathsan: No, the second question is only contingent on the fact that if you're expanding the product offering the service suite, then can you resell it to others? So, if it's not as much expansion, then the scope for or the need for resell also...

Sachin Gupta: No. So actually, you know, obviously, the other opportunity that this creates is that it becomes a live lab for the continued evolution of our platform, right? And so, irrespective of this deal, we were working on some other features, which we constantly are. And with this deal, part of the expectation that the doctors have and the part of the expectation that the MSO has from the doctors is they will actually engage in co-creation of additional features that they will benefit from and that then IKS can include in its care enablement platform in the future.

So, and also keep in mind that this medical group operates on Epic. And so, this will also give us the opportunity to dive deeper into how to integrate with the Epic EHR more effectively across features and build new features that are further complementary to what Epic already does, right?

So, yes, there will be some co-development that will be emerging from this. And, you know, obviously, we'll be the benefactors of this. So, another byproduct benefit of this deal is that now we get a live lab to manifest our whole platform and continue to develop features in a live environment, which has always been the ethos of IKS.

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Srivathsan: Sure. Just one last question. Is there any tailed risk that you see in this whole thing, right? And pardon my ignorance. Is there any insurance, given health care insurance-related cost that this MSO takes, which can sink the whole

thing if there is a class action suit against the hospital or the MSO, etcetera? I just want to understand any tailed risks that you could conceive of.

Sachin Gupta: That's one of the reasons why we were very careful in structuring this, right? We structured this in a manner where we did not take on any clinical decision-making responsibilities in the MSO. It's an administrative support entity, call it a special purpose vehicle or something that has gotten created, and that's where we want equity.

The medical group that is actually responsible for care delivery and clinical decision-making around care delivery, obviously, is the party that would be liable in situations like that. And they obviously carry the medical malpractice liability insurance to cover themselves appropriately from that perspective. So, I think based on the way we've structured this, I don't necessarily foresee any fundamental risk around that.

Srivathsan: Sure. One last question. Apologies. Going by the numbers broadly, right, about one-fourth of the business of the MSO would be an area where you have extreme expertise, but the residual three-fourths is an area where so far at least you don't have very explicit expertise. Just wanted to get your thoughts. Is that something you would want to build up or it will be the existing team that will run and it's not an area where IKS will be actively involved in?

Sachin Gupta: Yes, look, I mean, this is a fully functioning operating entity today, right? It's just that we are carving out those functions into the MSO. And so, they today have a fully functional operating infrastructure that has been supporting these practices for the last 30 years and that infrastructure

works well.

So it would not be our intent to go and disrupt that infrastructure, but it has been agreed upon with the medical group leadership, part of which, by the way, the way this deal is structured is all the administrative functions leadership that used to run the medical group in absence of this entity will actually now transition to this entity as its employees and leaders, right?

So obviously, those people are fully capable of taking this forward, but we have also agreed with them that as IKS comes in, to the extent that we find opportunities for efficiency in some of those traditional functions, be it IT, be it payroll, be it back office, front office type stuff, we'll together embark on realizing those efficiencies. But I think it is important to realize that those functions work well today, they're just getting transitioned from the medical group into the MSO.

Srivathsan: Thanks a lot, Sachin. All the best.

Sachin Gupta: Thank you.

Moderator: Thank you. Our next question is from the line of Nilesh Jain from Astute Investment. Please go ahead.

Nilesh Jain: Hi. Thank you for the opportunity and congratulations on this deal.. My first question is, I wanted to understand the structure much better, given they were already existing clients and in last

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quarter, we announced that we are selling them the entire breadth of platform. And now we are entering a JV where we are selling the same platform.

So how does the entire structure differ in terms of opportunity when we are directly selling it and or selling it through a platform through a JV? If you can help me understand it much better.

Yeah.

Sachin Gupta: Yeah. I mean, I think in the end, the fundamental difference is that one, as a part of this, they are already committed to all the features of the platform that are applicable, number one. And number two, the only difference is earlier, there was one set of economics that we would get from it, which is the economics we get from the implementation of the platform, the revenue that IKS gets from it and the margin that IKS creates in it.

Now, we get to participate in a second set of economi

cs, which are around the value that the

platform creates on their P&L. And we have the opportunity to participate in about 62.4% or whatever that number is of that value through the MSO. So I think what it's essentially doing is one, it is for want of a better term, mandating the use of the entire platform.

Second, that capital is helping grow the medical group in a manner that it would not have traditionally grown at a pace that it would not have traditionally grown. And third, the economics that are getting created from the implementation of the platform on their P&L and the growth that is going to happen on their P&L, we get to participate in a part of those economics through the MSO structure. Does that help, Nilesh?

Nilesh Jain: It helps, it helps. Just to follow -up to that, given you've entered into a transaction with them, do you see this as a trend being developed where more physician groups would be open to directly enter into a partnership with you all or you think it's going to take time?

Sachin Gupta: Our instinct, the reason why we're doing this is essentially that we believe that this creates a model that is very attractive for other physician groups to either participate in a structure like this where they decide to become clients of this MSO that we're launching, which could probably be in that geography. But I think it creates a structure that will get many, many physician groups very excited about the prospect of what this model can create.

So yeah, I mean, one of the big reasons we're doing it is because our belief is that this will start to create a model for sustainability in US health care while achieving that quadruple aim and creating a set of economics by which physician aggregation can be successful. So yes, the whole hope around this is that this model creates a structure that is very attractive over a period of time.

Nilesh Jain: Okay. Thank you so much and I wish you all the best.

Sachin Gupta: Thank you.

Moderator: Thank you. The next question is from the line of Sandeep Kothari from Eastlane Capital. Please go ahead.

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Sandeep Kothari: Yeah, hi, Sachin. Just a question on how to think about the capital deployment going forward. Is it essential to deploy capital to be able to sell the entire suite of platform or now you have some reference clients and selling the platform as it is to the rest of the physician group so your customer base of 500 becomes easier or this capital deployment is case in point and wherever

you see opportunities for further upside, how to think about the whole thing?

Sachin Gupta: I think the answer is both. I think we should see more deals as a proliferation of the full platform as a result of the proof that will be generated from deals like Palomar, deals like Western Washington, deals like GIA, et cetera, right? So obviously we should see more momentum and I actually believe that we might see more momentum even from the perspective of this new aggregation model that we are creating with Western Washington Medical Group.

I will just say that we will continue to be more -- and continue to be careful about one, our ability to execute at the pace at which these opportunities are coming. Second, about our ability to do these in a manner where we do not breach our general conservative constraints of use of capital that we have kept for ourselves and where we do that, we are very confident of generating ROEs that are attractive over a period of time.

So I think that is as much as I can say because I do think this should be -- these type of deals will create more activity in both of these streams, pure arm's length organic transactions for full platform and potentially a lot of deal flow in these more aligned type of constructs.

My belief is that our first play more and more will always be guaranteeing outcomes versus pa

rticipating upstream because when you participate upstream like we are doing, we also have to think of things like, is there a culture fit between the physician leadership of the group and us? Will they be truly committed to operating as efficiently as we want to operate for an MSO type entity? So there are other nuances that come into a deal structure like this.

So I think these deal structures, even though deal flow might be significant, we will be more selective about these types of deal structures. But anything that we do will always go through the lens of can we execute effectively and are we still being conservative about the source of capital and the return on capital?

Sandeep Kothari: Understood. And just to follow -up to that, what do you think about the two parts of the businesses, selling the platform as a service provider versus participating like this? I know you mentioned execution risk, which you will keep in mind, but how different it is, how about the management bandwidth and how the whole culture of your organization and how you think about executing on both separate pieces sort of stand?

Sachin Gupta: Yeah. So just to be clear, the way we think about it is that our traditional sales engine, which we are continuing to grow, has the mandate to sell the platform at an arm's length before, right? So that's all that the traditional sales engine will focus on. They are not -- they don't have the mandate to go and try and create upstream alignment deals, certainly not of this nature where we'll be making investments.

Sometimes we will be able to sell deals where we are guaranteeing some outcomes. These type of deals where we're going all the way upstream and aligning more strategically and making Inventurus Knowledge Solutions Limited

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investments are generally created by the management themselves, i.e. me, Joe, our Chief Growth Officer, Pete, maybe our CEO, a couple of other people.

And so that's the way to think about it. Our traditional sales engine is still driving the traditional motion of full platform sales and/or land and expand platform developments as an executive leadership team where we selectively look at more strategic constructs. And that will naturally also then get reflected in the number of deals getting done in each of the two spheres.

Sandeep Kothari: Understood. Thank you.

Moderator: Thank you. The next question is from the line of Ruchi Mukhija from ICICI Securities. Please go ahead.

Ruchi Mukhija: Hi, Sachin. Here, why IKS don't have majority ownership or majority stake?

Sachin Gupta: So it's a good question, Ruchi. And I think, this is just a moment in time. I would not rule out the possibility of IKS eventually having a majority stake. Just know that we structured the governance of this deal in a way where we have protective rights for all what we call important decisions, right?

So even though we don't own majority right now for anything that is materially consequential, we have those protective rights as a 48% shareholder.

And the Board has been structured in a way where we have three Board members. They have three Board members. Western Washington Group does. And then we have an independent Board member as the seventh. So I think even though we don't have majority, I think we've secured ourselves in a way where there will be for any material consequential decisions, we are able to at least have a say.

Ruchi Mukhija: And the capital that we have infused in this entity, what would be immediate use as you see?

Sachin Gupta: The immediate use is physician growth. So we obviously want to increase their primary care physician base because that primary care physician base then becomes more of a feeder to the specialists in a fee -for-service model and then actually allows us to take on more value -based

care constructs or contracts from Medicare Advantage and the like.
So primar

y use of capital is in physician growth. And then associated with physician growth, there will always be capex associated with expanding clinic facilities, etc. So those are the two primary uses of the capital infusion, which then obviously gives growth to the top line and the bottom line resulting from the top line.

Ruchi Mukhija: Just to clarify, the capital will stay in JV or it goes to Western Washington Medical Group?

Sachin Gupta: So technically it has to go to Western Washington Medical Group, because that's where the physician recruiting and the clinic expansion happens, but it is routed through the MSO.

Ruchi Mukhija: Okay, understood. Thank you.

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Moderator: Thank you. We will now take our last question from the line of Srinath V from Bellwether Capital. Please go ahead.

Srinath V: Hi Sachin. Any timelines on the scale up of the care enablement platform to the MSO? Would it be like within the next month or so or there's a process of the implementation?

Sachin Gupta: No, typically these implementations, the way to think about the full platform implementations is somewhere between 3 to 6 months. That's the way to think about how long the full platform implementation takes. We obviously always push for closer to 3 months, but in some cases, depending on what is happening in the physician environment, it can be up to 6 months. So just not necessarily just for this deal, but for all full platform deals, that is how one should peg the implementation timeline.

Srinath V: Perfect. While I understand the MSO is, has current working operations, would like to understand if you have a particular action plan in mind for the cost lines outside of the care enablement, you know, in maybe some of the administrative functions of HR and Finance, to get offshore or what is your broader thought on the cost lines outside the care enablement platform?

Sachin Gupta: So it's the same as anything else. It's to be as efficient as we possibly can. And whenever we can use technology, wherever we can use globalization to drive efficiencies, we will absolutely do that. And that is why I was saying that for these type of deal s, it's very important that there is a meeting of mindsets with the physician leadership as much as there is a financially compelling piece.

And here we feel like we are fully aligned on what is the best trade -off between efficiency and still being able to deliver a world -class customer experience first and foremost for our patients, and then certainly for our physicians and the clinicians involved. And so there'll be obviously some decisions we'll be making over a period of time that attempt to achieve that trade -off.

Moderator: Thank you. I would now like to hand the call over to the management for any closing remarks.

Sachin Gupta: Yes, so I would just say that, again, a very exciting step in our journey of enabling better, safer, more efficient care. We will continue to talk about it. And I just, on an ending note, I'll repeat the five strategic themes that we are executing on for o ur business. I had mentioned them in the last earnings call.

I will continue to reiterate those on every call so that we're able to put in perspective what we're executing on. First, we will continue to take our platform to becoming more of AI -led and AI -native so that we can go from the model that we'd started with, which was human -led, tech -in-the-loop, to tech -led, human -in-the-loop, to eventually becoming autonomous through technology and some of the features.

We continue to stay focused on the last phases of effectively driving the value from the acuity acquisition, both from a cross -sell perspective and getting the margins to an optimum place. We continue to solve the predicament of still wanting to be number one, two, or three in each of the Inventurus Knowledge Solutions Limi

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16 features of our platform, even as we're emerging as the only company that has the full manifest of the platform and the breadth of the platform.

We're continuing to focus on a differentiated go -to-market strategy for different segments of the market. For example, continue to proliferate, land, and expand in the large health system segment. Of course, there'll be some platform deals that come about there as well. And really, then, the go -to-market on the other segments, the private equity -owned segment and the independents, will continue to be full platform.

And then last but not the least, we will continue to take steps towards becoming more and more of an outcomes company enabled by our platform versus a company that just provides the platform and stops there. Because I actually believe that that will create over a period of time a very differentiated moat if you can execute successfully to it.

So today's news was more in the fifth dimension of these five strategic pillars that we are executing on in a business. And I look forward to continuing to report our progress and talking to you all again soon. Thank you for your time.

Moderator: Thank you. On behalf of IKS, that concludes this conference. Thank you all for joining us, you may now disconnect your lines.

Call: Aug 2025

August 6 , 2025

BSE Limited
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BSE Scrip Code: 544309
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The Listing Department
Exchange Plaza, Plot No. C/1, G Block,
Bandra Kurla Complex
Bandra (East), Mumbai 400051
Maharashtra, India

NSE Symbol: IKS
Dear Sir/Ma'am,

Sub: Transcript of the Q 1 FY 25-26 Earnings Conference Call

Pursuant to Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, please find enclosed the transcript of the earnings conference call held on Friday , August 1, 2025 for the Company's financial results for the quarter ended June 30, 202 5.

The said transcript has also been uploaded on the Company's website and can be accessed through the following link:

<https://ikshealth.com/ir/2026/q1/transcript -q1.pdf>

This is for your information and records.

Thanking you.

Yours sincerely,
For Inventurus Knowledge Solutions Limited

Sameer Chavan
Company Secretary and Compliance Officer
Membership No. F7211
Encl: As above

"IKS Health Q1 FY-26 Earnings Conference Call"

August 01, 2025

MANAGEMENT : M R . S ACHIN G UPTA – F OUNDER & G LOBAL CEO

M S . N ITHYA B ALASUBRAMANIAN – W HOLE -T IME D IRECTOR &

C HIEF F INANCIAL O FFICER

M R . S ARANSH M UNDR A – VP, I NVESTOR R ELATIONS

M ODERATORS : M S . R UCHI M UKHIJA – ICICI S ECURITIES

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Moderator: Ladies and gentlemen, good day and welcome to IKS Health Q1 FY26 Earnings Conference Call hosted by ICICI Securities.

As a reminder , all participants' lines will be in the listen-only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing '*' then '0' on your touchtone phone. Please note that this conference is being recorded.

I now hand the conference over to Ms. Ruchi Mukhija from ICICI Securities. Thank you and over to you, ma'am.

Ruchi Makhija: Good morning, ladies and gentlemen. Thank you for joining us today on Q1 FY26 Earnings Call of IKS Health.

On behalf of ICICI Securities, I would like to thank Management of IKS Health for giving us the opportunity to host this Earnings Call.

Today, we have with us Mr. Sachin Gupta – Founder and Global CEO, Ms. Nithya Balasubramanian – Whole Time Director & CFO and Mr. Saransh Mundra – VP, Investor Relations.

I turn it over to Saransh for the safe harbour statement and to take the proceedings forward.

Thank you and over to you, Saransh.

Saransh Mundra: Thank you, Ruchi. Welcome, everyone. Thanks for joining the call early in the morning. We will start with a disclaimer and then I will hand it over to Sachin. As part of our prepared remarks, we may make certain statements that are forward-looking and may involve uncertainty. We don't take any responsibility to update those statements, and your discretion is warranted while making any investment decisions. Over to you, Sachin.

Sachin Gupta: Great. Thank you, Saransh. Thank you, Ruchi. Just a minor correction, it's Nithya Balasubramanian, not Subramanian. Anyway, good morning, everyone and welcome to our earnings call for the 1st Quarter of Fiscal '26. This is our third earnings call since it went public in December last year. So excited to talk to everybody again.

On the call today, I will do probably three things:

One, just because there is always some participants that are relatively new, I will start off with giving a very quick high-level overview of the business, talk specifically about our execution this past quarter as it relates to those five strategic vectors along which we want to measure our progress and along which I think our discourse is most relevant because I think those vectors drive the financial outcomes that all of us are interested in. And then, of course, we will dive into our points of high-level commentary on our financial performance for the fiscal Q1, after

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which I will turn it over to Nithya to talk through some of the details, and then we will open it up for questions.

But before I dive into the high-level overview, let me kick the call off:

Obviously, all of you have seen the release that came out last night. Hopefully, you have studied the numbers. One of the things that I think some of you saw in the release that perhaps might have caused some confusion, and I want to just clarify it up front, was the change in director status in the Indian listed entity. And I just want to clarify; there's nothing really that's changing fundamentally. This is barely a technical change associated with the fact that I live in the US and hence my employment pretty much needs to reflect with the US entity and as a result of which I can't really be employed by the Indian mother ship. And so, as a result of that, I am no longer a full-time director and because the company obviously needs a whole-time director, Nithya is being made the whole-time director. I continue to be the group global CEO and continue to be at the helm and at your service for as long as you guys desire. So I just wanted to clarify that up front, because it felt like it was causing concerns or ripples. We can certainly take any other questions around this when we get to the questions.

Now to just jump into the business:

Obviously, you have seen the numbers, a solid quarter of performance. But before I go into the quarter, just a quick high-level recap of the business model. So, we are perhaps the most comprehensive care enablement platform that delegates chore tasks from large physician groups in the US so that they can focus on their core of patient care and patient experience. We do so in a manner where we have built proprietary technology to eliminate as much of these tasks as possible. And because technology often doesn't entirely eliminate the task, our technology is coupled with our global human capital, which becomes a very important human in the loop, also from a safety perspective, to deliver these tasks. We do so in a manner where not only is it quickly transformative for the patient experience and the quality of care, but also, it's financially transformative for the practice, which as you know, US healthcare is under tremendous financial stress. And so, through these 16-odd tasks or actions that we delegate for these physician groups in the US, we are often able to, at the full manifest of the platform, deliver between 800 to 1,000 bps of additional economic value for them after paying for the platform. So, it's financially transformative in addition to being clinically accretive for them as well. We have over the last 18 years grown our business to about 150,000 physicians across the US. Remember, there's about 900,000 odd in total. So, 150,000 physicians in the US use some manifest of our platform or the other. And those 150,000 are spread across now about 650 organizations that these physicians are a part of. So 150,000 physicians, 650 organizations. Remember, this number was close to 800 in October of 2023 when we completed the AQuity acquisition. We have then said that our strategic business model is always to have the consolidators as our clients, which are the large groups that are buying the other groups. And so, with AQuity, we got a lot of good stuff.

But one of the things we got was a long tail of small physician groups. And strategically, intentionally, we are continuing to prune that tail.

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We are now down from a combined customer base of (+800) in October '23 to about 650. And as I have stated in the past, a sort of steady state will probably end up closer to 500. So that's just one thing to keep in mind, (+650) customers, 150,000 providers.

Our business model, because of its unique nature by which we actually delegate all of these tasks that are chores but are very important for the sustenance of their practice. By that very nature, business model becomes annuity also. So, 95% of our revenue is repeat revenue from the same customers. And in addition to that, it becomes really, really sticky. So, it's a very sticky annuity business model. And the human element of what we do now comprises about 12,368 FTEs across the US and India. Again, remember this number in Q1 of FY25 was actually 13,277. So, on a year-on-year basis, our global headcount is down about 7%, which is again something that we will talk a little bit more about when we discuss our strategic vectors of execution. Obviously, because we are very intentional about working with consolidators, our top 10 and top 5 clients drive a large part of our revenue and our growth. That is very intentional. The good news is that our top 10 and top 5 clients have great vintage and are actually growing. So that always is a good sign of the progress in the business.

Also, as you probably already know, we operate in a very large TAM. Our TAM traditionally was about \$225 billion just in the physician market in the US. Starting about a couple of quarters ago, as I have mentioned, we have also pivoted our business model based on the pool that we saw in the market into the acute or hospital RCM space. And that has expanded the TAM to more than \$260 billion, of which really only about \$34-\$35 billion has been delegated. So, it's still largely, automatically, an insourced market. And yet, I think with the pressures that the US healthcare system is facing, cost-quality access, it's becoming imminent that this outsourced TAM of \$34-\$35 billion, is actually going to grow faster than the overall market. So, the market's growing at 8%, but the outsourced market is growing at 12%. And this is important, again, because I keep saying, if we are growing faster than 12% on a constant currency basis, we are gaining market share. If we are growing slower than 12%, we are losing market share. So that's an important indicator and something to always watch out for. Now, if that 12% further accelerates because of the pressure in US healthcare, we will certainly keep you updated and that should also reflect in our growth. And our idea, obviously, is to grow significantly faster than the outsourced TAM over a period of time.

One of the other dynamics that it creates, because there's such a large TAM, which also then leads to our competitive positioning, is that because these 16-odd tasks have a TAM of about \$260 billion, the reality is that the TAMs of several of these tasks individually is quite large.

For example, clinical documentation, which is reflected in our scribble product, could be a \$30 billion TAM. The four or five features that comprise RCM is a \$100+ billion TAM. And so, as a result, there are several genres of competition that we have to deal with. One of those is actually what I call these point solution companies that are focused

on one or two or five of

these 16 tasks because the relative TAMs of each of these tasks itself is so large that you can build a very large company just focused on them. Now, our thesis always was that as the market matures and buying behavior matures, buyers will realize that they can't be in the

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business of buying many point solutions and integrating them, carrying the burden of managing tens of vendors across these point solutions. And so, over a period of time, they realize that the value of the whole is much greater than the sum of the individual parts. And that is a better strategy. And we see that the buying behavior will slowly but surely migrate towards the platform buying behavior. And so, what that means is, and we have set our strategic competitor advantage to that, which is we are the only company that has the full breadth of the platform for those buyers that are willing to buy the platform thesis today. But because there are several buyers, especially in the large health system market, that are still interested in a point solution best of breed approach, the reality is for those who still need to be one of the top two or three vendor partners in each of these point solutions. And that creates a very important strategic predicament for me where we have to execute as a leading point solution vendor across 16 point solutions, even as there are perhaps one of the very few companies that are building out the entire platform. But that really continues to be a competitive position.

So, with that, let's just quickly touch upon the five strategic vectors of execution that we are consistently focused on, I think should shape the discourse that we should have on a continued basis, unless something changes, in which case I will let you. So, the first of the five is the fact

that we are moving rapidly to this AI-native, agentic platform manifest for doing all these tasks. And I am happy to report that there's been a lot of progress on this strategic vector over this last couple of quarters, but specifically last quarter. We launched Scribble Now, which is our fully ambient, autonomous clinical documentation solution. And with that, I think IKS now has the most comprehensive set of variants that meet physicians wherever they are in their journey of choosing the right clinical documentation solution for their needs. We are also in the process of actually launching a very unique multi-variant Scribble option, which will allow each physician user to choose different variants of Scribble for different type of visits. So, there will be some easy patient visits where the fully autonomous ambient listening solution without supervision from the human produced instantly on a synchronous basis is best for them. And then there will be other situations where there will be more complex encounters, where there is a need for more comprehensive documentation, where the human in the loop or the physician in the loop becomes very relevant. And the ability to pick and choose different variants of clinical documentation solutions for various parts of their patient panels, we suspect could be a very exciting differentiator over a period of time. Just based on the utilization trends that we have seen across the country from ambient, autonomous-only AI-driven solutions where a lot of doctors perhaps have bought them, but the utilization of those solutions as a percentage of their overall visits is still in the 50s and 60% odd out there. So, I think that should be very exciting.

Also, happy to report that we have actually been able to build out our autonomous medical coding technology, Gen A

I-led for two medical specialties, which we now continue to optimize. And now, of course, we are expanding to several other medical specialties. Again, remember, just like clinical documentation, there are companies out there that all they do is medical coding. So, this is to my point of wanting to be number one or two or three in each of

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the point solutions, even as we are trying to build the full platform out. And then, of course, we have done significant progress in the denial prediction and prevention space using our AI technologies that we have built. And we have also launched an AI-led patient engagement hub that actually drives behavioural economic inputs-driven patient engagement to help shape various aspects of patient behavior, including reducing no-shows for visits that help improve access to care, including nudging patients differentially around adoption of our adherence to various clinical protocols, etc. So significant progress in dramatically moving from sort of that human-led tech in the loop to not only tech-led human-in-the-loop, but with AI now moving towards a fully autonomous set of features which don't need a human-in-the-loop at all over a period of time. That's the first vector of our execution this quarter.

The second one is obviously the big AQuity acquisition that we did in October 2023. Within that vector, there were three dimensions of execution that we had to accomplish to be successful with the acquisition. The first was actually the people-process technology and the cultural integration. And I am happy to report that about nearly 18 months out, we feel like that effort feels complete. We feel like one company. And a lot of synergies associated with that are now evident in our performance. The second big piece was the margin expansion. As you know, when we acquired AQuity, the proforma blended margins of IKS that were traditionally in the high 30s EBITDA had dropped to about 24% proforma. And we had said at the time that as we transform AQuity's delivery model from a sort of human-led model to an IKS technology-led global human-in-the-loop model, we will bring our EBITDA numbers back towards sort of the early to mid-30s. I would like to say that I think a large part of that work is moving faster than we had anticipated, which is obviously being reflected in the numbers that we talk about for Q1. So I would say that that sort of part of the work is maybe two-thirds done.

And last but not the least, and perhaps the most exciting part of the pieces, really was the cross-sell motion of our platform into AQuity's large customer base. And happy to note that I think we have now figured out how that motion is going to work. We are starting to see some wins in that space. Now, obviously, those wins and the revenue impact of those wins is slightly offset by the intentionality by which we are cutting the tail of AQuity's small customers, number one. And second, the revenue in terms of sort of revenue per unit of physician coming down because now we are replacing the sort of AQuity human execution model with an IKS technology-led model. So important to note that even as we are starting to feel early tailwinds of the cross-sell motion, some of them are offset by the intentionality of the pruning of the tail, as well as the revenue per unit coming down even as margins improve in the AQuity customer base. So overall, I think a solid quarter of execution on the AQuity vector.

Third, this predicament of establishing leadership in each of the point solutions, even as we continue to build the platform, I think it's great to receive some recognition from Black Book

as the number one partner in AI-driven RCM

and in medical coding and then as it relates to clinical documentation, both by KLAS and Black Book. So, continued progress in that dimension and of course, the biggest validation of progress there is customer wins, customer

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expansions, which we continue to talk about every quarter and that we continue to make some exciting progress in. We have now figured out our go-to-market strategy and have established our go-to-market teams in line with that go-to-market strategy. So, we have four segments of the market - large single specialty group, large primary care and multi-specialty groups, large and medium-sized health systems, and then last but not the least, absolutely, a whole market segment is UnitedHealth care, which is the largest employer of physicians in the world, (+50,000) physicians.

And happy to note that I think we have now dedicated teams across all of these market segments that are really starting to hum and drive some energy. And along the lines of the buying trend, I want to clarify, we continue to see more platform-based buying behavior across the large single specialty and the large multi-specialty group market segment. The large health system market segment is still more point solution oriented, so we have to drive the land and expand motion there. And then the mid-sized health systems actually are starting to show more and more proclivity towards the platform pieces. So, we continue to drive along those lines. And then last but not the least, here is a very important vector for our future, especially those of you that are not only interested in our next 1-2-3 years, but our next 5-7-10 years. We feel like we are in the process of building a very important long-term moat by becoming more and more of an outcome-oriented company. So, what do I mean by that? Our pricing model was always percentage of customers, revenue based. It was never FTE-based pricing, which has obviously been an advantage for us. But when I talk about being outcome-oriented, we are now actually talking about these constructs that fundamentally align us upstream with our customers in a manner where we achieve three things by doing this alignment of outcomes. First, we actually, while aligning ourselves with the outcomes, we are only in those constructs like we did in Palomar. We will talk about Western Washington Medical Group again. We are actually only bringing the full manifest of the platform to those customers. So, as a byproduct, we are creating more and more examples of the platform pieces and the value of the whole being greater than much of some of the individual parts. Second, these deals are typically much longer-term deals. So, for example, Palomar was a 15-year deal, Western Washington is a 30-year deal, for all practical purposes perpetually. So, they bring a different level of long-term stickiness to our core business model of the platform. And third, they actually create a second set of economics if we can do them right. Because now, not only are we getting our economics for the deployment of the platform, but we also get to participate in the outcomes that we generate through the platform on the client's P&L. So, those three key criteria are the ones that we will continue to use to drive this outcome-oriented company thesis. And the biggest execution challenge that it presents is that now, not only are we responsible for the deployment of the platform, but we are also responsible for driving change in the customer's organization so that they actually produce the outcomes relative to what the pl

atform can do.

This has been a big challenge for customers, so we are solving for that challenge, but it is a unique capability that we have to build over a period of time. And I am happy to note again that in these dimensions, we have made some really good progress. You are all aware of the Western Washington deal, which we will just recap here in a second. Also, the Palomar deal seems to be progressing tremendously ahead of plan. And then the one concern that some of

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you had about Palomar in terms of their financial health, which I mentioned at the time that likely that one of the three big systems in the area eventually buys Palomar, it looks like they are getting closer and closer to an alignment with UC San Diego, which could also have other positive effects for us if we actually end up delivering the value prop that we have for Palomar. So, across each of the five strategic vectors, solid execution, happy to note. And all of that, I think, has resulted into a fairly strong quarter for the fiscal Q1 of fiscal year '26. Happy to note that we have been able to drive 16% year-on-year growth for the quarter, coming to a revenue of about 740 crores. On a constant currency basis, that's 13%. So, as I was saying, we are gaining market share, not losing market share. Also important to note that this is in spite of the continued headwind of pruning the sort of AQuity tail, if you would, and the transformation that we continue to do with the AQuity install base as it relates to that affects their unit price

realization as we get to the physician, but improves our margin significantly . And that margin improvement is actually reflected in our very strong EBITDA performance for the quarter , coming in at 32% or 238 crores, which really is a year-on-year growth of 36% in EBITDA margins, which then translated to an even stronger PAT growth of nearly 59% with our PAT coming in at 151 crores for the quarter . Obviously , the PAT growth is even faster than the EBITDA growth, primarily because our interest costs are coming down as we are continuing to pay down the debts, which is something I am sure that Nithya will drive into.

And then very important to note that all of this has been achieved, like I was saying, with 12,368 employees compared to the 13,277 that we had in Q1 of last fiscal. And that's, as I said, really important because this is the true reflection that for 16% growth or 13% constant currency growth, we have actually reduced the workforce net by 7%. So, the gross reduction, as you can imagine, is pretty significant. And that shows the continued non-linearity in the business and a true demonstration of moving from human-led to tech-led while driving industry-leading growth. So that's been a high-level summary of how we have done in the quarter .

And I am going to invite Nithya shortly to talk a little bit about more details about this, but also just wanted to highlight a few key customer wins. Like I was saying, last quarter , we had announced a very significant customer win in Sky Lakes Health System in the Pacific Northwest, where they had signed up for the full adoption of our platform, not only in the ambulatory side, but also had taken on our acute RCM solution end-to-end. Happy to note that we now have our second end-to-end—we have other acute RCM customers that are also ramping that are not end-to-end, but Mission Community is our second end-to-end—acute RCM customer . So obviously , that segment, that mid-size health system segment of the market is continuing to show promise. Behavioral health is one of the most important and fastest-growing specialties in healthcare today , given the ramifications it has on chronic disease. And so happy to note that we have announced a relationship with Bicycle Health, which is a private equity-owned leadin

g platform, already emerging as a leading platform for behavioural health. And then we have expanded our relationship with OrthoNY to now include our virtual clinical assistant program, which then moves that relationship to a full-platform

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thesis. And then, we have seen some continued growth in our top health system customers. This one is an example of our top health system where we have been able to drive tremendous cross-sell and are now expanding the coding relationship. So, remember , like I said, large health system market, it's still a sort of land and expand motion, whereas in the other segments of the market, we are seeing more and more proclivity to the platform.

So, an exciting quarter of client success and then last but not the least, actually two other points. Western Washington, we had a specific call dedicated to it, but just to recap, this is a monumental opportunity for us for several reasons. One, here we actually get to go all the way upstream and participate in the value creation with a significant sized primary care and multi-specialty group. As you know , invested \$17 million to get a 48% stake in the MSO that has a perpetual contract to manage all of the non-clinical operations. Except in fact, all of the operations other than the actual delivery of care by the doctors is managed by the MSO. And we have a 48% stake in the MSO for our investment. And that MSO is now contracted with IKS back-to-back to leverage our entire care enablement platform to help doctors in delivering better , safer , more efficient care. And so, this also becomes, if you would, our live lab to demonstrate the full value of our platform in deep integration with the EPIC system, which is, as you know , one of the most significant health record systems or systems of record pervading in the provider community in the US. And again, I think for us, it checks all the three boxes of long-term stickiness, full platform adoption in integration with EPIC, and then the second set of economics where our 48% stake in the MSO would actually create that second set of economics in addition to the traditional economics that the platform creates on our P&L. So super excited about this deal and its success will be defined by, we anticipate at the deployment of our platform, likely upside of (+15%) in revenue for the same patient mix and the same payer mix. Now , that could be so transformative in terms of value creation, both for the medical group and then our stake in the MSO, but if they are able to do this, it creates a whole different model for the sustainability of physician-led transformation of healthcare in the US and creates a fundamentally different set of growth vectors for us. So, we will continue to drive such opportunities thoughtfully and in a disciplined manner over a period of time and continue to report against them.

And then again, last but not the least, I want to report out some recognition from the industry . Happy to note that our CFO Nithya, has been awarded the best woman CFO by Business World and also, we were recognized for significant success in the emerging acquisitions

organizational category for our AQuity deal. And then this one is a very important vector where we have been working very hard on making our employee experience unique and happy to note that not only do we have recognition, but we are actually at a point where I think our employee turnover is at an all-time low in the history of IKS. So, with that, all in all, a strong quarter, continued progress along the dimensions that we had set,

and I will now turn it over to

Nithya to talk us through some financial details of the quarter after which we will open up for questions.

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Nithya Balasubramanian: Thank you, Sachin. Good morning, everyone. We have already covered the highlights on this slide, so why don't we dive straight into our cash flow metrics, Saransh if we can go to the next slide. In terms of operating cash flow, we ended the quarter at Rs. 165 crores and free cash flow came in at Rs. 137 crores. These numbers are net of an upfront performance guarantee we have extended to the tune of \$5 million to a multi-specialty primary care organization, one of the market segments that Sachin had highlighted earlier in the call today. With our continued strong cash generation, our net debt position continues to improve. As you can see by the end of the quarter, our net debt position stood at Rs. 448 crores and the continued cash generation in the future quarters is where you will continue to see this number improve.

If you can go back to the previous slide. Our EPS for the quarter stood at Rs. 9 which represents a 58% growth year-on-year and a 2% growth quarter-on-quarter. Our return on equity metrics again remains very healthy and very strong. For the quarter, our return on equity metrics stood at 31%. So, I call out a few highlights on this slide. In terms of FOREX impact, it was quite insignificant and minimal in the quarter. Our employee benefit expense in the quarter stood at 52.3% of revenues compared to 51.8% in the previous quarter. This increase is despite the net reduction in employee count because we do continue to invest in technology as well as increments. And despite this employee benefit expense increase, our EBITDA for the quarter came in at 32% which is a meaningful 90 bps improvement from Q4 where we reported 31.2%. Looking at PAT; PAT came in at 20.5% or Rs. 152 crores. Again, the growth in PAT is in fact even faster than EBITDA growth largely because the finance cost continues to come down. You will note that the number was actually Rs. 26 crores in Q1 of last year versus Rs. 18 crores in the current quarter. And that is reflective of our debt repayment as well as the lower interest rate. Our ETR for the quarter stood at 22%. We have lost tax breaks in one of the SEZ units that we operate out of. For the full year, it's likely to remain in the same range. Our adjusted profit for the period adjusting out amortization of intangible assets stood at 22.7%. Saransh, if we can go and look at the next slide where we talk about our important KPIs. Our adjusted EBITDA number for employee annualized continues to improve quarter-on-quarter. As you can note, it was INR 0.8 million in the quarter. Our revenue from top 10 as well as top 5 customers continue to improve with our continued winning of client deals that you would have noted both in Q4 as well as Q1 now. Aging of top 10 and top 5 customers, again, the vintage remains very-very strong at more than 5 years in both these categories. Compared to Q4, the slight dip is in fact due to the fact that some of our newer customers have ramped up much faster than what we have historically. Thank you, Sachin. Over to you. And we continue to expect to report healthy numbers in the future quarters as well.

Sachin Gupta: Great. Thank you, Nithya. Again, excited about our performance this last quarter and what it means for the future. Also, in line with continuing to build both best-in-class gover

nance as

well as surround ourselves with some of the wisest and most contextual minds in healthcare, happy to note that we are adding Dr. Garheng Kong to our board. Garheng is quite an exceptional professional and leader with under grad degrees from Stanford and MD, PhD, MBAs from Duke. But equally importantly, after a flourishing career in consulting, including

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at McKinsey, he started his own fund, HealthQuest Capital, which now invests across the entire spectrum of deals from startups to publicly traded companies. He is a lead independent director on large publicly traded companies like LabCorp and also happens to be on the board of the Duke University Health System. So tremendous context and leadership and we will continue to see that we will do whatever we can, whatever makes sense to surround ourselves with thoughtful leaders that can keep us both grounded but continue to expand our aspirations in the marketplace. Excited to have Garheng. With that, I think that concludes our prepared remarks for the quarter. Again, very excited about our performance last quarter. The teams have worked really hard and happy to take questions from you. Over to you, operator.

Moderator: Thank you very much, sir. We will now begin the question-and-answer session. The first question is from the line of Sagar Dhawan from V aluequest. Please go ahead.

Sagar Dhawan: Thank you and congratulations on a good set of numbers. My first question is on the growth that we are seeing in the top 5 client bucket. I just wanted to understand better what is driving this. Is this just sort of one client scaling up or is it more broad-based? Is it other ideas which I thought I was assuming was it due to cross-selling of AQuity into the cross-selling to a new AQuity client or is it more outsourcing by some of the clients? Just wanted to understand what is driving this and how sustainable is this in the near term? Should we assume that such kind of growth will continue in the near term as well?

Sachin Gupta: Great, thank you for the question, Sagar and thank you for joining this morning. Yes, look, I think our top 5 customer trend is healthy and I think the answer lies in all of the above in the points that you noted. There is momentum being seen on the cross-sell of AQuity in one of the customers. There are these large platform deals that we have signed that have kicked in into that top 5 and the reality is, Sagar , if you think about it, when I look at the wallet of our top 5 customers, the wallet potential for our platform, we are nowhere near 100%. So, I think one, it's a great sign that the top 5 customers are growing and sure, if we continue to execute like this, there is no reason to believe that the top 5 customer growth should be there. Now , you might not see it linearly like this in every quarter , but when you look at it over a significant time horizon, given the wallet opportunity of our top 5 customers and the strength of our platform, there is absolutely reason to believe that that growth should sustain.

Sagar Dhawan: Understood, sir. And just on the Palomar deal, if you could provide an update on how it is scaling up because it's been like 8-9 months now. Are you seeing the upsides that you had thought about?

Sachin Gupta: Sagar , thank you for asking again. Yes, I think we are ahead of plan actually . I can't go into the specifics here, but I would like to say that so far, we have been delighted. Our teams have executed tremendously on the deal, even as we are not even fully implemented on all the features of the platform. Our financial pro forma 6 months in is better than what we thought it was going to be, even though we are not fully implemented. So, we c

ontinue to be really

optimistic about what the Palomar deal will produce for us and at what pace. Also, as you

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know now, the one concern that some of you had about the financial future of Palomar , that continues to be looking a little bit more constructive and that creates its own additional opportunities, depending on which system eventually ends up with them. So yes, very positive on the Palomar deal.

Sagar Dhawan: Got it. Understood. And one last question from my side is on the tail cutting that we are doing on the AQuity side. Just wanted to understand how meaningful of a drag that has been on the growth in this quarter and what could have been the growth if the tail cutting was not happening?

Sachin Gupta: Sagar , I think I will refrain from providing those specifics, but I will say that it's material. I am comfortable saying that the drag is material and that is why even as we are continuing to be intentional, we are trying to manage the drag in a manner that the cutting of the tail is real, because the reality is that cutting of the tail is also helping with margin growth. But at the same time, it's not so dramatic that it offsets our organic growth, which I think is returning back very strongly . So, material drag, but managed to the best of our abilities.

Sagar Dhawan: Got it. And till when is this process going to continue? You said you want to reach about 500 clients, but just a rough timeline as to when this process could be completed?

Sachin Gupta: You know , that's a tough one, Sagar . Look, it could be as much as another two to three quarters, because like I said, there's two factors here. One, even as you want to cut the tail, we don't want to let the customers down. They have been relying on us for performance, so we can just sort of knee-jerk out of it. And second, the reality is we don't want it to have a dramatic drag in just one or two quarters. And so, I think perhaps another two to three quarters is the way to think about it. And we will let you know if that changes.

Sagar Dhawan: Understood.

Sachin Gupta: Because the question is about 150 odd customers.

Sagar Dhawan: Got it. Thank you for taking my questions and all the best. Thank you.

Sachin Gupta: Thank you, Sagar .

Moderator: Thank you. The next question is from the line of Srinath V from Bellwether . Please go ahead.

Srinath V: Hi, Sachin. Taking on the same kind of Palomar discussion, I want to understand what is the path of the full implementation of the care enablement platform? How does this whole thing work? And I want to understand, if we assume they have 100 physicians, roughly how many of their physicians have like partially taken one of the main suites of products of ours? What percentage of the physicians have actually taken the full platform? And would it be fair to

assume that somewhere in this financial year, we would reach peak revenue potential in Palomar? Just want a qualitative understanding.
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Sachin Gupta: Yes. Great. Thank you, Srinath. Great question. So first of all, the commitment as a part of the deal is for 100% of the physicians to take the full manifest of the platform. So that is the definition of the full implementation. And the way we are rolling it out is there are some features that are already centralized in the way they operate. Those were the features that we implemented first, because what is already centralized is easier to drive as a part of the platform. And so those have been clearly implemented already. And the others are well on their way. It is totally fair to assume that perhaps by the end of this fiscal, which is fiscal Q2, we would be fully implemented across all the features, across nearly 100% of the physicians at Palomar. And so, yes, we would have reached our peak revenue potential at Palomar at that point. Now, again, our peak revenue potential is, as you know, driven by two aspects. One is the platform fee, but then really the kicker comes in the upside. And so those upsides will obviously be evident in our accounting numbers at the end of the year, because that's when they are calibrated. But yes, on the platform fee, the peak would have been achieved by the end of fiscal Q2. You were asking about how long does it typically take? It's a work in progress. It depends on the change management sophistication in the organization. It depends on how much is already centralized versus not centralized. How good are the physician champions in the organization that are able to drive the change? How long does it take us if it's an EHR when we already have integration versus an EHR where as a part of the deal, we have to build the integration. So, there are three or four factors that go into defining how long the full platform implementation might take. But of course, our objective is, on an overall basis, eventually to get to a point once it's more mature, where over a 120 to 150 day period, we can have the full platform implemented across the entire physician base of the customer that we are committed to.

Srinath V: Perfect. I wanted to understand, given if Palomar does get acquired by one of the other players, this is a very IT question, bear with me on this. There's normally some sort of vendor consolidation that takes place. How have you assessed risk? Would it be fair to assume since we are so well integrated into Palomar that we are somewhat safe from vendor consolidation? Any broad views that you have on this, that would be great.

Sachin Gupta: So given that we were potentially anticipating a change of control, even when we did the deal, Srinath, we had already built a no out in the event of change of control. So first of all, there's a very penal contractual protection that will probably be a huge deterrent. But we don't like to rely on just the contractual protection. So, the real value is the excitement that we are experiencing from the Palomar physicians. And what we are most excited about in talking to the CEO is the fact that if we are able to demonstrate the type of ROI that we are talking about, Srinath, I think it will be like, impossible to imagine that whoever the acquirer is, whether it's UC San Diego or one of the other two systems, they would not want to adopt some manifest of such an approach in

their employed physician base, which are, their employed physician base is 10X the size of Palomar's employed physician base. So, you never know what exactly will happen. But one, I think we are contractually totally protected. And second, I think there is more opportunity than risk in Palomar integrating with one of the large systems in the area.
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Srinath V: Perfect. Just the last question on AQuity, I want to understand what percentage or anything you can share on AQuity customer migration to the Ambient AI IKS product, given that it delivers significantly more value at a significant lower price, one would assume that within one year, which would be a standard contracting cycle, then a large part of customers would have migrated, but I don't think that that is the case. So, I want to understand, where are we on the migration? Is it 30%-40%-50% of the customers? And if there's a bottleneck, what would be the bottleneck? And thanks a lot for answering all my questions in great detail.

Sachin Gupta: Sure, Srinath. No, I think, look, the bottleneck is really organizational inertia more than anything else. And so, these are large health systems that have relatively long decision cycles. And so I would say that we are not near the completion of this transition. And like I was saying earlier, I think just like the cutting of the tail, perhaps has another two to three quarters of runway. I think the remainder of this fiscal year is probably a good way to think about when that transition might get completed. Different customers are in different stages of that transition. And remember, the transition is in two features, not just a little documentation, but we are also driving that transition in medical coding. But again, they were in medical coding in a very heavy human-led, stateside human-led model. And now with our autonomous coding

across two specialties and our superior global execution coupled with that autonomous coding, we are also driving transition there. So I would say perhaps a fair way to think about it is over the rest of the fiscal, we should have largely completed that transition.

Srinath V : Thanks a lot, Sachin.

Sachin Gupta: Which is why by that time, we would have gotten our operating margins or EBITDA to a place that could perhaps be called, sort of steady state.

Srinath V : Perfect. Thanks a lot. I will get back in the question queue.

Sachin Gupta: Thank you.

Moderator: Thank you. The next question is from the line of Ruchi Mukhija from ICICI Securities. Please go ahead.

Ruchi Mukhija: Thank you for the opportunity and congratulations, Sachin and team, for a great set of numbers. First question, we are now in 18-month journey of transitioning AQuity business.

Also, the loss of large client and the start of last fiscal is in the base. So, is it fair to assume that as we transition to different quarters of current fiscal, we should see the YOY growth momentum of our business accelerate and move more closer to what we used to do prior to AQuity , more like a 20% growth mark?

Sachin Gupta: Hi, Ruchi. Thank you for the question. As you know , we don't give guidance on future growth, but I think the cues lie in the answers that I sort of tried to give earlier , which is, I think we still have two to three more quarters of AQuity customer tail reduction as well as AQuity margin optimization. And so perhaps fair to assume that the headwinds in revenue associated with

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those two endeavors should be completed by towards the end of this fiscal, let's say fiscal Q4.

So, I think that probably is the timing to think about the return to pure growth versus pure organic growth being offset by some of this intentional headwind that we are driving to try that sort of balance between revenue growth and margin growth, so end of this fiscal.

Ruchi Mukhija: Secondly , in our top 5 clients, looks like the Palomar deal has scaled up and now we have different set of top 5 clients. Also, it would be great if you could talk about the transition time for Palomar and do we expect transition time for the Western Washington Medical Group with two to three quarter transition? Should that also lead to change in our top 5 customers?

Sachin Gupta: Good question. Palomar , like I was saying in my earlier question, I think by the end of this quarter , the current quarter that we are in, we should have completed the full implementation.

On Western Washington, yes, I think it's a fair assumption that in about 120-odd days from now, we should be fully implemented. It's subject to us getting the full epic integration going, which we are working on. That's the only sort of bottleneck there and we are working through that process. So perhaps another 120-odd day, whether or not Western Washington enters our top 5, I can't tell you, Ruchi, right now because it's also a function of what happens to the current top 5 and how quickly they grow over the next 120-150 days. But obviously , it will become a substantial customer and certainly in the top 10, perhaps in 120-150 days from now .

Ruchi Mukhija: Got it. And lastly , could you highlight to us how the cross-sale business or cross-sale activities are planned out during the quarter , selling IKS solution to AQuity customers?

Sachin Gupta: Like I was saying earlier , I think we are still in the early innings of that journey . But the good news is that I think we have figured out how to orchestrate it, more or less, how to work in that motion that is activated by the legacy AQuity salespeople and then how they partner with the sort of overlay partner sales engine that we built that can elevate the conversation. So, I think plenty of green shoots. One, we actually announced publicly our top 5 health systems in the country where we have driven a cross-sell motion very successfully . And there are several others in the hopper . Another one of them is a top 5 health system in the country . So, I would say still early , but I feel like after a whole bunch of trial experimentation, what works, what doesn't work, how to incentivize both the sales engines appropriately , first trying to do it with a platform approach, but then learning that large health systems don't yet have the appetite for the platform approach, but have much greater proclivity for point solution. I think we have figured out the motion. And so, hopefully we will see the fruits of that. And I can do a second tick mark on that cross-sell motion in the next quarter .

Ruchi Mukhija: Got it. This one is for Nithya. You did mention that this quarter we paid \$5 million as part of one of the deals. Do we expect such payout during the current quarter besides Western Washington Medical Group in any of other deals?

Nithya Balasubramanian: Ruchi, I will obviously not be able to talk about what might or might not pan out in the current

quarter , but I think from a more philosophical perspective, Sachin and I had mentioned earlier

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that in each of the market segments that we operate in, we might do one deal or two. These

almost always tend to be full platform deals and these tend to be very, very long-term deals as well. This helps us establish the benchmark in terms of what a platform deal can deliver in each of these market segments.

Ruchi Mukhija: Got it. Thank you and all the best.

Moderator: The next question is from the line of Nilesh Jain from Astute Investment Management. Please go ahead.

Nilesh Jain: Thank you for the opportunity and congratulations, Sachin, for a great set of numbers. My first question is on your revenue growth. Obviously, top 5 has grown very well to the north of 70%.

But when I look at your other top 5 clients, apart from top 10, it has been flat more or less.

How should we look at the top 10 growth apart from your top 5? Maybe you can talk about the organic side of the growth. We can understand that over time, once you cut down the tail, we can expect that at least you grow faster than the outsourcing industry, which is expected to grow at 12%.

Sachin Gupta: I think, again, I will just sort of at the risk of being repetitive, I will say that, first of all, on a quarter-on-quarter basis, it's not easy to... I wouldn't call one quarter a trend where the top 5 are growing and the top 10, the next 5, which are the top 10, are not growing so much that now that's a trend. I wouldn't go so far as to say that. Let's play it out. Right now, I can tell you that we are seeing fairly secular growth across existing customers and the new pipeline. And as you can imagine, given that the cutting of the tail and the transformation of the AQuity book has a drag on the revenue growth, and I said it's a bit of a material drag, you can estimate what the organic growth really looks like. And that's what I would say. It's very hard for me. And I am deliberately not trying to not answer your question, but I don't want to give guidance, number one. And number two, I continue to believe passionately that if we continue executing it the way we are, we will continue to grow significantly faster than the 12% TAM growth.

Nilesh Jain: Probably, maybe you can just help me with the organic growth, just a rough range, that will be helpful.

Sachin Gupta: What do you mean by organic growth? All growth is organic only.

Nilesh Jain: So last year base would have an AQuity number as well which we might not have grown as compared to, I guess, legacy clients.

Sachin Gupta: By the same quarter last year, the integration was complete. So, all growth compared to Q1 FY25 is all absolutely organic growth. So, because there is growth in some AQuity customers as well. If you are asking me to tell you what is the reduction of revenue in AQuity and increase in IKS, that unfortunately I can't do because we are really operating like one company.

Now there's no longer an AQuity account or an IKS account. There are so many accounts that where we have already activated a cross-sell motion. And so those accounts are joint

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customers. So, I don't know whether to account their revenue in legacy AQuity or legacy IKS.

I don't know how to strip those out together. But again, I think if you just do broad math around, we have 16% growth in spite of a material headwind associated with what we are deliberately doing on customer tail and revenue optimization, it should give you a fairly decent idea of what the growth net of that or prior to that would look like, right?

Nilesh Jain: Okay. No problem. Thank you. My second question is on; you talked about radiology partners. So just wanted to understand how is that progress going on that side and are we looking at any JV sort of transaction there?

Sachin Gupta: So that relationship is interesting in that it has not yet converted to a JV. The way we have structured that relationship was that it has to grow to a certain threshold, at which point it makes sense to JV. The relationship has not yet grown to that threshold. So, I don't see a JV conversion happening in the next couple of quarters. But we will continue to track how that goes. The key there is my whole concept on a JV here Nilesh on any of these JVs is first the customer should have fully manifested the platform in their install base. Now there are 3,000 radiologists, there's still a long-long way to go before a large part of their 3,000 radiologists have adopted this program. Also, we have already started to figure out a whole bunch of tech interventions that are changing that virtual radiology system model, even as we were implementing them. So, I think more to come, not close to JV yet.

Nilesh Jain: My last question is to Nithya. I wanted to understand on the employee side, obviously we have been reducing the employee count since last few quarters. So, I wanted to understand how do we see the general trend in terms of, are we done on more or less on the rationalization of the employee side? How do we see there's further scope there?

Nithya Balasubramanian: So, you will obviously see a balance in terms of our continued optimization of the legacy equity workforce. I think like Sachin pointed out, there is at least another two to three quarters where we will continue to optimize and we deploy our technology and achieve the right balance between on-shoring and off-shoring. However, we are of course growing, even as I say that at the same time, we are also growing significantly in other parts of the business. And therefore, we do need to support that growth with additional employees, both in terms of

technologies as well as other administrative employees. So, I think overall, you will probably see that number inch up in the rest of the year as we continue to support the growth that we have been facing.

Nilesh Jain: Sure. Just on the EBITDA margin, you know, once we reach mid-30s, do we expect to see further scope of expansion there of margins over time 2 to 3 three years timeframe?

Sachin Gupta: Look, I think we have said continuously that we expect to get somewhere in the early to mid-30s. As you can see, we are already past the early 30s, almost well ahead of what we had said. And so I just still maintain that, Nilesh, that we will see ourselves getting to early to mid-30s and we think margins should stabilize at that rate. To try and say that margins could

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improve beyond the early to mid-30s, I don't feel comfortable saying that. I think our target still continues to be early to mid-30s. And as you can see from our performance, we feel very confident about achieving that.

Nilesh Jain: Thank you and I wish you all the best.

Moderator: Thank you. The next question is from the line of Seema Nayak from ICICI Securities. Please go ahead.

Seema Nayak: Hi, congratulations on a good quarter. With the headcount down about 300 Q-on-Q, how far are we pushing this lever? And with new deals announced, is there going to be a hiring expected going forward? And what is your ideal annualized EBITDA per employee that you are targeting?

Sachin Gupta: So, look, I think on the headcount, yes, there will be quarter-on-quarter fluctuation based on deal ramps. But I think, which is why I constantly point everyone to please look at year-on-year trends versus quarter-on-quarter trends. The trend that you are seeing year-on-year is a real trend. The trend that you are seeing in consecutive quarter is loaded with all sorts of noise around one customer ramp related to things happening in that customer, a new customer start. So, I would suggest, if you really want to trend that, please look at it year-on-year. And that year-on-year trend is probably the most telling factor.

On the EBITDA per employee, look, the way to track this is really, like we have said, early to make EBITDA at the corporate level. Because also remember, we were traditionally 1.5%-2% R&D expenses. Today, we are close to 5% R&D expense. And we are setting up an AI centre of excellence. We are just about to announce a chief AI officer under which the centre of excellence will be built. There's a whole bunch of things happening that are factored into our sort of general thinking of early to mid-30s. And at least my humble belief is that with the type of growth that we are driving organically, with these types of early to mid-30 margins sustained and the type of ROE, I think this is probably a good place to be. And that's sort of how I would suggest we play it out. But if there is anything else specifically that you are looking for in the EBITDA per employee, we can take it offline. But I think in general, I would say, I will point you back to early to mid-30s operating EBITDA in spite of continued investment in sales and marketing, and significant continued investment in R&D.

Seema Nayak: That's helpful. Thanks.

Moderator: Ladies and gentlemen, that was the last question for today. I would now like to hand the conference over to Mr. Saransh Mundra for closing comments. Thank you and over to you, sir.

Saransh Mundra: Thank you, everyone, for joining the call. Please reach out in case you have any additional questions. We will be very happy to answer. Thank you.

Sachin Gupta: Thank you, everyone. Appreciate it.

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Moderator: Thank you very much, sir. Thank you, members of the Management. Ladies and gentlemen, on behalf of ICICI Securities that concludes this conference. We thank you for joining us and you may now disconnect your lines. Thank you.

Please note that this transcript has been edited for readability.

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Call: Nov 2025

(no extractable text — likely a scanned image PDF)

Call: Feb 2026

(no extractable text — likely a scanned image PDF)

Call: Apr 2026

April 29, 2026

BSE Limited
The Listing Department
Phiroze Jeejeebhoy Towers
25th Floor, Dalal Street
Fort, Mumbai 400 001
Maharashtra, India

BSE Scrip Code: 544309
National Stock Exchange of India Limited
The Listing Department
Exchange Plaza, Plot No. C/1, G Block,
Bandra Kurla Complex
Bandra (East), Mumbai 400051
Maharashtra, India

NSE Symbol: IKS
Dear Sir/Ma'am,

Sub: Transcript of Conference Call held on Friday, April 24, 2026

This is in reference to our letter dated April 23, 2026, regarding the schedule of the Conference Call and submission of the presentation and our letter dated April 24, 2026 regarding the submission of the audio recording of the said Conference Call.

Pursuant to Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, please find enclosed the transcript of the conference call held on Friday, April 24, 2026, for the "IKS Health Announces Agreement to Acquire TruBridge".

The said transcript has also been uploaded on the Company's website and can be accessed through the following link:
<https://ikshealth.com/ir/sebi-disclosures/update-call-transcript.pdf>

This is for your information and records.

Thanking you.

Yours sincerely,
For Inventurus Knowledge Solutions Limited

Sameer Chavan
Company Secretary and Compliance Officer
Membership No. F7211
Encl: As above

"Inventurus

Knowledge

Solutions

Limited

Update

Conference

Call"

April

24,

2026

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ICICI

SECURITIES

Moderator:

Good

morning,

ladies

and

gentlemen,

and

welcome

to

the

IKS

Limited

Company

Update

Conference

Call

hosted

by

ICICI

Securities

Limited.

As

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the
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concludes.
Should
you
need
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during
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conference
call,
please
signal
an
operator
by
pressing
star

then

zero

on

your

touchtone

phone.

Please

note

that

this

conference

is

being

recorded.

I

now

hand

the

conference

over

to

Ms.

Seema

Nayak

from

ICICI

Securities

Limited.

Thank

you

and

over

to

you,

ma'am.

Seema

Nayak:

Hi,

good

morning,

ladies

and

gentlemen.

Thank

you

for

joining

us

on

today's

call

pertaining

to

IKS

Health's

acquisition

of

TruBridge,

a

prominent

provider

of

healthcare

technology

solutions

for

rural
and
community
hospitals.
On
behalf
of
ICICI
Securities,
I
would
like
to
thank
the
management
of
IKS
Health
for
giving
us
the
opportunity
to
host
this
call.
Today
we
have
with
us

Mr.

Sachin

Gupta,

Founder

and

CEO;

Ms.

Nithya

Balasubramanian,

CFO;

and

Mr.

Saransh

Mundra,

Head

of

Investor

Relations.

I

turn

it

over

to

Saransh

for

his

brief

statement

and

to

take

the

proceedings

forward.

Thank

you

and

over

to

you,

Saransh.

Saransh

Mundra:

Hi,

hi

everyone.

Welcome.

I

would

start

with

the

disclaimer

statement

before

handing

off

to

Sachin.

As

part

of

our

prepared

remarks

and

Q&A,
we
may
make
certain
statements
that
are
forward-looking
in
nature,
and
these
involve
significant
uncertainty .
We
don't
take
any
responsibility
to
update
such
forward-looking
statements,
and
your
discretion
is
warranted
while
making

any

investment

decision.

Over

to

you,

Sachin.

Sachin

Gupta:

Thank

you,

Saransh,

and

good

morning,

good

evening

everyone,

depending

on

which

part

of

the

world

you're

joining

from.

Thank

you

for

taking

the

time

at

short

notice

to

attend

our

conversation

today

about

what

I

think

is

going

to

be

a

very

significant

milestone

in

the

journey

of

IKS.

We're

here

today

to

discuss

the

strategic

acquisition
of
TruBridge
that
we
have
embarked
upon
starting
yesterday
where
we
signed
a
definitive
merger
agreement.

To
cover
that
topic,
I
will
stratify
our
discussion
into
four
components.
We'll
start
off

with
the
strategic
rationale
of
the
deal,
go
into
a
little
bit
about
the
transaction
itself,
talk
about
the
opportunities
that
the
transaction
creates
from
a
growth
and
a
market
positioning
perspective,
and

then
finally
end
with
a
sort
of
a
vision
and
a
horizon
statement
about
what
value
we'll
be
able
to
create
from
this
strategic
acquisition
over
the
next
four
years.
So,
and
then

of
course
we'll
turn
it
over
to
people
for
Q&A.
Hopefully
you
all
will
have
a
bunch
of
questions
that
will
make
for
a
good
dialogue.

With
that,
let's
just
dive
in.

As
many
of
you
might
recall,
in
this
journey
of
building
the
industry's
best
cloud-native
AI-first
system
of
action
for
the
ambulatory
care
market,
one
of
the
things
that
we
learned
as
we

kept

building

that

is

that

if

you

remember

the

big

moat

that

I've

described

over

a

period

of

time

that

helps

delegate

chore

tasks

from

these

providers'

continuum

so

that

they

can

focus
on
their
core
was
that
we
would
be
perhaps
one
of
the
only
companies
that
has
the
full
breadth
of
the
platform,
which
is
about
16-odd
tasks
that
our
system
of
action

is
able
to
do
on
behalf
of
these
providers
so
that
they
can
focus
on
their
core.

And
I've
often
described
the
fact
that
the
industry
consists
of
many
point
solution
systems
of

action
where
there
are
companies
that
are
doing
two
or
three
or
five
of
these
tasks,
and
yet
we
believe
that
the
buying
paradigm,
especially
in
some
segments
of
the
market
and
eventually
in

the
entire
market,
will
move
from
sort
of
these
individual
point
solutions
to
a
comprehensive
platform
system
of
action.
So
we
always
believed
that
that
was
going
to
be
our
medium-to-long-term
moat
as

we
go
about
this
mission.

One
of
the
things
that
we
clearly
ran
into
as
we
were
going
about
that
was
the
system
of
record.
And
like
I've
described
in
the
past,

the
system
of
record
is
obviously
a
regulatory
requirement,
it's
part
of
the
core
operating
infrastructure
of
these
large
provider
organizations.

And
the
reality
is
that
there
are
a
whole
bunch
of
native

workflows

in

the

system

of

record

that

don't

necessarily

go

away .

And

then

there

is

obviously

the

single

source

of

truth,

the

longitudinal

data.

And

by

longitudinal,

I

mean

the

data

of

the
patient
across
their
care
journey ,
no
matter
where
they
go
in
the
continuum
of
care,
you
know ,
whether
it
is
the
physician's
clinic,
the
hospital,
the
post-acute
rehab
settings,
it's
a
single

longitudinal
source
of
truth.

And
the
reality
is
what
we
realized
is
that
for
a
successful
platform
system
of
action
to
really
operate
and
create
value,
it
obviously
needs
access
to
the

data
and
it
needs
to
orchestrate
all
of
the
actions
in
the
native
workflow .
Now
traditionally ,
all
these
actions
were
being
orchestrated
in
the
native
workflow
using
humans,
and
then
eventually
we
started

going
from
this
human-led
tech-in-the-loop
to
a
tech-led
model
by
orchestrating
these
actions
through
application
programming
interfaces,
APIs,
that
these
systems
of
record
put
out
there
where
we
can
then
drive
and
orchestrate

these
actions
through
those
APIs.

You
can
access
the
data
in
the
system
of
record
through
the
APIs
and
we
can
orchestrate
the
actions
through
the
APIs.
Now
as
you
can
imagine,
while

that
was
still
very
effective,
the
Holy
Grail,
if
you
would,
would
be
if
you
had
a
deeply
integrated
platform
system
of
action
with
the
system
of
record
where
the
platform
system
of

action

didn't

need

to

leverage

APIs

to

orchestrate

actions

in

the

native

workflow ,

didn't

need

APIs

to

access

the

native

data

that's

lying

in

the

system

of

record

and

in

a

compliant,

de-identified

way,
manipulate
that
data
so
that
the
actions
can
be
orchestrated
appropriately .

And
obviously
it
became
apparent
to
us
that
that
would
be
a
very
distinct
advantage,
and
especially
in
the
world

of
AI,
generative
AI,
and
then
agentic
AI,
where
you
can
then
have
intelligent
interconnected
workflows.

In
that
world,
clearly
the
moat
would
get
compounded
from
one
part
of
the
moat
being

we
would
be
the
only
platform
system
of
action,
but
the
other
part
of
the
moat
would
be
if
we
were
able
to
deeply
integrate
with
a
system
of
record
versus
the
API

integration
that
is
traditionally
provided,
where
we
could
then
orchestrate
the
workflows
in
the
system

of
record
directly ,
manipulate
the
data,
again
in
a
compliant
way,
in
the
system
of
record
directly ,
create

AI
training
corpus
within
the
system
of
record.

That
would
become
an
even
greater
competitive
moat
over
a
period
of
time.

So
in
order
to
compound
the
moat
that
we
were
already

creating
in
the
platform
system
of
action
and
to
make
that
moat,
for
want
of
a
better
term,
AI-proof
or
rather
AI-led,
where
we
are
now
building
and
leveraging
the
AI
versus
having

any
reason
to
worry
about
the
AI,
it
became
apparent
to
us
that
if
there
were
market
segments
that
we
could
find
where
we
could
actually
become
the
system
of
record
and
the

system

of

action.

And

as

long

as

those

markets

were

niche

but

large

markets,

and

as

long

as

those

markets

did

not

put

us

in

competition

with

other

systems

of

record

vendors

that

are
prevalent
in
the
large
health
system
market.

Remember ,

a
core
segment
of
our
business

is
the
large
health
system
market;

there
the
dominant
players

are
the
Epics
of
the
world.

We
do

need
to
continue
to
integrate
with
them
and
continue
to
be
Switzerland
in
those
market
segments,
for
want
of
a
better
term.

We
needed
to
find
market
segments
that
were
not
competitive

to
the
segments
where
the
Epics
of
the
world
were
playing,
that
were
niche
where
we
could
find
a
niche
player
that
would
be
a
dominant
system
of
record
in
that
niche
but

large
market,
and
where
that
system
of
record
had
the
ability
to
combine
with
our
platform
system
of
action
to
create
an
absolute
dominant
force
in
that
market.
And
so
we
very
strategically

began
a
quest
for
finding
such
opportunities
perhaps
about
a
year
ago
as
we
realized
how
to
make
our
platform
system
of
action
most
effective.

And
very,
very
happy
to
note
that

through
a
series
of
conversations,
our
dialogue
developed
with
TruBridge,
which
is
the
absolute
leader
in
the
rural
hospital
market
segment,
if
you
would.
It's
a
niche
market
segment
but
still
a
very

large
market
segment.
Just
to
put
it
in
perspective,
there
are
2,200
rural
hospitals
in
America
that
drive
\$164
billion
of
revenue.
So
even
as
I'm
calling
it
a
niche
market
segment,
it's

a
very
large
market
segment.
20%
of
America
lives
in
this
rural
setting,
so
you're
talking
about
60
million
plus
patients
driving
\$164
billion
of
revenue
across
2,200-odd
hospitals.

And
TruBridge
focuses

on
the
small
segment
of
these
hospitals,
which
is
the
0
to
50
beds.

Typically ,
these
rural
community
hospitals
tend
to
be
about
0
to
200
beds.

TruBridge
focuses
on
the
0
to

50

beds,

and

by

number

of

hospital

count,

they

have

about

700

hospitals,

which

is

maybe

40%

plus

market

share

in

the

0

to

50

beds

and

overall

30%

market

share

over

the

2,200

beds.

So

like

I

said,

a

leading

system

of

record

for

the

rural

healthcare

market,

which

is

a

niche,

but

very

large

market.

20%

of

America

lives

in

rural

healthcare.

It's

a
market
that
continues
to
be
under
significant
challenges,
so
it's
very
similar
to
our
traditional
physician
market.

That
was
the
other
thing
that
was
really
important,
is
that
it's
very
similar
to

our

traditional

physician

market,

although

there

is

zero

overlap

with

our

traditional

physician

market.

So

complementary

but

similar .

How

is

it

similar?

It's

similar

because

in

these

urban

populations,

the

prevalence

of

chronic

disease

is

very,

very

high.

Lifestyles

are

sedentary ,

becoming

more

and

more

sedentary ,

and

access

to

care

is

very

sparse.

So

that

holy

grail

of

lowering

cost

of

care

in

chronic

disease

patients

by
getting
proactive,
improving
quality
of
care
by
freeing
up
the
provider's
time
and
allowing
providers
to
engage
with
the
patients,
and
improving
access
to
care
by
giving
the
providers
time
to
do

more
with
less
proactively ,
that
logic
applied
in
the
rural
health
system
market
as
much
as
it
applied
in
our
traditional
ambulatory
market.

Moreover ,
another
big
similarity
was
that
even
though
the
market

we're
talking
about
is
the
rural
hospital
market,
nearly
60%
to
70%
of
care
that
is
provided
in
rural
hospitals
actually
tends
to
be
outpatient,
which
is
essentially
very
similar
to
the
outpatient

care
that
we
provide,
our
providers
provide
in
our
traditional
customer
base
of
physician
groups.
So
that
also
gave
us
the
confidence
that
a
lot
of
the
capabilities
that
we
had
built
for

our
traditional
physician
market
would
actually
be
relevant
to
that
rural
hospital
setting
because
60%
or
70%
of
the
care
that's
provided
is
actually
outpatient
care,
which
is
very
similar
to
our
physician

care.

So

fully

complementary

market,

very

large

at

\$164

billion,

a

dominant

system

of

record

in

that

market,

also

a

system

of

record

that

already

recognizes

like

we

do

that

the

true

winner

in
this
market
or
in
various
segments
of
these
markets
will
be
the
right
combination
of
a
native
system
of
record
that's
a
leader
and
a
native
platform
of
action
--
platform
system

of
action
company .
And
I
think
their
ability
and
their
recognition
of
the
fact
that
they
needed
to
start
embarking
on
an
integrated
system
of
action
is
reflected
in
the
fact
that
they

proactively
launched
an
RCM
business
deeply
integrated
with
their
system
of
record
back
in
2013.
And
the
whole
idea
was,
if
I
were
just
to
recap,
our
overall
platform
system
of
action
consists

of
16-odd
features,
of
which
maybe
about
seven
to
eight
features
can
be
called
revenue
cycle.
And
so
they
started
building
what
you
would
call
a
partial
platform
system
of
action
proactively ,
deeply

integrated
with
their
system
of
record
in
recognition
of
the
same
understanding
that
we
had,
which
is
that
the
real
winner
in
any
segment
is
going
to
be
the
right
combination
of
a

platform

system

of

action

and

native

system

of

record.

So

it

was

with

these

realizations

that

we

got

excited

about

the

opportunity

to

bring

TruBridge

into

our

fold.

The

other

reason,

as

we
move
to
the
next
slide,
is
when
you
combine
a
system
of
action
with
a
system
of
record,
it
truly
starts
to
create
a
new
operating
system
for
healthcare
that
has
a

flywheel
effect
on
both
patient
engagement,
patient
quality
of
care,
cost
of
care,
and
access
to
care.
Now
how
does
it
do
that?
What
it
does
is,
as
you
all
know ,
the
system

of
record
has,
as
I've
been
saying,
the
single
source
of
truth
longitudinal
data.
It
has
the
native
backbone
for
all
the
workflows
that
the
hospital
has
to
work
and
orchestrate,
be
it

things

like

computerized

physician

order

entry ,

discharge

of

patients,

all

of

that

native

workflows

are

embedded

in

the

system

of

record.

And

what

we

would

do

to

our

system

of

action

is

we

would

ingest

all

that

data,

aggregate

it,

normalize

it,

analyze

it,

and

then

start

to

convert

it

into

the

true

strategic

moat

that

would

get

created

here,

which

is

this

proprietary

AI

training

corpus.

See,

the

difference

between

data

that's

lying

i

n

a

system

of

record

and

an

AI

training

corpus

is

the

fact

that

one

needs

to

truly

aggregate,

normalize,

structure

the

data.

Once

you

start
to
do
that,
then
you
start
to
create
what
I
call
a
longitudinal
dataset,
which
is
a
dataset
of
patients
across
the
continuum
of
care
that
is
labeled.
As
you
know ,
labeling

is
very,
very
important
for
AI,
that
has
awareness
of
the
actions
taken,
right?
So
it's
longitudinal,
it's
labeled,
it
has
awareness
of
the
actions
taken,
and
then
that
starts
to
create
this

AI
training
corpus
of
data
that
really
links
what
I
call
clinical
context
with
actions
taken
and
financial
outcomes
delivered.

That,
ladies
and
gentlemen,
becomes
the
true
moat
of
this
combination.
Because
that

moat

really

lies

at

the

confluence

of

the

data

in

the

system

of

record

and

the

system

of

action

coming

in.

And

being

able

to

in

a

compliant,

de-identified

way

manipulate

that

data

so
that
that
then
becomes
what
you
need
to
agentically
orchestrate
interconnected
workflows
to
basically
eliminate
all
these
tasks
that
we
eliminate
through
our
system
of
action
and
orchestrate
them
in
the
native

backbone

itself.

So

the

users

in

the

hospital

don't

have

to

go

out

of

the

native

backbone,

which

is

the

EHR,

to

orchestrate

these

tasks;

they're

getting

orchestrated

in

the

native

backbone.

And

it's

that

proprietary

AI

training

corpus

and

then

the

native

agentic

workflows

that

one

is

able

to

build

leveraging

that

proprietary

AI

training

corpus

that

starts

to

create

a

structural

long-term

moat.

And

I
think
this
is
very
important.

I
don't
want
to
necessarily
overemphasize
but
actually
can't
overemphasize,
that
one
of
the
big
rationales
of
this
strategic
combination
is
going
to
be
our
ability
to

continue
to
build
this
proprietary
AI
training
corpus.
And
what's
really
sort
of
interesting
about
this
is
the
more
actions
you
keep
taking,
the
more
this
proprietary
AI
training
corpus
keeps
learning
because

it

gets

more

clinical

context,

it

gets

more

understanding

of

what

actions

taken.

Gets

more

understanding

of

what

the

financial

outcomes

were

derived

from

that.

And

it

just

keeps

on

maturing

more

and

more
and
gets
more
and
more
intelligent
over
a
period
of
time.
So
I
think
that's
one
of
the
most
important
strategic
rationales
of
this.
And
why
I
believe
that
this
puts
IKS

in
a
place
from
where
we
can
truly
be
an
AI-led
platform
system
of
action
that
is
orchestrating
all
these
actions
in
the
native
workflow
of
the
EHR.
It
eliminates
any
lag
that

you
would
have
from
the
friction,
because
traditionally
what
happens
is
the
platform
system
of
action
has
to
integrate
with
the
system
of
record
through
middleware.
That
creates
latency ,
it
creates
friction,
it

creates
perhaps
some
errors
in
the
data.
And
if
you're
able
to
do
this
in
an
integrated
manner ,
it
truly
starts
to
reduce
human
involvement.
Now
we
fundamentally
believe
that
the
human-in-the-loop
will

still

be

important,

it

will

continue

to

be

important.

But

the

ability

to

dramatically

reduce

the

human-in-the-loop

is

also

evidenced

by

the

fact

that

if

you

look

at

the

TruBridge

business

and

specifically

the
RCM
part
of
the
system
of
action
that
they've
built.
You
know ,
it's
about
a
\$220
million
business
and
they
do
that
business
using,
give
or
take,
3,500
FTEs
globally ,
right?
Which

is
a
whole
different,
if
we
want
to
put
a
r
evenue
per
FTE
metric,
it's
a
whole
different
revenue
per
FTE
metric
than
the
revenue
per
FTE
metric
that
one
is
able

to
deliver
in
our
--
technology-led
platform
system
of
action
in
the
RCM
part
of
it.
So
I
think
the
evidence
is
clear
that
it
definitely
leads
to
reduced
human
involvement.

And

if
you're
able
to
do
all
that
effectively .

So
if
you
go
to
the
next
slide,
then
a
lot
of
these
actions
that
I'm
talking
about
in
the
platform
system
of
action
truly

get

transformed

from

what

were

asynchronous

type

actions

that

happen

outside

the

native

workflows

or

outside

the

system

of

record.

And

are

then

fed

back

into

the

system

of

record

asynchronously

through

APIs,

they

can

actually

happen

real-time

in

the

native

workflows.

For

example,

take

something

like

our

post-visit

documentation,

the

ambient

scribing

technology ,

Scribble

Now

instead

of

it

being

a

post-visit

tool,

now

it

can

truly

function

like

a

clinical

co-pilot

where

during

the

live

encounter

it

is

servicing

diagnosis

for

the

doctor ,

suggesting

diagnosis

based

on

the

conversation.

It's

actually

servicing

care

gaps

that

they

need

to

plug,

documentation

that

they

need

to

plug

in

order

to

be

able

to

bill

in

the

most

comprehensive

compliant

manner

as

possible.

So

like

I

said,

it

transforms

from

a

post-visit

documentation

tool

to
a
true
clinical
co-pilot.
Or
if
you
take
some
of
the
revenue
cycle
actions
within
our
platform
system
of
action.
It
starts
to
really
go
from,
all
of
these
things
around
payers

deny
claims,
those
claims
have
to
be
worked
at
the
back
end
after
they're
denied.
Now
if
you
are
in
the
native
workflows
of
the
EHR,
you
can
actually
build
denial
prediction
algorithms

because

you

already

have,

like

I

said,

the

clinical

context,

the

action

taken,

and

the

outcomes

achieved

from

the

past.

You

start

to

predict

denial,

and

based

on

the

prediction

of

denial,

you

are
doing
real-time
checks
in
the
workflow
itself
where
you're
correcting
the
documentation
and
the
coding
and
the
charge
capture,
which
actually
eliminates
the
denials
in
the
first
place
instead
of
having
to

work

them

at

the

back

end.

So

again,

you

move

from

a

reactive

platform

system

of

action

to

what

I

call

a

proactive

real-time

platform

system

of

action

where

both

of

those

are

combined

effectively .

Several

other

examples

of

tasks

like

these

that

get

fundamentally

transformed

as

you

integrate

the

system

of

record

and

the

system

of

action.

And

you

know ,

the

one

other

thing

that

I've
heard
is
people
say
oh
my
god,
even
SaaS
technology
in
the
world
of
agentic
AI
is
going
to
get
redundant.
And
I
think
it's
really
important
to
understand
what
when
people

say

SaaS

is

going

to

get

redundant

or

AI

will

replace

SaaS,

what

they're

effectively

saying

is

what

is

the

SaaS

business

model?

The

SaaS

business

model

is

that

you

are

orchestrating

tasks

within
the
core
system
of
record
in
the
native
workflows,
but
those
tasks
are
being
orchestrated
by
humans
and
the
SaaS
technology
providers
charge
you
on
a
usage
based
on
the
per
human

that
is
using
the
technology
or
the
per
seat
license.

Now
imagine
a
world
where
all
of
those
processes
that
the
humans
orchestrate
are
getting
autonomously
orchestrated
by
agentic
AI.
Let's
imagine

that
world
going
forward,
although
I
believe
in
healthcare
you'll
always
have
some
human
in
the
loop,
but
for
argument's
sake,
let's
say
that
it
gets
totally
autonomously
orchestrated,
then
you
don't
need

the
humans.

So
the
argument
people
were
making
is
if
you
don't
need
the
h
umans,
then
how
will
the
SaaS
companies
charge
a
per-user
usage
fee
on
an
ongoing
basis?
But
the

reality
is,
the
reality
is
yes,
you
don't
perhaps
you
don't
need
the
humans,
but
for
autonomous
agents
to
be
able
to
orchestrate
these
tasks,
they
need
access
to
that
AI
training
corpus,

that

key

corpus

that

I

keep

saying

links

the

clinical

context

with

the

actions

taken

and

the

outcomes

achieved,

that

longitudinal

de-identified

labelled

action

aware

data

layer

that

I'm

talking

about.

And

that

ladies
and
gentlemen
is
the
moat
that
we
are
talking
about
creating
here.
No
matter
what
happens
in
that
scenario,
yes,
the
pricing
model
of
SaaS,
which
is
true
in
even
TruBridge's
case,

will

eventually

change

from

per

user

per

month

to

an

outcome-driven

pricing.

But

even

in

the

world

of

fully

autonomous

interconnected

agentic

orchestration

of

all

the

tasks

in

our

system

of

action,

if

that

were

to

be

a

reality .

First

it

gives

us

an

opportunity

to

be

the

leader

to

build

that

ourselves.

Second,

what

it

does

is

even

if

that

were

to

come,

it

moves

us

from

a

SaaS

pricing

model

to

an

outcome-based

pricing

model

and

guess

what

IKS

at

its

core

already

has

experience

with

the

outcome-based

pricing

model.

So

I

think

it

was

really ,

really

important
for
me
to
lay
out
this
strategic
rationale,
which
is
the
core
of
why
we
are
embarking
on
this
transaction
in
the
first
place.

And
then
just
to
summarize
the
benefits

of
the
integrated
model
Saransh
if
we
go
to
the
next
slide.
They
could
be
summarized
in
five
buckets.
You
have
unsiloed
real-time
data,
the
users
don't
need
to
switch
from
one
application

to
the
other ,
there
is
a
closed-loop
virtuous
continuous
learning
cycle
that
gets
built
into
that
AI
training
corpus,
which
is
that
whole
writing
back
as
sort
of
labelling
the
as
you
write

back

the

data

you're

labelling

it.

The

other

thing

that

it

does

is

starts

to

build

the

cross-client

AI.

So

think

about

what

you

learn

about

a

payer

in

one

client,

how

that's

applicable
towards
another
client,
you
can
start
to
build
all
of
that
in
a
cloud-native
AI-first
system
of
action.
And
then
over
a
period
of
time,
think
about
the
ecosystem
advantages
that
it

starts
to
build
and
the
stickiness
that
it
creates.

So
I
really
feel
like
as
we
execute
on
this
strategy
for
the
very
large,
the
niche
but
large
and
very
important
rural

healthcare

market

in

the

US,

this

puts

us

in

a

unique

position

to

truly

become

a

dominant

integrated

system

of

record

and

platform

system

of

action

player

that

is

a

very,

very

long-term

defensible

thesis

over

a

period

of

time.

That's

the

high

level

of

the

strategic

rationale.

Again,

this

is

a

conversation

that

I'm

happy

to

continue

to

have.

I

would

ask

everybody ,

if

you're
genuinely
interested
in
the
company ,
please
try
to
internalize
it.
I
know
I've
been
accused
in
the
past
of
perhaps
using
too
much
jargon
and
so
please
in
the
questions
or
otherwise,

if
you
think
I'm
still
using
too
much
jargon,
challenge
me/
Challenge
us
and
we
will
try
to
keep
simplifying
it
so
that
you
really
understand
because
the
core
of
the
value
of

this
transaction
outside
of
the
obvious
financial
value
will
lie
in
our
ability
to
demonstrate
and
make
this
happen
more
than
anything
else.
With
that,
diving
a
little
into
the
transaction
overview .
So

IKS
is
buying
TruBridge,
which
is
listed
on
the
NASDAQ,
for
an
overall
enterprise
value
of
\$557
million,
which
is
about
\$427
million
in
equity
value
and
then
we
inhe
rit
their
debt
of

about
\$130
million.

That
will
lead
to
a
total
debt
of
a
little
under
\$600
million
for
IKS
at
the
time
of
closing,
which
we
have
secured
for
a
5-year
term
on

a

fairly

decent

interest

rate

of

a

SOFR

plus

275

bps,

which

eventually

slides

down

to

SOFR

plus

175

bps

as

our

leverage

ratio

keeps

coming

down.

And

this

puts

our

starting

leverage

at

a

little

over

3x

EBITDA,

which

traditionally

speaking

I

would

say

is

a

bit

more

than

I'd

genuinely

like.

But

I

think

given

the

strategic

rationale

and

the

rapid

pace

at

which

we

think

we

can

deleverage

through

internal

accruals

given

that

the

deal

is

instantly

EPS

accretive

at

close

and

there

is

a

whole

bunch

of

opportunity

to

drive

both

syner gies

and

growth

in

that

niche

but

very

large

rural

healthcare

market,

we

feel

very,

very

confident

and

comfortable

because

along

with

this,

you

know ,

right

at

the

very

start,

we

also

end

up

with

nearly

a

5x

interest
coverage
on
the
EBITDA
that
we
will
be
generating
pre-synergies.

So
I
think
that's
the
fundamentals
of
the
transaction.

Quickly
about
TruBridge,
like

I
was
saying,
trailing

12
months
2025
calendar

year,
which
is
also
their
fiscal
year,
\$347
million
in
revenue,
about
\$126
million
of
that
comes
from
the
EHR
business
and
about
\$221
million
comes
from
the
RCM
business.
That
demonstrates
that

they
were
one
of
the
early
visionaries
to
say
that
you
have
to
build
at
least
a
partial
system
of
action
deeply
integrated
with
the
EHR
to
truly
be
able
to
bring
the

relief
that
these
healthcare
providers
need
to
make
them
both
financially
viable,
but
then
also
be
able
to,
as
you
expand
the
system
of
action,
enable
providers
to
do
more
with
less.

The
other
thing
that's
very
interesting
about
TruBridge
we'll
delve
into
is
that
for
every
piece
that
they've
embarked
into,
they've
taken
a
tech-led
approach.
So
they've
built
technology
not
just
in
the

EHR

but

also

in

all

the

three

dimensions

of

their

cycle:

front

office,

middle

office,

and

back

office.

Doing

all

that,

they've

been

delivering

an

adjusted

EBITDA

of

about

\$69

million

in

fiscal

25

and

obviously

the

opportunity

to

continue

to

grow

that.

Next

slide,

I

think

very

important,

the

deal

is,

like

I

was

saying,

expected

to

be

PAT

and

EPS

accretive

instantly

at

close.

And

that

is

demonstrated

by

the

fact

that

the

combined

business,

if

you

take

trailing

12

months

calendar

25,

would

be

about

\$698

million

in

revenue

and

\$159

million

in

EBITDA,

but

really

\$186

million

in

EBITDA

when

you

adjust

out

a

couple

of

items

from

TruBridge's

\$43

million

EBITDA

\$9.5

million

in

ESOP

costs

which

will

obviously

go

away .

And

then

several

non-recurring

expenses

that

will

naturally

go

away ,

a

large

expense

around

being

a

public

company

in

the

US,

some

large

expenses

around

one-time

consulting

services

that

they

were

using.

So

when

you

do

that,

their
adjusted
EBITDA
really
adds
up
to
\$69
million.
Ours
continues
to
be
\$117
million
because
we
don't
generally
adjust
our
EBITDA.
And
so
together
we
start
off
at
\$186
million
EBITDA
on

about
\$698
million
in
revenue
at
close.

And
when
you
take
the
\$69
million
EBITDA,
put
the
\$36
million
of
interest
cost
for
the
SOFR
plus
275
for
the
\$565
million
of

debt
that
is
attributed
to
this
transaction
of
the
total
\$600
million
debt,
and
then
based
on
our
estimates
of
the
amortization
of
acquisition
intangible
assets
and
the
recurring
depreciation
that
they
have

on
their
capex,
it
starts
to
instantly
be
accretive
to
the
tune
of
about
\$5
million.

Now
obviously
these
numbers
will
hopefully
probably
get
a
litt
le
bit
better
because
the
final
actual

enterprise

value

of

the

transaction

might

change

a

little

bit

from

that

\$557

million

because

it's

a

function

of

how

much

net

debt

they

have

at

the

time

of

close.

This

\$130

million

of
net
debt
was
at
March
31st.

And
so
given
that
it's
probably
going
to
take
somewhere
between
three
to
four
months
to
close
this
transaction,
that
\$130
million,
given
that
they're
a

cash
flow
generating
business,
that
\$130
million
of
debt
will
come
down,
which
will
then
take
down
our
total
debt
or
possibly
the
debt
attributable
to
this
deal
and
the
interest
cost
associated

with

it.

So

certainly

will

be

EPS

accretive

from

the

get-go

even

prior

to

any

syner gies

whatsoever .

That's

a

little

bit

of

a

high-level

overview

of

the

transaction

itself.

Let's

dive

into

the
market,
why
do
we
think
this
is
an
interesting
market
even
though
it's
a
difficult
market
from
the
market's
own
financial
performance.

First,
let's
just
talk
about,
see,
again,
I
just
want

to
remind
everybody
when
we
talk
about
our
platform
system
of
action,
there's
about
16
to
18
tasks
depending
on
which
market
it
is
that
we
delegate
out
of
the
provider's
workflow
and

do

for

them

better ,

cheaper ,

faster

at

scale,

right?

These

are

the

chore

tasks.

Now

of

that,

there

are

probably

about

seven

to

eight

tasks

that

can

be

grouped

together

to

call

them

RCM

tasks

across

the

front

office,

middle

office,

and

back

office.

For

these

seven

to

eight

RCM

tasks,

the

reality

is

that

the

RCM

market

is

still

highly

fragmented.

The

top

five

players

have
less
than
20%
market
share,
there's
400-plus
players,
and
then
there's
a
massively
long
tail.
So,
there's
really
no
significant
leaders
that
have
emerged
across
the
healthcare
continuum
on
the
revenue
cycle.

And
then
when
you
look
at
the
rural
hospital
RCM
market,
the
way
to
think
about
it,
like
I
was
saying,
is
the
rural
hospital
market
is
about
\$164
billion
of
the

\$1.75

or

\$1.8

trillion

of

the

overall

hospital

market.

And

they

spend,

we

have

taken

a

take

rate

just

on

these,

remember ,

on

these

seven

or

eight

RCM

tasks.

This

is

not

our

full
platform
system
of
action,
this
is
just
the
RCM
part
of
it.

We're
taking
a
take
rate
of
2%.
Why
are
we
taking
two
percent?
because
today's
take
rates,
by
the
way,

in
their
business
are
even
higher ,
they're
closer
to
3%.
But
we're
modeling
it
at
2%
because
we
believe
that
as
we
ourselves
build
agentic
AI-based
orchestration
of
tasks
that
can
be
orchestrated

autonomously

or

even

with

some

human

in

the

loop,

there

will

be

some

price

compression,

so

we

took

a

very,

very

conservative

number .

That

itself

comes

to

about

a

\$3.5

to

\$4

billion

RCM
TAM
within
these
rural
hospitals.

Now
the
revenue
that's
running
through
TruBridge

's
700
hospitals
is
about
\$32
billion
of
the
\$164
billion.

So
remember ,
there's
2,200
hospitals,
TruBridge
operates
in

the
small
hospital
segment
of
that
market,
so
0
to
50
bed
hospitals,
and
those
700-odd
hospitals
drive
about
\$32
billion
of
revenue.
And
taking
that
sort
of
2%
odd
take
rate,
that

creates

a

\$660

million

immediately

addressable

TAM.

And

TruBridge's

RCM

revenue

is

around

\$220

million,

so

it

creates

about

\$450

million

of

white

space

in

their

captive

install

base.

Please

keep

in

mind

that

\$450

million

is

assuming

some

rate

compression,

which,

you

know ,

we

can't

predict

how

quickly

that

would

happen,

so

we've

just

taken

it

a

little

bit

conservatively .

And

so

that

becomes

the
immediate
low-hanging
fruit
that
we
can
go
after
to
drive
growth
in
the
TruBridge
customer
base.

It's
a
simple
cross-sell
of,
you
know ,
the
other
thing
that,
you
know ,
you
might

recall

is

but

we

had

also

thought

of

doing

cross-sell

in

the

Acuity

deal,

it

took

longer

than

expected.

I

think

there's

one

big

distinction

here:

the

buyer

of

the

electronic

health

record

is
the
same
as
the
buyer
of
the
revenue
cycle.

The
buyer
of
the
electronic
health
record

is
the
CEO,
the
CFO,
and
the
CMO.

The
buyer
of
the
revenue
cycle

is
the

CEO,
CFO.
And
so
we're
not
going
to
now
cross-sell
a
new
service
into
a
different
part
of
their
organization,
and
that
creates
a
whole
different
type
of
immediacy
in
the
opportunity
that

it
creates
from
a
growth
perspective.
So
this
\$450
million-odd
is
going
to
be
just
the
immediate
low-hanging
fruit.

Now
when
we
start
to
model
the
other
tasks
that
are
part
of

our
platform
system
of
action
and
those
being
cross-sold
also
into
the
TruBridge
EHR
install
base,
that
\$440-\$450
million
number
expands
dramatically .
We
think
in
the
immediate
term
it
becomes
close
to
\$600

million,
but
then
even
expands
dramatically .

The
other
thing
that's
interesting
is

there
are
already
several
other
rural
hospital
customers

where
TruBridge
is
providing
RCM
services

or
the
RCM
system

of
action
independent

of
their
EHR.
Now
that
gives
us
another
growth
avenue
as
we
continue
to
build
on
that,
that
creates
because
that
market
overall
is
about
a
\$3.5
billion
RCM
market,
the
rural
healthcare

market.

Even

if

you

take

out

the

TruBridge

install

base

of

\$660

million,

you're

still

talking

about

a

\$3

billion

hospital

market

on

which

there

is

about

30%

penetration.

So

there's

a

\$1

billion

outsourced

rural

hospital

market,

and

that's

growing

at

12%

to

13%.

So

in

addition

to

the

RCM

cross-sell

in

the

TruBridge

customer

base,

there

is

also

RCM

selling

opportunity

in

the

non-T ruBridge

EHR

install

base.

Again,

something

that

IKS

knows

to

do

from

its

legacy

business.

So

that's

one

of

the

big

growth

drivers

in

this

opportunity .

Just

to

give

you

a

quick

sense
of
the
technology
stack
for
RCM,
like
I
was
saying,
TruBridge
has
always
taken
a
tech-first
approach
in
whatever
system
of
record
and
system
of
action
that
they
have
built.
And
so

they
have
very
significant
technology
interventions,
proprietary
technology
interventions
across
the
front,
middle,
and
back
office
of
the
revenue
cycle.

All
the
way
from
patient
engagement
tools
like
Get
Real
Health
to
their

own

clearinghouse,

which

by

the

way

is

not

only

going

to

be

useful

in

orchestrating

the

revenue

cycle

for

TruBridge,

but

also

parts

of

that

clearinghouse

would

be

relevant

in

our

traditional

ambulatory

business.

Remember ,

in

our

traditional

ambulatory

physician

business,

we

use

outside

third-party

vendors

as

the

clearinghouse.

Now

if

we're

able

to

repurpose

TruBridge's

clearinghouse

in

those

settings,

that

will

also

give

us

some

cross-leverage

there.

And

then

there's

a

very

exciting

tool

that

TruBridge

owns

called

TruCode,

which

is

a

coding

assistance

tool.

And

that

actually

is

a

pure

SaaS

model

that's

been

growing

pretty

significantly

over

the

years.

And

we

actually

believe

that

the

TruCode

tool,

while

it's

a

very

integral

part

of

their

revenue

cycle

offering,

is

also

sold

stand-alone

outside

of

the

TruBridge

customer

base.

In
fact,
traditionally
IKS
has
been
one
of
the
customers
of
TruCode,
which
is
a
coding
assistance
tool,
it's
a
tool
that
coders
use
to
deliver
better ,
mo
re
accurate,
compliant
coding.
And

so
the
potential
market
in
which
one
can
sell
the
TruCode
tool
actually
itself
is
about
\$650
million
TAM,
and
so
one
of
our
endeavors
will
be
to
accelerate
that
sales
motion.
And
then

last
but
not
the
least,
they've
got
some
very
interesting
analytics
technologies
that
they
have
on
the
back
end
of
the
revenue
cycle,
like
Viewgol.
So
a
comprehensive
front-to-back
revenue
cycle
platform
of

action,
which
is
remember
about
8
of
the
18
tasks
of
our
overall
platform
system
of
action,
which
is
truly
technology-led.

So
the
revenue
cycle
is
the
predominant
vector
of
growth.
As

you
know ,
the
EHR
market
is
highly
sticky
and
highly
mature,
and
so
just
anyways
let's
take
a
quick
look
at
the
EHR
market.
And
the
EHR
market,
like
I
was
saying,
has

about
2,200-odd
rural
hospitals,
of
which
700-odd
are
TruBridge
clients.

Now
given
that
those
700
tend
to
be
the
smaller
hospitals,
they're
highly
dominant,
they
have
40%
market
share
in
that
0

to
50-bed
hospital
range,
and
that's
where
TruBridge
tends
to
operate,
they
have
about
\$32
billion
of
revenue
flowing
through
those
customers.
So
overall,
40%
penetration
in
the
0
to
50-bed
market
and

about
between
25%
and
30%
penetration,
closer
to
30%
penetration
in
the
overall
rural
hospital
market.

What
this
does
is
it
creates
two
other
vectors
of
growth
opportunity .
One,
as
we
modernize

and

truly

take

their

EHR

architecture

to

a

cloud-native

AI-first

technology

architecture,

what

this

will

do

is

it'll

allow

us

to

go

penetrate

more

dominantly

some

of

the

other

0

to

50-bed

hospitals,

but
slowly
even
perhaps
go
upstream
into
the
50
to
200-bed
hospital
range,
which
then
becomes
a
growth
market.

And
so
those
would
be
the
two
vectors
of
growth
as
it
relates
to

the
EHR
market,
which
will
be
a
slower
vector
of
growth
than
the
RCM
market,
which
really
will
be
the
biggest
vector
of
growth.
And
then
in
addition
to
RCM,
there
will
be

the
other
features
in
the
platform
system
of
action
that
IKS
has
that
we'd
be
able
to
bring.

A
quick
overview
on
the
overall
tech
stack
that
TruBridge
has
and
what
are

some
of
the
key
competitors.

Like

I

was

saying,

the

RCM

stack

has

technology

interventions

across

the

front,

middle,

and

back

office.

There

are

some

pure-play

RCM

players,

they're

not

systems

of

record,

they're
not
even
really
systems
of
action,
but
they
are
much
more
human
BPO-type
of
RCM
players
like
Med-Metrix
and
CorroHealth
that
one
often
runs
into
in
this
rural
hospital
market
segment.

On
TruCode,
which
is
their
encoder
SaaS
technology ,
coding
assistance
technology ,
the
two
major
competitors
are
3M
and
Optum.
What's
interesting
is
that
lately
we've
seen
TruBridge
take
market
share
in
a
small

way
from
3M
and
Optum,
which
are
the
dominant
players
in
this
space.

We've
already
spoken
about
the
EHR.
Like
I
was
saying
earlier ,
we
were
very
careful
to
look
for
niches

where

there

is

no

competitive

angle

at

all

with

the

Epics

of

the

world,

the

Athenas

of

the

world,

the

eClinicalWorks

of

the

world,

the

NextGen

of

the

world,

with

whom

we

need

to
integrate
deeply
in
our
other
legacy
part
of
the
business,
which
is
the
physician
business.

So
we
definitely
did
not
want

to
create
even
the
semblance
of
a
dynamic
of
competitiveness
there.

And
so
the
only
two
EHRs
that
are
meaningful
competitors
in
the
rural
hospital
market
tend
to
be
MEDITECH
and
then
Oracle,
which
is
basically
Cerner .
As
they
lost
market
share
to
Epic

in
the
large
health
system
space.
They've
been
trying
to
come
downstream
into
this
space,
not
so
successfully ,
but
really
MEDITECH
has
been
the
more
successful
player
in
this
space.
So
I
would

say

the

rural

hospital

market

is

already

a

mature

market

and

nearly

already

mature

enough

to

be

just

a

two-player

market

between

TruBridge

and

MEDITECH,

with

Oracle

trying

to

be

the

challenger

to

the
two
main
incumbents,
which
are
MEDITECH
and
TruBridge.

And
then
what
they
are
doing,
obviously ,
because
they
have
not
been
able
to
build
their
own
comprehensive
system
of
action
outside
of

RCM,
they
are
using
other
vendors
to
complete
their
system
of
action.
For
example,
for
virtual
clinical
documentation
or
ambient
clinical
documentation,
they
are
partnering
with,
you
know ,
the
Microsoft-owned
company
called
DAX.

There
are
obviously
other
players
even
in
rural
healthcare
that
are
pure-play
documentation
systems
of
action
like
Augmedix
and
Abridge
and
Ambience
Healthcare.
And
then
of
course
on
the
patient
engagement
side

where

they

actually

have

their

own

technology

play,

which

I

think

we'll

be

able

to

bolster

dramatically

with

our

latest

AI-led

autonomous

patient

engagement

technology

that

we've

built,

is

players

like

Phreesia,

which

is
a
publicly
traded
company
in
the
US.
But
again,
I
think
especially
in
markets
like
these,
these
rural
hospitals
have
no
appetite
for
buying
point
solution
tools
outside
of
the
system
of

record
that
don't
work
in
the
native
system
of
record.
So
I
think
we'll
have
a
very,
very
distinct
advantage
here.

I'll
end
this
section
by
saying,
like
I
was
saying
earlier ,

the
TruCode
encoder ,
which
is
a
coding
assistance
tool,
of
which
IKS
itself
is
a
customer ,
actually
has
potential
well
above
and
beyond
the
TruBridge
install
base.
And
it
really
is
shown
in

scenarios

where

autonomous

coding

--

agentic

autonomous

coding,

will

not

be

a

reality

because

remember

one

thing:

Generative

AI

is

a

non-deterministic

technology .

Coding

is

a

deterministic

outcome.

So

Generative

AI

will

not

be

able

to

solve

coding

in

a

fully

autonomous

way

in

most

of

the

specialties.

Very

few

specialties

might

get

really

close

to

autonomous.

And

so

coding

assistance

is

going

to

be

a

reality

in

the

world

of

medical

coding

for

a

long,

long

time,

and

hence

TruCode

Encoder

becomes

a

really

interesting

growth

vector

opportunity

not

only

in

the

TruBridge

install

base,

but

in

the

entire

healthcare

continuum,

if

you

would.

So

that's

an

overview

of

TruBridge

and

the

growth

vectors.

Again,

the

biggest

growth

vector

and

the

most

immediate

is

continuing

to

grow

the

RCM

platform

of
action,
or
the
RCM
part
of
the
platform
of
action,
in
their
installed
base,
EHR
installed
base.
Take
the
RCM
platform
of
action
to
the
non-EHR
installed
base
within
the
rural
healthcare

setting,
and
then
start
to
also
sell
other
features
of
IKS's
platform
system
of
action
into
their
EHR
installed
base
because
they're
already
plugging
in
other
partners
in
that
installed
base.
So
those

would

be

the

three

big

low-hanging

fruit

growth

vectors

to

start

with.

And

then

of

course

as

we

modernize

the

EHR

and

truly

build

it

in

an

integrated

cloud-native

AI-first

system

of

record

plus
system
of
action,
there'll
be
a
real,
real
opportunity
to
then
manifest
all
of
the
other
features
of
our
system
of
action
outside
of
revenue
cycle
in
the
TruBridge
EHR
installed
base.

Last
but
not
the
least,
value
creation.
So
again,
I
just
want
to
reiterate,
the
biggest
driver
of
this
transaction
for
us
is
to
create
a
long-term
strategic
moat
that
truly
puts
us

at
the
forefront
of
leveraging
AI
to
magnify
our
impact
versus
perhaps
being
concerned
about
what
AI
does
to
us.
I
think
that
that
is
a
very,
very,
very
critical
part
of
the

value
creation.
But
before
I
dive
into
the
specifics
of
the
overall
vectors
of
value
creation,
let's
just
look
at
how
TruBridge
and
IKS
are
better
together .

Let
me
go
to
Slide

21;

first

of

all,

customer

segments.

As

you

know ,

IKS

traditionally

was

focused

on

ambulatory

care,

which

is

physician

groups,

single

specialty ,

multi-specialty ,

and

large

health

system-owned

physician

groups.

And

now

more

and

more
so,
we
are
doing
end-to-end
platform
deals
with
mid-sized
health
systems
as
well
and
their
physician
groups.
So
that's
been
the
traditional
IKS
market.
Like
I
was
saying,
the
TruBridge
market,
which

is
the
rural
and
community
hospitals,
which
is
predominantly
0
to
200,
but
some
in
the
community
are
even
under
400,
that
is
the
TruBridge
core
market.
So
there's
complete
complementarity ,
and
yet

a
lot
of
the
other
features
of
our
care
enablement
platform,
which
is
our
platform
system
of
action,
are
applicable
to
the
TruBridge
install
base
because
even
though
they
are
hospitals,
60%
to

70%
of
the
work
that
is
done
in
the
hospitals
is
ambulatory
in
nature,
which
is
the
work
that
our
system
of
action
is
built
for.
Again,
so
a
complementary
market
but
a

market
in
which
our
core
capabilities
are
very,
very
relevant.

And
then
last
but
not
the
least,
from
a
delivery
standpoint,
they
really
are
a
strong
local
presence
company .
We
are
truly

an
entity
that
has
been
able
to
build
a
strong
blended
delivery
model
for
the
human-in-the-loop
part
of
our
platform
that
leverages
the
right
combination
of
talent
in
the
US
and
India
to

deliver
most
cost-effective
outcomes.
And
I
think
that
is
a
huge
complement
that
we
will
bring
to
the
TruBridge
operating
model
and
their
success
over
a
period
of
time.

So
net-net,
I

think

a

creation

of

a

very

complementary

organization

that

creates

a

long-term

strategic

moat

for

us,

the

four

key

vectors

to

summarize

are:

one,

like

I

was

saying,

having

a

deeply

embedded

EHR

system
of
record
in
such
a
large,
but
niche
market
segment
that
is
already
mature
actually
creates
a
tremendous
long-term
moat,
especially
as
we
modernize
that
EHR
and
convert
their
data
into
what

I
keep
calling
this
AI
training
corpus.

I
think
it's
a
huge
opportunity .

That
AI
training
corpus
then
allows
us
to
actually
execute
on
this
AI-first
strategy
so
that
we
can
truly
leverage

AI
as
a
tremendous
tailwind.
And
in
recognition
of
that,
we're
actually
launching
the
start
of
building
a
small
language
model.
We'll
start
with
an
acute
RCM
small
language
model.
Now
why
a

small

language

model?

Small

language

model

has

two

benefits,

right?

It

starts

to

build

its

own

--

it's

built

out

of

its

own

proprietary

data,

and

over

a

period

of

time

it

reduces

the
cost
of
compute,
right?
So
othe
rwise
you're
leveraging
the
LLM,
the
data
is
no
longer
proprietary
because
the
LLM
has
access
to
all
of
that
data.

In
a
small
language
model

that
sits
on
top
of
the
LLM,
now
the
data
is
proprietary ,
it's
a
self-learning
loop,
and
the
cost
of
compute
is
much
lower
because
you're
using
the
LLM
much
lesser ,
it's
this

SLM
sitting
on
top
of
the
LLM
that
is
actually
driving
a
lot
of
the
Generative
AI
that
you're
going
to
use
to
orchestrate.
We'll
start
on
the
RCM
side
and
then
we'll

expand

these

SLMs

into

other

tasks

in

our

platform

system

of

action

over

a

period

of

time.

Growth

opportunities,

like

I

said,

low-hanging

fruit

growth

opportunity

is

the

RCM

part

of

the

platform

system
of
action
cross-sell
into
the
TruBridge
customer
base
and
then
adjacent
to
their
customer
base,
the
50
to
200
bed
customer
base.
And
then
actually
going
from
RCM
to
other
features
in

our
platform
system
of
action,
including
but
not
limited
to
our
ambient
scribing
product.
And
then
of
course
that
\$650
million
plus
market
potential
for
TruCode,
which
consists
of
opportunities
even
outside
TruBridge

installed
base
that
can
be
leveraged.
And
all
this,
we
have
a
pretty
large,
further
scalable
efficient
operating
platform
with
17,000-plus
people,
of
which
2,000
are
clinically
trained,
800-plus
technologists.
So
we
can

really

accelerate

both

the

modernization

of

the

electronic

health

record

and

then

leveraging

that

modernization

to

dramatically

and

rapidly

build

I

think

a

one-of-its-kind

industry-first

integrated

platform

system

of

action

and

system

of

record
that
is
cloud-native,
that
is
AI-first,
that
is
able
to
have
the
best
chance
to
agentically
intelligently
orchestrate
interconnected
workflows
to
delegate
all
these
chores
of
healthcare
so
that
providers
can
focus

on
their
core.

Of
course,
from
a
syner gy
perspective,
there'll
be
opportunities
on
G&A,
there'll
be
significant
opportunities
from
a
delivery
transformation
perspective.
TruBridge
obviously
has
a
significant
headcount
in
the
US

doing
the
human-in-the-loop
as
it
relates
to
RCM,
there'll
be
opportunities
there.
And
obviously
that
gives
us
confidence
in
some
of
the
financial
opportunities
that
this
transaction
creates.
And
you
know ,
in
addition

to
this
very
important
strategic
rationale,
which
then
creates
a
truly
defensible
long-term
moat
that
leverages
AI
versus
the
one
that
worries
about
AI,
we
also
thought
that
we
will
combine
that
with

giving

you

all

a

sense

of

where

we

can

take

the

business

financially .

And

as

you

know ,

trailing

12

months

December

31st,

2025,

we're

about

a

INR1,000

crores

EBITDA

business

with

a

net

debt
of
about
INR300
crores.
And
we
feel
like
while
it
is
not
easy
to
predict
the
exact
trajectory
of
our
runway
over
the
next
3
to
4
years
because
there's
so
much

work
to
do,
there's
also
so
much
more
to
learn.
I
feel
like
we've
learned
enough
through
the
process
of
bringing
this
transaction
together
that
we
feel
comfortable
to
say
that
we
should

be,
in
my
--
and
please,
I
want
to
caveat
this
again
and
again,
this
is
not
a
guidance,
and
I
know
that
even
if
I
say
that
you
guys
will
assume
whatever

it
is,
but
this
is
our
true
north.
This
is
the
financial
manifest
of
all
the
execution
that
I
talked
about,
all
the
vectors
of
growth,
all
the
vectors
of
the
strategic
transformation

of
the
EHR,
creation
of
that
AI
training
corpus
moat,
the
financial
translation,
all
the
syner gies
that
we've
already
envisioned,
all
the
operating
transformatio
n.
We
think
that
we
would
be
disappointed
with
ourselves

perhaps.

Of

course,

there's

still

more

to

learn,

if

by

the

end

of

FY30

we've

not

delivered

something

in

the

range

of

a

INR3,000

crores

EBITDA

and

gotten

our

net

debt

levels

back

to
--
the
net
debt
levels
that
we
were
at
in
December
2025,
which
eventually
is
very
close
to
zero
in
the
larger
context.

So
I
think,
net-net
makes
for
a
very,

very
exciting
growth
opportunity
and
one
that
is
strategically
defensible
in
the
true
medium
to
long-term,
makes
us
an
undeniable
leader
in
a
niche,
but
very
large
and
important
market
segment
like
rural

healthcare,
allowing
us
to
continue
to
bolster
our
strength
in
the
ambulatory
setting,
which
is
our
traditional
physician
group
setting.
And
in
order
to
just
--
when
we
put
our
--
when
we

did

our

bottom-up

math

and

we

arrived

at

this

number

of

nearly

INR3,000

crores

of

EBITDA

potentially ,

we

said,

how

do

we

gut

check

this

number

because

when

you

just

look

at

it,

it
suddenly
feels
like,
"Oh
my
god,
you're
tripling
your
EBITDA
in
4
years,
it's
like
31%
CAGR
from
December
2025
to
March
2030."
But
when
you
put
it
in
perspective,
if
we

look

at

our

10-year

trajectory

at

IKS,

as

we

go

to

the

next

slide,

please.

Look

at

our

10-year

trajectory

at

IKS,

we've

actually

delivered

28%

revenue

growth

and

33%

EBITDA

CAGR.

So

that
gave
us
the
confidence
that
it's
kind
of
in
line
with
the
type
of
growth
that
we've
been
able
to
deliver .

If
you
remember
even
when
we
acquired
AQuity ,
which
really
is

one
of
the
proofs
of
our
abilities
to
do
a
large
transaction,
at
the
time
of
closing
the
deal,
the
pro
forma
EBITDA
was
24%,
we
were
able
to
take
that
to
the

early
to
mid-30s
in
a
little
over
4.5,
5
quarters.
We
were
able
to
integrate
two
equal-sized
companies
into
one
significant
organization.
One
of
the
learnings
was
that
the
cross-sell
took
longer ,
we

analyzed

why

it

took

longer

and

we

applied

those

learnings

here

to

say

why

will

this

cross-sell

work

versus

that

one

taking

longer .

Remember

I

said

the

buyers

in

the

AQuity

cross-sell

were

very

different.

We

were

cross-selling

features

of

our

platform

to

a

totally

different

buyer

base

within

the

large

customer

organization

than

we

would

be

selling

here,

the

buyers

are

exactly

the

same.

So

we
learned
that
through
the
AQuity
transaction.

And
yet,
over
a
period
of
time,
we
have
now
started
executing
multiple
cross-sell
deals
even
in
the
AQuity
install
base,
one
that
we
just
sold

recently

was

in

--

towards

the

end

of

Q4

that

is

going

to

be

announced

soon.

So

I

feel

like

we

do

have

an

organic

and

an

inorganic

execution

track

record

that

makes

us
feel
like
embarking
on
what
will
be
this
very
strategic
journey
to
create
a
very
exciting
long-term
defensible
story
to
improve
healthcare
in
the
US.

Next
steps,
going
into
the
transaction,

we

signed

the

agreement

yesterday .

It's

expected

to

take

90

to

120

days

to

close.

In

that

timeframe,

we

absolutely

operate

like

totally

independent

companies,

focus

on

our

individual

objectives,

work

through

the

regulatory
approval
process,
which
i
ncludes
the
HSR
approval
from
the
FTC,
which
is
the
Federal
Trade
Commission.
Once
the
transaction
closes,
we
anticipate
a
4
to
5-year ,
four
to
five
quarter
integration

process
where
we
start
combining
ourselves
into
one
company .
And
we've
factored
all
of
that
into
sort
of
this
4-year
journey
that
I
just
described
to
you
all
a
couple
of
minutes
ago.

That's

really

where

I'll

end

the

conversation

with

the

last

slide.

Again,

I

would

say

our

traditional

moat

was

we

were

building

the

only

platform

system

of

action.

As

we

evolved,

we

understood

that

the
platform
system
of
action
moat
needs
to
be
further
bolstered
by
if
we
were
to
combine
it
in
niche
markets
with
the
system
of
record,
that
would
make
it
really
long-term
defensible.

Here
we
have
the
opportunity
to
be
able
to
do
that
and
really
build
an
AI-first
integrated
platform
system
of
action
system
of
record.
TruBridge
specifically
has
some
exciting
growth
opportunities
within
its

install

base

that

we

will

rapidly

work

on.

And

together

I

think

we'd

have

created

an

industry-leading

scalable

and

efficient

operating

platform

which

I

really

believe

will

be

the

new

operating

system

at

least
for
rural
healthcare
and
then
continue
to
build
upon
our
traditional
physician
install
base.
So
I
will
pause
there
at
this
point
and
I
guess
turn
it
over
to
the
operator
and

we

can

go

into

the

Q&A.

Moderator:

Thank

you

very

much.

We

will

now

begin

the

question

and

answer

session.

We

will

take

the

first

question

from

the

line

of

Keyur

Ladhawala

from

ValueQuest.

Please

go

ahead.

Keyur

Ladhawala:

Yes.

Thanks

for

the

comprehensive

commentary

and

congratulations

to

Sachin

and

the

management

team

on

the

acquisition.

Sachin,

my

first

question

to

you

was,

can

you

give

us

some

color

on

the

competitive

intensity

in

the

rural

segment

across

both

the

EHR

and

RCM

verticals?

Can

we

leverage

IKS's

learnings

and

deep

domain

expertise

to

enhance

TruBridge's

right

to

win

in
the
rural
segment?

And
also
if
you
could
help
us
understand
what

is
the
existing
overlap
between

the

RCM

and

EHR

clients?

Sachin

Gupta:

Super .

Thank

you

for

the

wishes,

Keyur ,

and

for

the

thoughtful

question,

I

appreciate

it.

Saransh

if

you

don't

mind

going

to

Slide

18

so

that

I

can

answer

Keyur's

question

on

the

competitive

intensity .

So

Keyur ,

as

if

you're

able

to
see
the
slide,
what
we've
done
is
we've
laid
out
the
key
competitors
across
all
of
the
key
offerings
that
TruBridge
has
within
the
rural
healthcare
market.
So
let's
first
focus
on

the
EHR,
right
in
the
middle.

Like
I
was
trying
to
say,
the
EHR
market
is
already
very
mature
for
the
rural
healthcare
setting.

I
think
it's
already
more
or
less
a

two-player

market

with

TruBridge

and

MEDITECH

being

the

leading

pairs

and

Oracle

after

some

of

the

setbacks

they've

had

in

their

traditional

Cerner

large

health

system

market

segment.

They've

been

coming

a

little

bit

downstream

to

try

and

play

in

this

rural

market.

So

if

you

want,

the

way

to

think

about

it

is

TruBridge

and

MEDITECH

are

the

two

dominant

incumbents

and

Oracle

Cerner

is

a

challenger .

I

couldn't

tell

you

yet,

I'm

not

smart

enough

or

not

close

enough

to

say

whether

Oracle

is

a

worthy

challenger

in

this

space

or

not,

but

they

are

certainly

trying

to
make
inroads
from
an
EHR
perspective.

So
I
feel
like
that
the
EHR
install
base
that
they
have
is
pretty
stab
le,
as
it
anyways
mostly
is
in
mature
markets
like
this.

And

I

think

actually

if

we're

able

to

modernize

the

EHR

rapidly ,

we'll

be

able

to

perhaps

even

capture

market

share

further .

Second,

if

you

look

at

the

other

big

growth

opportunities

in

the
RCM
stack,
the
RCM
part
of
the
platform
system
of
action
and
honestly
Keyur
really
there,
it
is
really
like
MEDITECH,
for
example,
does
not
really
have
an
RCM
offering
at
all.

In
fact,
prior
to
this
transaction,
MEDITECH
was
having
conversations
with
us
about
whether
we
should
combine
our
RCM
capabilities
and
our
RCM
part
of
the
platform
system
of
action
with
their
EHR.

So
they
don't
have
an
RCM
offering.
Oracle
has
formally
exited
the
RCM
space
after
trying
it
twice.

And
so
what
you
have
is
what
I
call
the
sort
of
pure-play
BPO
vendors,

if
you
would,
Med-Metrix
and
CorroHealth,
that
are
trying
to
focus
on
this
small-to-mid
rural
hospital
market.
And
so
I
would
say
that
at
least
in
the
700-odd
hospitals
where
the
TruBridge
EHR

is
installed,
it
creates
a
very,
very
defensible
and
competitive
right
to
win
for
TruBridge.

And
I
think
you
also
asked
what
is
the
penetration
of
TruBridge
in
their
current
EHR
install

base
from
an
RCM
perspective.
And
in
that
current
install
base
like
I
was
saying
of
a
650
million
they're
about\$220
million,
so
about
30%.
So
about
230-240
hospitals
of
their
700
EHR

install

are

today

buying

RCM

from

them.

So

it

creates

a

pretty

large

white

space

from

RCM.

So

in

those

700,

the

right

to

win

is

dramatic,

which

is

the

lowest-hanging

fruit

growth
opportunity
for
us.
And
where
IKS
will
further
bolster
their
right
to
win
in
that
install
base
is
that
we
have
a
much
more
efficient
global
delivery
model.

TruBridge
has
done
its

own

attempts

to

build

its

own

captive

presence

in

India,

but

has

really

not

had

that

much

success.

And

one

of

the

other

complementary

natures

of

this

thesis

is

the

fact

that

IKS

already
has
an
optimized
offshore-enabled
delivery
model
for
the
human-in-the-loop
part
of
RCM.

So
I
think
in
that
RCM
stack,
the
right
to
win
in
the
balance
500-odd
hospitals
where
the
RCM

doesn't
exist
is
dramatic.
And
then
even
beyond
the
700,
which
is
another
1,500
hospitals
in
the
rural
setting
where
TruBridge
already
has
300
customers,
400
customers,
there
is
an
opportunity
to
actually

go

full-stack

RCM

there

even

if

the

EHR

is

not

installed.

So

I

think

tremendous

rights

to

win

there.

As

it

relates

to

encoder ,

which

is

their

TruCode

business,

there

are

two

very
large
players,
3M
and
Optum,
but
given
how
large
they
are,
they've
actually
become
sort
of
aircraft
carriers.
So
I
will
say
that
in
the
encoder
business,
you
can
think
of
TruCode

as
a
challenger
rather
than
the
incumbent.

But
what
we've
seen
is
over
the
last
2
to
3
years,
there's
been
tremendous
growth
in
that
business
and
including
IKS,
for
example,
using
TruCode.

So

I

think

given

that

many

companies

tried

to

completely

autonomize

medical

coding

and

failed

when

they

realized

that

it's

a

deterministic

outcome

that

GenAI

really

can't

service

by

itself.

I

think

the

encoder

or

the

TruCode

opportunity

is

significant.

On

clinical

documentation,

obviously ,

if

this

transaction

closes

as

you

can

imagine

we

would

manifest

our

entire

Scribble

suite

in

their

entire

install

base

and

because

of
their
EHR
incumbency ,
their
take
on
that
is
just
a
no-brainer ,
I
mean
the
customers
are
dying
for
it.
And
we
will
install
it
in
a
way
where
it's
not
just
a

post-visit

ambient

clinical

documentation

but

it's

a

true

co-pilot,

like

I

was

saying

earlier .

And

then

on

the

patient

engagement

side,

while

they

have

good

technology

and

by

the

way

Phreesia

has

been

generally

struggling

in

the

market.

Our

patient

engagement

is

already

winning

against

Phreesia

and

others

in

the

marketplace.

It's

actually

one

of

the

most

successful

parts

of

our

platform

system

of

action

lately ,

especially

as

we

rolled

out

our

latest

version,

which

is

an

AI-first

and

which

consists

of

a

patent-pending

technology

as

well.

And

so

I

do

think

that

there

is

an

IKS

offering.

So
think
about
it
as
in
the
clinical
documentation
and
patient
engagement
the
IKS
offering
will
bolster
their
right
to
win
dramatically .
EHR
continues
to
have
a
right
to
win
which
we
will

mature,
encoder
they
are
a
challenger
to
the
market,
but
they
are
gaining
momentum
and
RCM
we'll
really ,
really
have
a
tremendous
right
to
win
in
their
EHR
install
base
and
IKS's
combination

with

them

will

enhance

their

right

to

win

in

even

their

non-EHR

install

base.

Hope

this

answers

your

question,

Keyur .

Keyur

Ladhawala:

Yes,

it

does.

Thanks,

Sachin.

And

secondly

so

while

you've

laid

out
the
rationale
of
the
merger
and
the
value
creation
opportunities
quite
well,
could
you
also
quantify
the
syner gies
that
on
the
basis
of
your
initial
workings
are
likely
to
flow
in?
Sachin

Gupta:

So,

that

is

the

one

thing

I

don't

want

to

do,

Keyur ,

right?

I

mean,

the

reason

why

I

put

out

a

4-year

True

North

is

because

what

I

don't

want

to

do

is,

look,

it

is

a

strategic

transformative

acquisition.

It

will

have

short-term

learnings,

surprises.

I

can

tell

you

that

if

you

look

at

my

track

record

and

our

company's

track

record

and

our
management's
track
record
as
a
public
company
over
the
last
five
quarters
that
we've
declared.

As
a
company
even
before
we
went
public,
we
try
to
be
in
the
space
where
we

like

to

under -promise

and

over-deliver .

And

hence

we

intentionally

took

this

strategy

of

putting

a

4-year

True

North,

but

I

will

tell

you

that

when

we

put

a

4-year

True

North,

we

factored

in
a
certain
amount
of
uncertainty
in
the
short
term,
learnings
in
the
short
term,
mistakes
perhaps
new
mistakes,
hopefully
not
the
same
mistakes
in
the
short
term.
And
we
still
feel
like

this
4-year
True
North
is
what
we
feel
that
will
be
a
good
definition
of
success
for
us.
I
really
would
like
to
avoid
going
into
the
specifics
of
the
journey
from
INR1,000

crores

to

INR3,000

crores

in

terms

of

what

are

the

key

drivers.

I've

given

you

the

growth

vectors,

you

understand

already

that

this

is

a

40-year -old

company

with

a

very

large

G&A.

There'll

be

syner gies

in

G&A,

they

still

have

a

very

largely

US-centric

RCM

delivery

model.

Remember

they

have

2,000-plus

RCM

FTEs

between

actually

3,000

RCM

FTEs

between

US

and

India

already

of

which

1,000-plus

are
in
the
US.
So,
obviously
that
creates
both
operating
and
G&A
syner gies,
but
again
let's
not
go
into
the
specifics.

Give
me
what
I
ask
of
all
of
you
is
this
is

a
strategic
transformation.

This
is
creating
a
long-term
defensible
strategic
moat
with

a
lot
of
thought
and
carefulness.

I've
put
a
4-year
True
North,
very
hard
for
me
to
give
you
even
further

details

even

before

the

transaction

has

closed.

Keyur

Ladhawala:

Understood.

No,

fair

Sachin.

Thanks

a

lot

and

wish

you

all

the

best.

Sachin

Gupta:

Thank

you,

Moderator:

Thank

you.

We

will

take

the

next

question

from

the

line

of

Aashray

Vasa

from

Nippon

AIF.

Please

go

ahead.

Aashray

Vasa:

Hey,

hi,

hi

morning.

First

of

all,

thanks

for

explaining

the

strategic

rationale

very,

very

clear .

Two

questions.

First

on,

I

mean

our

track

record

with

AQuity

suggests

we

are

very

strong

at

turning

around,

you

know ,

slightly

low-mar gin

businesses,

but

my

question

is,

how

difficult

is

it

to

do

what

TruBridge

does

organically?

I

mean

in

the

sense

of

access

to

rural

etc.,

the

EHR

etc.

Just

trying

to

understand

organically

does

it

take

too

much

time?

Our

four-year

plan

etc.,

is

it
just
a
thing
that
the
opportunity
came
and
we
have
taken
it?
That's
the
first
one.
And
the
second
one
is
on
TruBridge
itself.
Obviously
the
valuation
multiple
seems
favorable
for
us

in
the
sense
their
market
cap
etc.,
it's
been
struggling
slightly
in
the
last
year
from
a
market
cap
perspective.
6%
CAGR
over
five
years,
1.3%
last
year
revenue
growth,
and
there
are

some

when

I

read

about

the

recent

results,

there

are

some

issues

with

regards

to

retention

of

customers,

contracts,

some

you

know ,

guidance

that's

on

the

revenue

front.

So

just

trying

to

understand

the
rationale
from
a
seller's
point
of
view?
Yes,
that's
it.
Thanks.
Sachin
Gupta:
Okay ,
great.
Thank
you
for
those
questions,
Aashray .
I
guess
the
first
part
of
the
question
was
to
put

it
simply ,
the
timing
of
the
syner gies.

And
as
you
can
imagine,
we're
not
going
to
provide
the
details
of
when
the
timing
plays
out.

We've
provided
a
four-year
timeline.

And
so
as

you
can
imagine,
there'll
be
some
syner gies
that
are
realized
very
quickly ,
right?
We
talked
about
significant
syner gies
of
being
a
public
company ,
I
mean
it's
a
pretty
significantly
large
number .

Syner gies

around

ESOP

costs,

other

G&A

syner gies

that'll

get

realized

easily

within

sort

of

the

first

year

of

operations.

Then

there

is

the

nature

of

syner gies

that

are

operational,

right?

Like

I

said,

there's

a

huge

RCM

transformation

opportunity

both

from

a

tech

and

globalization

perspective.

Now

those

syner gies

obviously

take

some

time

because

you

have

to

work

with

the

customers

to

transform

the

customers,

first

get

the
customers
comfortable
about
how
they
will
get
transformed
from
a
delivery
model
perspective
and
then
walk
the
customers
through
that
journey ,
hold
their
hand
through
that
journey .

So
I
would
say
that

the

G&A

syner gies

will

probably

come

in

ahead

of

the

operational

syner gies.

But

I

feel

like,

you

know ,

over

a

two

and

a

half

to

three

year

period

all

the

syner gies

should

be

fully
tucked
in.
And
that's
probably
as
much
as
I
can
say
about
the
timing
of
the
syner gies.

As
it
relates
to
TruBridge's
own
performance
and
the
multiple
that
they
are
commanding

in
the
market
or
not
commanding
in
the
market,
look,
I
think
the
simple
reality
is
that
like
I
said
earlier ,
t
hey
have
tried
two
attempts
in
the
past
at
globalizing
their

RCM

operations.

And

they

have

been

less

than

successful

at

those

two

attempts.

So

one

of

the

complementary

natures

of

this

transaction

was

that

IKS

already

has

I

won't

necessarily

call

it

a

perfect
but
a
highly
optimized
onshore-of fshore
delivery
model
as
it
relates
to
RCM
and
significant
tech
interventions
in
addition
to
the
tech
interventions
they've
already
built.
And
so
I
think
that
was
a

very
attractive
aspect
of
the
thesis,
if
you
would,
was
that
the
one
area
where
they
have
struggled
in
leveraging
a
globalized
model
is
a
strong
suite
of
IKS.

The
other
challenge
that

they

had

is

because

they

were

struggling

in

that

area,

you

know ,

their

ability

to

invest

significantly

in

what

needs

to

happen

in

the

EHR

from

a

modernization

perspective,

building

other

features

of

the
platform
system
of
action
like
clinical
documentation,
became
a
strain
and
a
drain
on
their
financials.
And
obviously
with
IKS
coming
in,
one
we
already
have
those
features,
second,
we
have
a

much

more

agile

as

well

as

cost-optimized

model

to

build

technology

rapidly

as

well.

So

I

think

all

of

those

together

really

were

becoming

important

parts

of

the

combination

and

that's

where

the
arbitrage
in
the
multiple
lies,
is
that,
you
know ,
one
of
the
things
that
we
said
when
we
did
AQuity
was
we're
starting
off
as
24%
pro
forma,
we'll
get
to
early

to
mid-30s
in
we
had
said
at
that
point
between
two
to
three
years.
We
actually
got
there
faster
than
we
had
thought
and
we
actually
got
further
than
where
we
thought
we

were

going

to

get.

So

I

feel

very

similar ,

you're

going

to

start

off

with

26%

combined

pro

forma

here

and,

you

know ,

we

feel

like

we'll

be

in

the

early

30s,

you

know ,
relatively
rapidly .
And
that's
how
the
syner gies
will
play
out
and
that's
what
makes
it
attractive.

Now
why
did
they
decide
to
sell?
Because
I
think
they've
realized
that
these
core

competencies

are

going

to

be

critical

and

they

felt

like

finding

a

partner

that

is

able

to

bring

these

capabilities

natively

to

them

is

a

much

better

idea

than

continuing

to

try

this

to
try
to
do
this
a
third
time
after
having
tried
to
do
it
twice.
They
tried
to
first
do
it
through
an
acquisition
of
a
company
called
Viewgol,
which
didn't
work
very

well,
and
then
second
they
tried
to
do
it
in
a
captive
way
with
using
consultants
that
were
helping
them
and
that
didn't
go
well
either .
So
that's
sort
of
the
way
to

think

about

it.

Aashray

Vasa:

Got

it,

perfect.

Just

on

the

first

one,

so

organically

it

would

have

taken

a

lot

of

time

and

effort

to

get

access

to

the

rural

hospitals,

physicians,

etc.?

Just

trying

to

understand,

yes.

Sachin

Gupta:

I'm

sorry ,

were

you

saying

that

our

ability

to

drive

growth

in

their

install

base?

Aashray

Vasa:

No,

no,

I'm

saying

without

TruBridge

could

we

have
just
as
an
organic
entity
got
access
into,
yes,
the
rural
part
of,
yes?
Sachin
Gupta:
No,
no,
even
if
we
would
have,
we
would
not
have
gotten
access
first
of
all,

you're
talking
about
700
hospitals
in
their
customer
base.
To
sell
across
700
hospitals
would
be
a
decade,
right,
number
one.
And
number
two,
the
strategic
thesis,
where
we
would
be
the
only

company

in

the

world

that's

truly

building

an

integrated

system

of

record

with

a

platform

system

of

action

which

is

super

defensible,

that

also

made

it

tremendously

attractive.

So

yes,

I

think

doing

this

organically

would

be

next

to

impossible.

Aashray

Vasa:

Perfect,

perfect.

Thank

you

so

much,

all

the

best.

Sachin

Gupta:

Thank

you.

Moderator:

Thank

you.

We

will

take

the

next

question

from

the

line

of

Vishnu

Gopal

from

Marcellus

Investment

Managers.

Please

go

ahead

Vishnu

Gopal:

Yes.

So

congrats

on

the

acquisition.

And

I

just

had

a

question

regarding

the

nature

of

the

client.

So

just

wanted
to
understand,
like,
what
are
the
nuances
of
the
RCM
management
between,
let's
say,
a
larger
hospital
system
versus
the
rural
healthcare
system?
That's
one.
And
the
second
part
of
the
question

was,
I
mean,
when
the
AQuity
acquisition
was
consummated,
you
had
mentioned
that
the
long-tail
clients
were
kind
of
getting
rationalized
to
focus
on
the
larger
guys.
So
but
in
the
rural
healthcare

system,

by

nature

all

the

clients

are

smaller

in

size.

So

how

do

you

look

at

rationalization

or

the

lack

of

it

in

those

client

base?

And

does

it

also

signal

even

in

the
organic
business
a
readiness
to
focus
on
the
smaller
clients
going
ahead?
Thank
you.
Sachin
Gupta:
Great,
great
questions,
Vishnu.
Thank
you
so
much.
So
I
think
let
me
take
the
second

part
of
your
question
first,
which
is
our
ability
to
handle
a
large
number
of
small
customers.
And
as
you
know ,
there
are
already
700
hospitals
in
their
EHR
install
base,
and
most

of
the
RCM
is
delivered
to
customers
in
their
EHR
install
base.
Now
while
IKS
does
not
have
a
traditional
track
record
of
handling
a
large
number
of
small
customers,
but
we
like

handling

a

small

number

of

large

customers,

Vishnu,

the

difference

here

is

this:

all

of

the

RCM

customers

are

operating

on

the

same

operating

system,

which

is

their

EHR.

So

what

happens

is
even
though
the
number
of
customers
is
large,
the
type
of
work
that
you're
doing
the
RCM
for,
which
is
all
rural
hospital
care
delivery ,
which
is
very
similar
to
physician
care

delivery

by

the

way,

the

type

of

work

is

the

same.

And

second,

all

of

it

is

being

done

in

their

one

EHR.

Now

in

the

traditional

IKS

business,

when

we

have

the

small
number
of
large
customer
thesis,
that
is
because
what
happens,
Vishnu,
is
there
each
of
the
customers
often
is
in
a
different
system,
which
is
the
system
of
record
is
different,
so

we

have

to

integrate

with

a

different

system

of

record

because

we

are

Switzerland,

right?

And

the

type

of

care

delivery

that

they

are

doing

is

different,

right?

We

have

single

specialty

customers.

Within

a

single

specialty ,

there's

derm,

there's

GI,

there's

urology ,

there's

ophthalmology ,

all

of

that,

right?

So

each

of

those

is

different

in

nature.

Then

there's

multi-specialty ,

then

there's

hospital-owned

medical

groups

that
have
a
totally
different
orientation.

So
the
beauty
here
is
the
type
of
work
that
we're
doing
RCM
for,
number
one,
and
the
system
on
which
we're
doing
RCM
is
all
one,

which
allows
us
for
80%
of
the
work
to
treat
this
as
one
large
customer
and
drive
a
lot
of
efficiencies.
So
even
though
there
are
300
small
RCM
customers,
they're
all
on

the
same
system
with
the
exact
same
type
of
service
delivery ,
which
is
rural
healthcare
of
which
70%-80%
is
outpatient.
It
allows
us
to
build
that
standardized
engine
and
treat
them
as
one

large

cust
omer .

Now

for

the

balance

20%,

yes,

we

have

to

create

a

slightly

differentiated

customer

management

layer

where

each

of

the

customers

will

feel

like

they

are

being

treated

uniquely ,

and

that
investment
for
managing
the
customers
differently
has
already
been
factored
into
our
plan.

But
I
think
it's
very
important,
I
think
your
question
is
brilliant
to
know
that
why
does
IKS

feel
comfortable
in
managing
a
large
number
of
small
customers
in
the
TruBridge
customer
base
when
traditionally
in
the
IKS
business
or
even
in
the
AQuity
business,
we
were
not
comfortable
handling
a
large

number
of
small
customers.
But
I
hope
this
clarifies,
because
it's
a
very,
very
important
distinction.
Vishnu
Gopal:
Sure,
this
clarifies.
And
just
on
the
nuances
of
the
RCM
business
between
larger
hospital

systems

and

the

rural

hospital

systems.

Sachin

Gupta:

Yes.

So

clearly ,

like

I

was

saying,

these

rural

hospital

systems,

even

though

they

are

hospitals,

70%

of

the

care

being

delivered

is

outpatient

care,

which

is

IKS'

traditional

strong

suite,

right?

Physician

delivery ,

physician-based

outpatient

care

is

IKS'

strong

suite.

The

balance

30%

also,

if

you

really

think

about

it,

is

critical

care

but

there

is

no

really
tertiary ,
quaternary
care,
specialty
surgeries,
complex
surgeries.

Those
type
of
infrastructure
doesn't
exist
in
rural
settings
at
all.

That's
where
the
RCM
for
tertiary ,
quaternary
care
can
get
a
little
bit

more
complex.
That's
why
when
we
analyzed
the
RCM
capabilities
needed
to
do
justice
to
these
rural
hospital
settings,
they
are
very
proximate
from
a
capability
perspective
to
the
strengths
that
we
already

have
and
that's
how
we
were
comfortable.

Nithya
Balasubramanian:

And
it's
also
important
to
note,
I
think
Sachin
mentioned
this
earlier ,
that
for
a
larger
hospital
system,
because
of
the
scale,
what
they

end

up

spending

on

RCM

is

somewhere

in

the

zip

code

of

4%

to

5%.

That's

almost

double

in

a

rural

community

hospital.

They

end

up

spending

somewhere

in

the

zip

code

of

7%

to

10%

on

RCM.

So

the

value

proposition

from

outsourcing

becomes

even

more

stronger

for

these

smaller

hospitals.

Vishnu

Gopal:

Great,

great.

Thank

you

and

wish

you

all

the

best.

Sachin

Gupta:

Thank

you,

Vishnu.

Nithya

Balasubramanian:

Thank

you.

Moderator:

Thank

you.

We

will

take

the

next

question

from

the

line

of

Chirag

Kachhadiya

from

Motilal

Oswal

Financial

Services.

Please

go

ahead.

Chirag

Kachhadiya:

Yes,

hi,

Sachin.

I

have

one

question.

As

you

mentioned,

the

integration

will

take

four

to

five

quarters'

timeframe.

Can

you

specify

the

steps

or

actions

you

will

take

to,

you

know ,

integrate

the

operations

of

this

TruBridge?

Sachin

Gupta:

Okay .

So,

look,

I

think

there's

obviously

a

significant

amount

of

work

involved

in

the

integration,

right?

And

so

the

first

aspect

of

the

integration

is

to

learn.

And

given

that

we

are

both

public

companies

and

from

an

FTC

perspective,

the

Federal

Trade

Commission

perspective,

we

really

can't

do

too

much

work

prior

to

the

close

of

the

transaction.

So
the
first
two,
three
months
will
be
to
really
understand
the
operating
nuances
of
their
structure
and
work
with
them
together
to
arrive
at
what
a
combined
operating
structure
would
look
like.

That's

the

first

thing.

Then

maybe

by

Q2

post-close,

we

will

start

embarking

on

making

our

journey

towards

that

new

operating

structure,

start

to

realize

some

of

the

G&A

benefits

of

a

combined
operating
structure,
and
slowly
but
surely
start
to
embark
on
the
operational
transformation.
Probably ,
it'll
be
early
Q3
by
the
time
we
start
the
operational
transformation.
Because
we'll
also,
you
know ,
a
big

part
of
the
first
two
quarters
will
be
meeting
with
their
customer
base,
understanding
the
nuances
of
the
customer
base.

It's
always
fun
to
have
an
outside-in
view .
But
when
you
dive

in,
the
inside-out
view
becomes
much
more
informative
and
instructive
about
the
nuances
that
have
to
be
applied
in
how
you
do
this.
And
so
I
would
say
Q1
post-close
learning,
Q2
actioning,

start

to

combine

into

one

organization

structure,

start

to

realize

G&A,

really

start

to

understand

the

customer

base,

and

perhaps

towards

the

end

of

Q2,

early

Q3

post-close

is

when

we

start

actioning

the
operational
transformation.

There's
also
a
lot
of
work
to
be
done
in
understanding
the
next
level
details
of
the
tech
debt,
you
know ,
what
it
will
take
to
overcome
the
tech
debt

because

a

big

vector

of

this

will

be

to

modernize

the

EHR

and

move

it

to

a

true

cloud-native

AI-first

model.

So

there's

the

tech

piece,

yes.

And

we

do

all

this

in

a
way
where
we
keep
a
keen
focus
on
one,
customer
retention.
Customer
retention
is
key.
That's
the
gold
mine
of
this
transaction.

Second,
talent
retention.

The
key
talent
will
have
to

be
identified
in
the
first
quarter
and
retaining
the
talent
will
be
critical.
And
third,
as
we
embark
on
our
transformation,
how
do
we
keep
a
keen
eye
on
energizing
growth
again?
So

I
think
those
would
be
the
three
key
vectors
of
or
three
key
considerations
that
we
will
keep
in
mind
as
we
embark
on
the
integration
effort.
Chirag
Kachhadiya:
Okay .
And
Sachin,
if

we
look
at
the
employee
--
total
number
of
employee
headcount
in
TruBridge
versus
our
entity ,
I
mean,
and
if
we
compare
it
with
the
overall
revenue
which
both
these
entities
make,
I

mean,
there's
a
vast
difference.
So
why
is
that?
Sachin
Gupta:
Simply
because
that's
what
I
said,
right?
The
EHR
business
is
a
pure
tech
business.
So
that
really
has
a
very,
very

low
marginal
cost.
And
even
the
RCM
business,
like
I
said,
is
highly
tech-led.
And
so
that's
really
where
the
magic
is,
right?
And
this
is
even
before
they've
made
it
truly
AI-first.

And

so

that's

what

I

keep

saying

is

having

that

system

of

record

combined

right

now

with

the

platform

system

of

action

for

RCM

and

eventually

all

the

systems

creates

a

fundamentally

different

leverage

in

how

much

human-in-the-loop

you

need

to

be

able

to

orchestrate

these

actions,

and

that's

what

you

see

as

evidence

in

their

numbers.

They

are

a

tech

organization.

Chirag

Kachadia:

Okay ,

all

the
best.
Sachin
Gupta:
Thank
you,
Chirag.

Sachin
Gupta:
Let's
take
the
next
question.

We'll
come
back
to
Vamshi
when
he's
able
to.

Moderator:
Thank
you.
We
will
take
the
next
question

from

the

line

of

Abhishek

Gupta

from

Axis

Asset

Management

Company .

Please

go

ahead.

Abhishek

Gupta:

Good

morning,

Sachin

sir.

Thank

you

so

much

for

the

explanation

on

the

acquisition

and

the

presentations.

So

sir,
just
to
start
with,
you
know ,
just
give
me
an
example
like
how
does
this
EHR
capabilities
which
you
are
acquiring
differentiate
from
the
other
EHR
bodies
like
Epic
and
all,
and

how

are

you

going

to

take

these

capabilities

towards

the

bigger

hospital

and

all?

Sachin

Gupta:

So

actually ,

Abhishek,

that

is

not

our

intent

at

all.

We

are

not

trying

to

take

this

EHR
capability
to
the
bigger
hospitals.
What
we're
doing
is
we
are
taking
the
EHR
that
is
already
installed
in
the
small
hospital
base
and
we're
going
to
deeply
integrate
our
platform
system

of
action
with
the
EHR
to
make
it
a
combination
that
becomes
highly
sticky
and
defensible
and
both
financially
advantageous
to
the
customers
and
to
us
over
a
period
of
time.

We

are
not
envisioning
taking
this
EHR
to
large
hospitals.
The
only
endeavor
will
be
once
we
modernize
their
EHR,
we
will
attempt
to
take
it
from
the
0
to
50
beds,
which
is

where
they're
strong,
to
the
next
segment,
which
is
sort
of
the
50
to
200
beds.
But
the
idea
is
not
to
take
it
to
the
large
hospital
segment
at
all.

In
fact,

in
that
segment,
we
don't
even
want
to
be
known
as
an
EHR
player
at
all
because
there
we
integrate
our
platform
system
of
action
with
the
prevalent
EHRs
in
that
market,
so

there
is
no
question
of
trying
to
drive
that
EHR
in
that
market.
Abhishek
Gupta:
Sir,
last
question
from
my
side
is
just
that
we
--
as
we
have
always
said
that
we

will

grow

faster

than

the

industry .

And

as

the

both

companies

get

combined

are

we

still

maintaining

that

guidance?

Sachin

Gupta:

Okay ,

so

I

think

somebody

has

background.

See

on

the

growth,

like

I
said,
I
need
two-three
quarters
to
better
understand
this
market
once
it
closes
to
know
what
exactly
the
growth
trajectory
will
look
like
in
the
rural
healthcare
market.
We're
not
changing
our

aspirations

in

our

traditional

market

segment

at

all.

That

market

segment,

which

is

the

physician

group

market

segment,

we

are

continuing

to

say

we

will

want

to

continue

to

capture

market

share,

which

means
we'll
grow
faster
than
the
12%
that
the
industry
is
growing
at.
In
this
market,
I
think
and
that's
why
I've
put
out
a
four-year
true
north
on
the
earning
side,
because

I
don't
want
to
make
the
mistake
of
prematurely
being
able
to
tell
what
the
growth
trajectory
will
look
like.
I'm
very
confident
or
rather
fairly
confident
on
where
I
think
the
margin

trajectory

of

the

business

will

go.

But

on

the

growth,

I

want

to

wrestle

with

the

growth

for

two-three

quarters

after

close

and

then

I'll

have

a

better

understanding

of

what

the

growth

will

look

like

in

the

TruBridge

traditional

rural

healthcare.

Moderator:

We

will

take

the

next

question

from

the

line

of

Madhuchanda

Dey

from

MC

Pro.

Madhuchanda

Dey:

I

have

some

very

fundamental

questions,

pardon

me

for

my

lack

of

understanding

of

this

acquisition.

So

the

first

quest

ion

is,

as

you

mentioned,

that

the

first

low-hanging

fruit

for

you

would

be

to

take

the

RCM

to

the
hospitals
where
they
are
not
present
but
present
with
the
EHR.
And
the
second
stage
would
be
to
take
it
to
the
larger
audience
where
they
are
not
present
at
all
but

within
that
range
of
smaller
hospitals.
So
my
question
is,
what
makes
you
confident
that
your
RCM
solutions
will
be
superior
to
the
competitor
or
put
it
differently ,
what
does
this
acquisition
bring

to
the
RCM
solution
which
was
not
there?
Is
it
something
to
do
with
technology ,
something
to
do
with
cost?
If
you
could
just
shed
light
on
that?
Sachin
Gupta:
Yes,
so
look,
I

think
in
the
install
base
where
they
already
have
the
EHR,
when
we
integrate
our
RCM
stack
deeply
with
the
EHR
for
example,
if
you
like
I
was
calling
out
the
denial
prediction

engine,
right?
Now
what
is
RCM?
In
the
end,
you
send
claims
for
the
care
that
was
provided
to
the
insurer ,
the
insurer
pays
you
for
the
claims
or
denies
the
claims.
If

they
deny
the
claims,
you
have
to
work
on
those
claims
and
still
try
to
get
them
collected
by
providing
supporting
documentation,
changing
coding,
whatever
it
is
right?
And
so
it's
a
very

reactive

cycle.

Now

if

you're

deeply

integrated

in

the

EHR

on

a

real-time

basis

at

the

point

of

charge

entry ,

at

the

point

of

clinical

documentation,

if

you

are

able

to

predict

through

this
AI
training
corpus
that
we
would
have
created
by
in
a
compliant
way
leveraging
the
data
in
the
EHR,
which
has
both
the
clinical
context
but
also
the
labelling
of
the
actions

taken
and
the
outcomes
achieved
from
those
actions,
you
will
now
start
to
predict
denials.
If
you
predict
denials
correctly
and
you
fix
them
at
the
point
of
care
itself,
then
when
the

claim
goes
out,
it
doesn't
get
denied
at
all.
And
fundamentally
that
transforms
the
RCM
outcomes.
Also,
it
reduces
the
cost
of
the
RCM
because
you
don't
have
to
do
that
much
effort

at
the
back
end,
right?
So
I
think
it
totally
transforms
both
the
outcomes
and
the
cost
of
the
RCM,
which
if
you
do
not
have
deep
integration
into
the
system
of
record,

you

are

not

able

to

do

those

things

on

a

real-time

basis

even

if

you

were

able

to

produce

those

insights

asynchronously .

So

I

think

that

would

be

one

clear

example

of

an

RCM
intervention
through
this
integration
that
fundamentally
transforms
both
outcomes
of
RCM
and
cost
of
outcomes.
There
are
obviously
several
other
interventions
like
those
that
come
into
play,
including
but
not
limited
to

the
real-time
coding
interventions.

The
real-time
clinical
documentation
interventions,

the
whole
process
of
prior
authorization

within
the
revenue

cycle
that
often
leads

to
a
whole
bunch
of
uncompensated
care.

So
a
bunch
of

examples

like

that.

Really

good

question

and

hopefully

this

example

gives

you

a

sense

of

why

our

right

to

win

in

the

TruBridge

EHR

customer

base

through

the

deep

EHR

integration

of

our

RCM

stack

fundamentally

transforms

outcomes

and

cost.

Madhuchanda

Dey:

But

my

question

is,

this

TruBridge

is

already

a

technology

company ,

why

could

they

not

do

it

on

their

own?

Sachin

Gupta:

Yes,

and

I
think
that's
where
I
was
trying
to
go
with
this
earlier
is
that,
remember ,
in
this
platform
system
of
action,
it
needs
tech
but
it
also
needs
human-in-the-loop.
All
the
human-in-the-loop
that

they've
generally
had
has
been
in
the
US.
And
that
creates
a
different
type
of
a
cost
structure.
And
it
really
needs
globalized
human-in-the-loop
to
successfully
orchestrate
this
becau
se
remember ,
some
of

these

interventions

are

not

autonomous

from

a

technology

perspective.

It's

tech

plus

human-in-the-loop.

And

the

human-in-the-loop

TruBridge

in

all

candour

has

not

been

able

to

execute

successfully

from

a

globalized

human-in-the-loop

perspective,

which

IKS

has

the

expertise

around

and

that

we

will

bring

again

as

a

complementary

strength

to

the

process.

Madhuchanda

Dey:

So

it's

basically

your

global

delivery

model

that

will

be

a

great

help

to

them,

right?

To

this

entity .

The

last

question

that

I

have

is

that

you

are

already

a

user

of

TruCode

as

you

mentioned,

as

IKS,

right?

So

what

value

do

you

add

to

TruCode

through

this

acquisition?

I

mean,

because

you

mentioned

something

like

a

\$650

million

kind

of

opportunity .

Sachin

Gupta:

Yes,

so

look,

first

of

all,

what

we

bring

is

yes,

can

you

hear

me,

ma'am?

Madhuchanda

Dey:

Yes,

sure.

Yes.

Sachin

Gupta:

Okay .

Yes,

so

the

way

to

think

about

it

is

one,

if

you

look

at

IKS's

traditional

customer

base,

it's

the

large

physician

groups

and
the
large
health
systems.

TruCode
is
a
very
applicable
product
for
all
of
those.

So
the
value
we
bring
to
TruCode
is
an
access
to
that
market
that
TruBridge
never
had
access

to,
right?
So
it
allows
us
to
bring
TruCode
as
an
offering
to
the
large
hospitals
and
the
large
health
systems
and
the
large
physician
groups,
which
TruCode
never
had
access
to.
That's

number

one.

And

the

second,

we

are

executing

coding

at

a

very

different

scale

than

TruBridge

does

themselves.

And

so

remember

TruCode

also

is

a

tool

that

continues

to

learn.

And

hence

all

of
the
work
that
we
are
doing
in
coding
will
now
become
a
virtuous
feedback
loop
into
TruCode
and
we
keep
enhancing
the
rules
that
are
embedded
in
TruCode
and
allow
it
to

assist

better

and

better

over

a

period

of

time.

So

two

things:

one,

the

market

of

the

large

systems

and

the

large

physician

groups.

And

two,

all

the

feedback

of

coding

at

a

different

scale

than

the

scale

that

TruBridge

has.

Moderator:

Thank

you.

We

will

take

the

next

question

from

the

line

of

Chetan

Shah

from

Jeet

Capital.

Please

go

ahead.

Chetan

Shah:

Yes,

hi.

Sachin
and
team,
congratulations
on
a
great
acquisition.

Just
one
specific
question.

Can
you
just
quantify
the
size
of
opportunity?

We
know
the
addressable
opportunity

for
IKS
is
\$200
billion
plus,
but
post

this
acquisition,
what's
the
size
of
the
cake
will
look
like?
That's
the
only
question
I
have.
Thank
you.
Sachin
Gupta:
So
you're
talking
about
what
is
the
additional
TAM
that
we
get

from

this

transaction?

Chetan

Shah:

Yes,

Sachin.

Yes,

yes.

Sachin

Gupta:

Yes.

Sure.

So,

like

I

said,

it's

a

combination,

right?

So

when

you

look

at

the

TruBridge

EHR

install

base,

the

TAM

opportunity

in

just

their

EHR

install

base

is

about

\$600

million

for

our

platform

system

of

action.

A

large

part

of

it

comes

from

RCM,

but

there

are

other

parts

of

our

system

of
action
that
also
would
get
installed
in
their
install
base
potentially .
So,
there's
at
least
a
\$600
million
opportunity
in
their
EHR
captive
install
base.

Now
when
you
expand
that
opportunity

into
their
non-EHR
RCM
customers,
that
multiplies
very
rapidly
because
like
I
said,
that
is
already
a
\$1
billion
outsourced
market
growing
at
12%
to
13%.
And
so,
depending
on
what
share
of

that
market
we
are
able
to
capture,
that
outsourced
market
itself
is
about
\$1billion
today ,

and
that
total
market
is
about
\$3.5
billion
to
\$4
billion.
So
that
becomes
another
market,
an
adjacent

market

in

the

rural

hospital

setting

that

we

can

sell

to.

So,

this

is

I

would

say

maybe

the

serviceable

obtainable

TAM

immediately ,

lowest-hanging

fruit

is

a

little

north

of

\$600

million,

and

then

there's

a

\$3

billion

plus

TAM

of

rural

hospitals

that

are

very

adjacent

that

could

also

be

TruBridge's

customers

but

not

EHR

customers,

that

also

becomes

part

of

the

TAM.

Chetan

Shah:

Yes,
thanks.
And
just
for
my
clarification,
our
existing
set
of
offering
which
we
do
traditional
opportunity ,
can
this
be
getting
integrated
into
our
newer
business
which
we
acquired
and
getting
it
merged,

and

will

that

expand

the

size

of

the

TAM

as

an

overall

offering,

maybe,

you

know ,

four

quarters

down

the

line

or

six

quarters

down

the

line

once

everything

gets

integrated?

I'm

just

trying

to

understand

that

apart

from

a

completely

new

vertical

as

a

business

opportunity ,

are

we

also

expanding

our

existing

offering

itself

as

a

size

of

TAM

so

that,

you

know ,

if

one

wants

to

look

at

the

business

from

a

decade

point

of

view

as

a

strength

and

opportunity?

Some

color

on

that.

Sachin

Gupta:

Yes,

the

way

to

think

about

that

would

be

that

our
platform
system
of
action
as
a
percentage
of
the
rural
hospital's
revenue
overall
would
be
at
least
about
5%
of
their
revenue.
So
if
you
really
think
about
it
in
the
TruBridge

install
base,
if
our
full
platform
system
of
action
were
to
be
installed,
that
itself
becomes
about
\$1.5
billion.
The
full
platform
manifest.

And
so
that's
when
if
you're
talking
about
our

capabilities

fully

manifesting

into

TruBridge's

install

base

even

outside

of

RCM,

that

becomes

about

a

\$1.5

billion

opportunity

in

their

EHR

install

base.

Moderator:

Thank

you.

We

will

take

the

next

question

from

the
line
of
Vamshi
Krishna
from
Kotak
Securities.
Please
go
ahead.
Vamshi
Krishna:
Hi,
thanks
for
the
opportunity
again
and
congrats
on
the
acquisition,
Sachin.
So
most
of
my
questions
on
the
business

have
been
answered.
Just
one
question
on
the
calculation
of
pro
forma
earnings.
I
think
TruBridge
also
had
around
\$12
million
impact
from
amortization
of
acquired
intangibles
related
to
the
acquisition
that
you

alluded

to.

So

why

has

that

not

been

included

in

the

calculation?

Saransh

Mundra:

No,

so

Vamshi,

as

you

know ,

right,

the

entire

thing

will

go

into

the

pot.

So,

if

you

look

at
their
schedule,
right,
their
schedule
was
on
a
reducing
trend
over
the
next
few
years.
What
will
happen
now
is
that
the
separate
parts
of
the
business,
which
is
the
EHR
business

and
within
the
financial
health,
maybe
the
coding
and
the
and
the
financial
health
business,
will
get
evaluated
separately
and
based
on
the
length
of
the
relationships
that
they've
had
with
their
customers,

a
new
asset
life
will
be
created,
right?

So,
while
you
see
historically
what
the
numbers
were,
it
was
on
a
declining
trend.

And
what
we've
now
put
is
sort
of
a
constant

number ,

and

we

can

discuss

that

in

more

detail.

But

as

you

know ,

right,

this

is

also

going

to

be

validated

again

by

valuation

experts.

But

that's

it

for

now.

Vamshi

Krish

na:

Okay .

So,

the

PAT

accretion

of

\$5

million,

so

is

there

a

revision

to

that

number

likely

given

the

higher

charge

and

that...

Saransh

Mundra:

There

might

be

minor

uptick

or

downtick.

But

again,
see,
we
haven't
taken
any
syner gies
in
that
PAT
accretion,
right?
I
mean,
our
interest
cost
we've
assumed
at
the
highest
possible
levels...
Vamshi
Krishna:
I
understand
that,
Saransh.
Just
the
\$12
million

number

is

a

little

large

compared

to

the

PAT

accretion

that

you

have

mentioned.

So

that's

where

I

was

just

wondering

what

the

number

could

have

been

for

FY

'27.

Saransh

Mundra:

We

don't

expect

that

number

to,

yes,

Vamshi,

we

don't

expect

that

number

to

move

significantly .

Vamshi

Krishna:

Understood.

Moderator:

Thank

you

very

much.

Ladies

and

gentlemen,

we

will

take

that

as

the

last

question

for

today .

For

any

further

queries,

you

may

reach

out

to

the

Investor

Relations

team

of

IKS

Health

Limited.

I

now

hand

the

conference

over

to

Mr.

Saransh

Mundra

for

closing

comments.

Over

to

you,

sir.

Saransh

Mundra:

Thank

you

everyone,

thank

you

for

joining

at

such

short

notice.

If

you

have

any

more

questions,

please

feel

free

to

reach

out.

My

details

are

there

on
the
Press
Release
and
most
of
you
have
my
contact.
Sachin
Gupta:
Yes,
and
thank
you
for
all
your
support
and
interest
so
far,
and
we're
excited
about
this
next
phase
of

journey
to
create
a
truly
differentiated
leader
in
now
two
segments
of
the
US
healthcare
market
and
one
that
has
probably
a
most
effective
defensible
and
competitive
moat
over
a
period
of
time.

Thank
you
everyone.

Bye-bye.

Moderator:

Thank
you
members
of
the
management.

On
behalf
of
ICICI
Securities
Limited,
that
concludes
this
conference.

Thank
you
all
for
joining
us
today
and
you
may
now
disconnect

your

lines.

Thank

you.

Please

note

that

this

transcript

has

been

edited

for

readability .

Call: May 2026

May 20, 2026

BSE Limited
The Listing Department
Phiroze Jeejeebhoy Towers
25th Floor, Dalal Street
Fort, Mumbai 400 001
Maharashtra, India

BSE Scrip Code: 544309
National Stock Exchange of India Limited
The Listing Department
Exchange Plaza, Plot No. C/1, G Block,
Bandra Kurla Complex
Bandra (East), Mumbai 400051
Maharashtra, India

NSE Symbol: IKS
Dear Sir/Ma'am,

Sub: Transcript of the Q 4 FY 2025 -26 Earnings Conference Call .

Pursuant to Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, please find enclosed the transcript of the earnings conference call held on Thursday , May 14, 2026, with respect to the Company's financial results for the quarter and year ended March 31, 2026 .

The said transcript has also been uploaded on the Company's website and can be accessed through the following link:

<https://ikshealth.com/ir/2026/q4/transcript -q4.pdf>

This is for your information and records.

Thanking you.

Yours sincerely,
For Inventurus Knowledge Solutions Limited

Sameer Chavan
Company Secretary and Compliance Officer
Membership No. F7211
Encl: As above

"Inventurus

Knowledge

Solutions

Limited

Q4

FY26

Earnings

Conference

Call"

May

14,

2026

M
ANAGEMENT
:

MR.

S
ACHIN

G
UPTA

—

F
OUNDER

A
ND

G
LOBAL

C
HIEF

E
XECUTIVE

O
FFICER

M
S
.

N
ITHYA

B
ALASUBRAMANIAN

—

W
HOLE
-T
IME

D
IRECTOR

&

C
HIEF

F
INANCIAL

O
FFICER

M
R
.

S
ARANSH

M
UNDRA

—

VP,

I
NVESTOR

R
ELATIONS

M
ODERA TOR
:

M
S
.

S
EEMA

N
AIK

—

ICICI

S
ECURITIES

Moderator:

Ladies
and
gentlemen,
good
day
and
welcome
to
the
IKS
Health
Q4
FY26
Earnings
Conference
Call
hosted
by
ICICI
Securities.
As
a
reminder ,
all
participant
lines
will
be
in
the
listen-only
mode
and

there
will
be
an
opportunity
for
you
to
ask
questions
after
the
presentation
concludes.
Should
you
need
assistance
during
this
conference
call,
please
signal
an
operator
by
pressing
star
then
zero
on
your

touch-tone

phone.

Please

note

that

this

conference

is

being

recorded.

I

now

hand

the

conference

over

to

Ms.

Seema

Naik

from

ICICI

Securities.

Thank

you

and

over

to

you,

ma'am.

Seema

Naik:

Good

morning,
ladies
and
gentlemen.
Thank
you
for
joining
us
today
on
the
Q4
FY26
Earnings
Call
of
IKS
Health.
On
behalf
of
ICICI
Securities,
I
would
like
to
thank
the
management
of
IKS

Health

for

giving

us

the

opportunity

to

host

this

call.

Today

we

have

with

us

Mr.

Sachin

Gupta,

Founder

and

CEO,

Ms.

Nithya

Balasubramanian,

CFO,

and

Mr.

Saransh

Mundra,

Head

of

Investor

Relations.

I
turn
it
over
to
Mr.
Saransh
for
his
brief
statement
and
to
take
the
proceedings
forward.
Thank
you
and
over
to
you,
Saransh.
Saransh
Mundra:
Thank
you,
Seema.
Good
morning
to

everyone

on

the

call.

Welcome

to

our

earnings

call

for

the

fourth

quarter

and

year

ended

31st

March

2026.

I

am

Saransh

Mundra,

VP

Finance.

We

hope

you

have

had

an

opportunity

to

review
the
earnings
release
and
the
investor
presentation
that
we
issued.
Before
I
hand
over
to
Sachin
and
Nithya,
let
me
begin
with
the
safe
harbor
statement.

As
part
of
our
prepared
remarks

and

during

Q&A,

we

may

make

certain

statements

which

are

forward-looking

and

involve

significant

uncertainty .

IKS

doesn't

take

any

responsibility

to

update

such

forward-looking

statements

and

your

discretion

is

warranted

while

making

any

investment

decisions.

Over

to

you,

Sachin.

Sachin

Gupta:

Thank

you,

Saransh.

Good

morning

and

good

evening

everyone,

depending

on

where

you're

joining

from.

It

is

my

pleasure

to

join

you

all

today

to

talk
about
our
performance
for
Q4
fiscal
'26
and
the
full
year
ending
December
31st,
2026.
I
think
this
is
our
sixth
earnings
call
since
we
went
public
in
December
2024.
So
excited

to
be
with
you
all
today .

We'll
start
off
with
a
quick
recap
obviously
of
the
overall
business
and
the
business
model.

Talk
about
some
of
the
key
strategic
drivers
for
success

in
our
business
and
give
you
a
little
update
on
how
those
are
faring.
And
then
talk
a
little
bit
specifically
about
how
we
are
leveraging
AI
most
effectively
to
actually
make
it

a
differentiated
moat
in
the
business
as
we
continue
to
grow .

And
then
dive
into
obviously
our
financial
performance
for
Q4
as
well
as
for
fiscal
2026.

And
then
invite
Nithya
to
make

some
remarks,
additional
remarks
on
our
financial
performance
and
we
will
then
turn
it
over
for
questions
and
discussion.

So
with
that,
obviously ,
as
you
know ,
IKS
is
in
the
business
of

delegating

chore

tasks

for

healthcare

providers

in

the

US.

It's

probably

the

largest

industry

in

the

world

at

\$5

trillion

plus

and

it

creates

a

very

large

TAM

of

north

of

\$260

billion

for
that
US
healthcare
is
spending
on
all
of
these
tasks
that
can
be
delegated
or
outsourced
by
our
platform.

Of
that
\$260
billion
TAM,
as
you
might
recall,
the
outsourced
TAM
today

is
about
\$35
billion.

And
the
overall
TAM

of
\$260
billion

is
growing
at

about
8%
and

the
outsourced
TAM

is
growing
faster

than
the
overall

TAM
at
about
12%.

And
so
just

to
recap,
anytime
IKS
grows
on
a
year-on-year
basis
for
any
period
faster
than
12%,
we're
gaining
market
share.
Of
course,
we
don't
want
to
be
in
a
situation
where
we're
not
growing

faster
than
12%
because
that
would
mean
losing
market
share,
but
in
general,
that
is
sort
of
a
good
marker
to
keep
in
mind.

Today ,
IKS
has
a
little
north
of
600
clients

or
client
organizations,
if
you
would,
which
are
provider
organizations.

You
might
recall
at
the
same
time
last
year,
that
number
was
700
plus
and
maybe
a
couple
of
years
ago
when
we

acquired

AQuity ,

that

number

was

closer

to

900.

As

we've

always

maintained,

we

have

been

cutting

down

the

tail

of

small

clients

that

we

had

inherited

through

AQuity

because

our

business

in

our
traditional
model
works
best
when
we're
servicing
large
clients.
And
so
that
number
of
600
odd
reflects
the
pairing
that
has
happened
over
the
last
year.

And
as
I've
always
stated,
we

feel

like

we'll

eventually

end

up

at

some

number

between

500

and

600.

So

we're

more

or

less

close

to

done

on

some

of

that

tail

cutting

that

we

had

embarked

on

as

we

acquired

AQuity .

Excitingly ,

90%

of

our

revenues

come

from

repeat

customers

and

we

have

a

very

healthy

vintage

of

our

top

10

and

top

5

customers

on

an

average

being

5

plus
years.
Our
headcount
as
of
March
31,
2026
is
13,331
people,
of
which
1,981
are
clinically
trained
staff.
Just
so
that
people
know ,
that
headcount
as
of
March
31,
2025
was
actually

a

bit

lower ,

but

it

was

about

5.3%

lower

at

12,661.

So,

our

headcount

has

grown

over

this

period

from

12,661

to

13,331,

which

is

about

a

5%

growth

in

headcount,

which

obviously

reflects
the
non-linearity
of
our
business
model
because
our
revenue
growth
is
significantly
higher
than
the
headcount
growth,
which
we'll
cover
in
our
Q4
financials
in
a
bit
more
detail.

Of
our

13,331

people,

the

fastest

growing

component

of

our

workforce

is

actually

our

technology

workforce

of

now

550

employees

that

is

focused

on

building

all

of

our

proprietary

technology

that

enables

our

platform.

And

a
large
part
of
that
workforce
now
tends
to
be
the
AI
engineering
workforce
in
addition
to
the
data
science
workforce.
And
then
we've
also
constantly
been
growing
our
sales
and
marketing
endeavors

and
our
investments
there
and
our
sales
and
marketing
headcount
now
stands
at
66
FTEs
and
growing.

Just
to
take
a
quick
recap
so
that
we
all
understand
the
competitive
landscape,
when
you

have
such
a
large
TAM
of
\$260
billion
and
the
outsourced
TAM
of
\$35-odd
billion
growing
at
12%,
you
naturally
expect
a
significant
amount
of
competition
in
the
ecosystem.
Just
to
recap
and

calibrate

our

language,

there

are

generally

three

broad

genres

of

competition

that

are

competing

for

this

TAM.

Those

three

broad

genres

are,

first,

what

I

call

system

of

record

companies,

which

are

nothing

but
electronic
health
record
vendors
that
were
created
back
in
the
early
2000s
as
the
US
government
gave
money
to
providers
to
roll
out
these
electronic
health
record
systems
that
one,
store
all

of
the
patient
data
for
the
care
that
is
being
provided
by
these
caregiving
organizations.

And
two,
they
have
some
workflows
as
it
relates
to
the
patient's
journey
with
the
caregiving
organization.
So,

call
them
the
core
operating
system
of
healthcare
providers.

Those
are
the
electronic
health
records.

Now ,
what
these
electronic
health
records
did
when
they
were
rolled
out
was
in
addition
to
becoming
patient

data
stores,
unfortunately ,
unintended
consequence,
they
made
these
healthcare
providers
and
the
administrators
of
these
healthcare
provider
organizations
almost
data
entry
operators.

So,
they
ended
up
having
to
start
spending
a
lot

of
their
time
entering
data
and
maintaining
data
in
the
electronic
health
record
versus
spending
their
time
managing
patients.
And
in
addition
to
that
and
a
number
of
regulatory
challenges
in
US
healthcare

given
how
regulated
it
is,
it
created
a
whole
bunch
of
tasks,
which
we
call
chore
tasks
that
the
providers
are
getting
distracted
with
and
their
administrators
are
getting
distracted
with.
And
so

in
order
to
do
those
chore
tasks
efficiently ,
and
not
get
the
providers
distracted
by
those
chore
tasks,
that
created
the
need
for
what
we
call
systems
of
action
that
would
take
all

these

chore

tasks

and

free

the

providers

and

their

administrators

up

from

those

tasks.

We

call

those

systems

of

action

that

can

do

that.

And

in

the

system

of

action

category ,

there

are

two
types.
There
are
point
solution
systems
of
interaction,
which
essentially
solve
for
one
or
two
or
three
of
these
tasks
because
the
individual
TAM
of
each
of
these
tasks
is
so
large

when
the
total
TAM
is
\$260
billion,
right?
So,
there
I
call
them
point
solution
systems
of
action.
And
then
there
are
very
few
rare
organizations
that
are
drinking
from
the
Kool-Aid
of

the

thesis

that

eventually

large

provider

organizations

can't

be

in

this

point

solution

hell

of

buying

multiple

point

solutions,

integrating

them

themselves.

And

so

eventually

this

market

will

gravitate

more

and

more

towards

what

I

call

platform

systems

of

action,

which

is

what

really

is,

where

we

have

a

comprehensive

platform

that

takes

all

of

these

tasks

at

once.

And

when

you

do

that

actually ,

one,

we're
able
to
take
accountability
for
the
outcomes
for
these
tasks.
And
the
provider
organizations
have
one
entity
to
hold
accountable
for
the
cost
and
quality
outcomes
for
these
tasks
and
revenue
outcomes

for
these
tasks.
And
then
second,
the
value
of
these
tasks
done
together
by
one
platform
compounds
because
all
of
these
tasks
are
interconnected
in
nature.
So
that's
what
I
call
the
platform

system

of

action.

So

those

are

the

three

genres

of

companies.

System

of

records

that

were

core

legacy

systems

that

are

now

waking

up

with

generative

AI

and

agentic

AI

and

thinking

they

should

become

the

platform

system

of

action.

Rare

platform

system

of

action

companies

like

us.

And

then

a

whole

bunch

of

large

point

solutions,

large

number

of

point

solution

systems

of

action.

I

think
that
language
is
somewhat
important
as
we
continue
to
think
about
IKS's
positioning
in
this
very
large
growing
market.

Moving
forward,
in
order
to
capture
a
significant
share
of
this
very

large
TAM,
IKS
as
a
platform
system
of
action
has
laid
out
five
key
strategic
pillars
of
execution
for
ourselves.
And
I'll
talk
a
little
bit
about
each
of
the
five
pillars.
There's,
obviously ,

going
to
be
a
sixth
pillar
that
will
likely
get
added
here
if
we
are
fortunate
enough
to
close
the
transaction
that
we
had
announced
a
few
weeks
ago,
the
acquisition
of
TruBridge,

because
obviously
the
integration
of
TruBridge
and
our
massive
opportunity
in
the
rural
healthcare
market
will
become
another
important
pillar
of
execution,
which
will
then
have
actually
some
cross
leverage
in
our
core

and
large
physician
group
market
as
well.
But
for
now,
let's
focus
on
these
five
pillars.
The
first
one
is,
remember
when
we
started
building
our
platform
system
of
action
18,
19
years

ago,
it
was
a
human-led
tech-in-the-loop,
kind
of,
endeavor ,
because
at
that
time
technology
was
not
advanced
enough
and
healthcare
was
too
fragmented
and
there
was
no
standardization
of
workflows
for
technology
to

truly
be
able
to
eliminate
all
these
tasks.
So,
we
started
with
a
human-led
tech-in-the-loop
model,
which
over
the
timeframe
of
say
2010
to
maybe
2020
became
a
lot
more
tech-led
and
human-in-the-loop,

where
the
humans
started
to
go
from
doers
of
tasks
to
auditors
of
tasks.
And
now
with
the
advent
of
generative
AI
and
really
specifically
agentic
AI,
back
in
about
2022,
2023
maybe,

we
started
moving
from
a
tech-led
human-in-the-loop
to
really
a
AI-native
agentic
platform
manifest,
where
our
ambition
obviously
is
to
move
as
many
of
these
features,
which
are
the
tasks
that
we
do

for
these
providers
of
our
platform
to
a
fully
autonomous
agentic
manifest.

And
so
that's
the
first
key
pillar
of
our
five
key
strategic
pillars
that
we
are
trying
to
execute
on.

Happy
to
note
that
we
continue
to
make
some
very
significant
progress
on
this
pillar .
As
you
might
have
noticed
through
our
various
releases
over
the
course
of
last
quarter ,
we've
launched
an

interconnected

agentic

workflow

for

autonomous

clinical

documentation,

coding,

and

prior

authorization.

These

three

tasks

tend

to

be

the

tasks

that

cause

some

of

the

greatest

friction,

for

want

of

a

better

term,

in

the
patient's
journey
with
the
provider
and
in
the
provider's
workflow .
And
so
really ,
really
important
to
alleviate
the
burden
of
these
tasks
from
the
provider's
workflow .
Also,
each
of
these
tasks
are

very
related
to
each
other .
So,
these
done
right
in
an
interconnected
agentic
workflow ,
they
can
have
a
very
nice
compounding
effect
on
the
outcome.

Also,
we've
launched
a
very
exciting
version
of

Scribble,

which

is

our

ambient

AI

scribing

product.

You

might

recall

we

had

launched

Scribble

Now ,

which

was

our

completely

autonomous

ambient

AI

scribing

product.

We

are

now

in

realization

of

the

fact

that

this

is

a

very

important

construct.

The

fact

is,

no

matter

how

good

generative

AI

models

get

in

healthcare,

there

will

always

be

some

edge

cases,

no

matter

how

good

generative

AI

gets,
that
will
require
some
level
of
human-in-the-loop,
especial-
ly
in
healthcare
where
we
cannot
afford
hallucinations
and
inaccuracies.
And
so
in
the
case
of
this
ambient
AI
scribing,
it
became
apparent
to

us

that

a

lot

of

these

ambient

AI

scribing

point

solutions

as

those

got

rolled

out

all

over

the

country

in

the

US,

one

saw

that

there

was

a

lot

of

doctors

using

it,
but
the
challenge
that
one
also
saw
very
quickly
was
that
the
number
of
patient
visits
for,
which
the
doctors
were
using
the
ambient
AI
scribing
products
was
topping
off
at
50%,
60%.

So,
technically
all
the
doctors
in
a
medical
group
or
in
a
hospital
system
are
using
it,
but
if
they're
only
using
it
for
50%,
60%
of
their
patient
encounters,
that
means
40%,

50%
of
their
burden
is
still
lying
there.
And
as
we
dug
in
deeper
into
that,
we
understood
that
this
whole
phenomena
of,
you
know ,
there
are
certain
types
of
patient
visits
where

the
ambient
AI
scribing
just
does
not
produce
the
type
of
outcome
the
doctor
needs
and
the
doctor
ends
up
having
to
do
too
much
of
the
editing
of
the
product
themselves.

And
so
given
our
legacy
of
tech-led
and
human-in-the-loop,
we've
actually
now
launched
a
new
multi-variant
product
of
our
ambient
scribing
solution
called
Scribble
Select
that
allows,
I
think
it's
the
only
product

in
the
industry
that
exists
like
that,
that
allows
the
doctor
to
choose
different
variants
of
Scribble
for
different
types
of
patient
encounters.

So,
for
some
of
the
very
complex
patient
encounters,
they

might
choose
the
Scribble
version
with
some
human-in-the-loop
where
there's
a
clinician
in
the
loop
that
oversees
the
documentation,
edits
the
documentation
and
cleans
it
up
before
it
goes
to
the
doctor .
And

for

some

of

the

more

simpler

visits,

they

might

use

the

fully

autonomous

ambient

AI

scribing

product

called

Scribble

Now .

And

having

such

a

multi-variant

option

for

each

doctor

is

actually

already

starting

to
show
in
our
early
implementations
a
very
fundamentally
different
level
of
utilization
by
these
doctors.
So,
very
excited
about
the
launch
of
Scribble
Select.
And
again,
it
signals
our
very
perhaps
pragmatic

approach
to
leveraging
GenAI
effectively
where
it
can
drive
autonomy ,
but
keeping
the
human-in-the-loop
where
it
can
truly
eliminate
the
burden
for
the
physician
users.
In
addition
to
that,
we
continue
to
make

advances
in
the
autonomous
coding
endeavors
that
we
have,
where
we've
already
developed
high
levels
of
accuracy
for
two
of
the
specialties
and
will
continue
to
expand.
And
then
super
excited
to
launch

MyCare
Hub
over
this
last
quarter ,
which
is
really
a
multi-agent
orchestration
technology
across
several
patient
engagement
features,
like,
scheduling
optimization,
patient
onboarding
eligibility
and
benefits
verification,
etcetera.
So,
again,
significant
progress
across

several
features
in
this
endeavor
to
move
into
a
AI-native
agentic
platform
manifest.

On
the
second
pillar
of
our
integration
of
the
AQuity
acquisition,
happy
to
note
that
the
first
two
dimensions

of

that

are

now

more

or

less

complete.

I

think

AQuity

and

IKS

really

operate

like

one

organization

now,

both

in

terms

of

how

we

are

operating

tangibly ,

but

also

I

think

culturally .

There
was,
obviously ,
a
significant
margin
opportunity
from
the
AQuity
acquisition
by
transforming
their
operating
model
from
a
heavy
human-led
US-based
operating
model
to
a
true
tech-led
and
offshore-enabled
operating
model
for
the

human-in-the-loop

part.

That

is

now

more

or

less

complete,

which

obviously

you've

seen

in

our

margin

expansion

over

t

he

last

couple

of

years.

And

then

the

third

big

vector

was

the

cross-sell

into

the

large

AQuity

customer

install

base.

Granted,

that

took

us

longer

than

we

had

originally

planned.

And

that

predominantly

was

because

we

made

some

mistakes

in

hindsight

in

how

we

went

to

market
in
some
of
the
large
health
system
owned
groups
in
the
legacy
customer
base
of
AQuity .
The
mistake
we
made
was
that
we
went
with
our
big
platform
pitch
to
these
buyers
of

the
large
health
system
owned
provider
groups.
And
the
reality
was
that
the
buyers
in
the
AQuity
customer
base
there
were
not
the
C-suite
buyers.
They
were
buyers
that
were
really
focused
users

of
certain
point
solutions
or
certain
tasks.
And
for
them,
the
large
platform
pitch
seemed
intellectually
interesting,
but
they
had
nowhere
to
take
it
because
they
were
not
the
decision
makers
for
this.

And
we
thought
we'll
be
able
to
leverage
those
people
to
elevate
the
conversation
to
the
C-suite,
which
took
longer
than
we
expected.

And
we
also
learned
that
over
a
period
of

time,
the
large
health
system
owned
groups
today
do
not
have
the
appetite
for
a
full
platform
manifest,
even
as
they
intellectually
are
intrigued
by
the
idea.
And
both
from
a
change
management

perspective,
how
they're
internally
organized.
And,
so,
we
actually
then
pivoted
our
strategy
where
we
are
going
with
the
mid-sized
health
system-owned
medical
groups,
we're
still
going
with
the
platform
manifest
and
the

platform-based

GTM.

But

for

the

large

health

systems,

now

we

have

pivoted

our

go-to-market

where

we

are

resorting

to

a

pure

land

and

expand

strategy

where

there

are

certain

point

solutions

that

they

are
very
interested
in
buying
today ,
i.e.
the
RCM
features,
the
coding
features,
perhaps
the
features
around
patient
access.
And
so
we're
going
to
market
specifically
to
those
large
health
system-owned
groups
with

those
features
and
then
expand
over
a
period
of
time.
And
the
mid-sized
health
systems,
we're
going
with
the
full
platform
idea.
And
happy
to
note
that
one
of
the
successes
of
that

strategy
is
manifested
in
this
quarter
where
a
large
health
system,
a
top
five
health
system,
has
chosen
to
expand
their
relationship
with
us
in
the
dimensions
of
RCM
and
value-based
care.
And

yet
at
the
same
time,
we've
announced
a
very
significant
full
platform
deal
with
a
mid-sized
health
system
that
also
came
from
the
AQuity
customer
base.
So,
I
feel
like
we've
now
figured

out
the
plot
to
truly
unlock
that
cross-sell
motion
in
the
AQuity
customer
base,
but
of
course,
the
proof
will
be
in
the
pudding
as
we
continue
to
manifest
that
strategy
over
the

next
several
quarters
and
years.

Driven
by
this
differentiated
strategy
where
for
the
large
health
systems,
the
go-to-market
is
still
more
point
solution
oriented,
because
of
the
land
and
expand
motion,
we
need

to
be
number
one
or
two
or
three
in
each
of
those
point
solutions
that
the
large
health
systems
are
buying,
namely
RCM,
coding,
patient
access,
while
being
the
only
company
that
has

the
full
platform
manifest
that
other
segments
of
the
market
are
loving.
And
we
actually
believe
eventually
the
large
health
system
markets
will
also
love
the
platform
manifest,
but
they're
not
quite
there

yet.

That

means,

we

needed

to

be,

like,

I

said

that

number

one

or

two

or

three

in

some

of

those

point

solutions.

Happy

to

note

as

our

third

pillar

that

we've

really

been

able

to

make

progress.

Today

we

are

number

one

in

AI-driven

RCM

as

recognized

by

Black

Book.

We're

a

top

performer

in

ambulatory

RCM

as

recognized

by

KLAS.

As

you

know ,

KLAS

is

probably

the

Gartner

equivalent

of

IT

services,

if

you

would,

and

so

the

Gartner

equivalent

of

healthcare,

if

you

would.

And

very

happy

to

note

that

we're

very

close

to

the

number

one
provider
in
the
ambulatory
RCM
category
by
KLAS
and
continue
to
have
significantly
good
ratings
in
both
documentation
and
coding
by
both
Black
Book
and
KLAS.
So
good
progress
there.

I

already

spoke

a

little

bit

about

our

differentiated

growth

strategy ,

which

now

is

stratified

between

a

platform

go-to-market

for

mid-sized

health

systems-owned

groups

as

well

as

independent

single

specialty

and

multi-specialty

groups.

And

then

for

the

large

health

systems,

focus

on

an

entree

through

two

or

three-point

solutions

and

then

expand

from

there.

And

then

last

but

not

the

least,

a

very

important

pillar

of

our

strategy

is

over

a

period

of

time,

what

would

our

moat

be?

I

think

our

moat

would

be

two-pronged.

One,

we

would

be

the

one

of

the

only

comprehensive

platform

systems

of

action

in
the
country .
And
number
two,
when
a
platform
system
of
action
is
actually
able
to
align
its
outcomes
with
the
provider's
outcomes,
it
becomes
even
more
appealing.
And
that's
where
when
we

take

accountability

for

outcomes

and

execute

to

those

outcomes,

like

we

have

done

recently

in

certain

deals,

we

saw

the

\$3

million

NEV A

earnings

in

Palomar

from

our

performance

there

in

the

year

2025,
that
makes
the
value
proposition
even
more
compelling.
So,
as
we
progress
here,
if
we
are
able
to
truly
demonstrate
this
ability
to
produce
transformative
financial
outcomes
even
as
we
are
making

the
administrative
burdens
of
physicians
lesser ,
that
would
put
us
in
a
whole
different
category
as
a
platform
system
of
action
and
that
continues
to
be
our
fifth
key
pillar
of
execution.

So
happy
to
note
that,
I
think,
we
have
fairly
good
progress
across
each
of
those
five
pillars.
And
as
we've
done
that
over
these
last
several
months,
it's
always
good
to
see

that
we're
also
getting
some
recognition
for
it.
We
were
recognized
as
one
of
the
best
users
of
AI
in
healthcare
and
life
sciences.
If
you
can
go
to
the
next
slide,
Saransh,

please.

And

then

one

of

the

things

that

we've

always

been

proud

of

is

the

fact

that

45%

of

our

workforce

is

women.

And

gender

diversity

is

a

really

important

aspect

in

our

business.

If

you

can

go

to

the

previous

slide,

please

Saransh.

And

one

thing

IKS

is

really

proud

of

is

not

only

is

45%

of

our

workforce

women,

but

even

when

you

look

at
our
leadership,
there's
a
very
significant
mix
of
women
leaders
in
our
leadership.
And
so
that
being
recognized
is
important,
where
you
have
the
likes
of
Nithya,
our
CFO;
Manisha,
our
CHRO;

Dr.
Grace
Terrell,
our
CMO;
Kathryn,
our
Chief
Marketing
Officer;
Christi,
our
Chief
Legal
and
Compliance
Officer.
We're
able
to
manifest
this
gender
diversity
not
just
at
the
bottom
of
the
pyramid,
but

even
at
the
top
of
the
pyramid,
which
puts
us
in
a
very
unique
position
over
a
period
of
time.

Also
now
talking
a
little
bit
about
some
of
the
endeavors
that
actually

accelerated

our

AI

initiatives

over

this

last

quarter

are

some

very

interesting

developments.

The

first

one

I

want

to

talk

about

is

Certilytics,

which

really

over

a

period

of

time

has

been

an

AI-driven
innovator
on
the
payer
side
of
the
business.

And
as,
you
know ,
in
US
healthcare,
why
revenue
cycle
is
such
an
important
and
such
an
expensive
function
for
payers
is
because
there's

this
constant
cat
and
mouse
game
that
gets
played
between
providers
that
want
to
get
paid
for
the
care
they
deliver
and
payers
that
have
to
pay
them
for
the
care
they
deliver ,

but
they're
always
worried
that
is
the
care
being
delivered
appropriate
for
the
type
of
condition
that
the
patient
had.
Are
we
unnecessarily
delivering
too
much
care
or
are
we
doing
too
many

surgeries,

too

many

diagnostic

tests

when

the

conditions

don't

demand

that,

right?

So

that's

called

utilization

in

the

payer

world.

And,

so,

the

payers

are

always

trying

to

make

sure

utilization

is

proper .

And,
so,
there's
this
constant
cat
and
mouse
game
that
happens
between
the
payers
and
providers.

And
we
actually
believe
that
if
over
a
period
of
time,
we
can
get
providers
and
payers

to
collaborate
better ,
we
might
actually
be
able
to
reduce
the
friction
in
this
relationship,
reduce
the
cost
of
revenue
cycle
on
both
sides,
the
payer
side
and
the
provider
side,
and
improve

the
efficiency
of
the
process.

And
in
light
of
that,
our
partnership
with
Certilytics,
it
has
deep
experience
on
how
payers
technology
is
oriented.

And
with
our
expertise
on
the
provider
technology ,
our

ambition

is

that

barring

certain

things

where

there

will

never

be

complete

collaboration,

for

example,

payers

will

never

put

out

their

APIs

as

it

relates

to

how

they

assess

whether

a

claim

that

is
submitted
by
a
provider
is
appropriate
from
a
utilization
perspective.
That
one
is
the
holy
grail,
it
will
never
be
exposed.
But
if
you
think
about
other
denials
that
payers
submit,
think

of
denials
related
to
medical
documentation
or
coding,
those
types
of
things
can
easily
be
eliminated
if
we
can
get
the
payer
systems
to
talk
to
the
provider
systems
and
collaborate
through
the

process.

So

instead

of

the

dynamic

being

the

providers

people,

RCM

people

versus

the

payers

RCM

people,

then

the

providers

technology

versus

the

payers

technology ,

and

now

the

providers

AI

versus

the

payers

AI,
we
can
actually
create
a
more
collaborative
framework
that
actually
reduces
the
friction
in
the
relationship.

So,
we're
super
excited
about
this
relationship
with
Certilytics
and
how
it
will
probably
elevate
our

positioning
in
this
RCM
space
to
a
place
where
not
only
are
we
improving
how
the
providers
conduct
their
RCM,
but
we
might
fundamentally
be
able
to
improve
outcomes
through
more
collaborative
technology-driven,

AI-driven
engagement
with
payers
at
least
on
certain
states.

Second
one,
that
is
exciting
that
we
had
already
announced
also
is
the
strategic
acquire
of
ThinkDTM,
which
really
was
an
AI-native
and

digital
services
company
where
they
have
now
built
to
the
fourth
point
and
launched
what
we
call
our
MyCare
Hub
product,
which
is
really
an
agentic
AI
self-orchestrating
platform
that
uses
an
active,

aware,
and
constant
multi-agent
behavioural
algorithm
to
create
this
adaptive
and
autonomous
patient
engagement
operating
system.

I
know
that
sounds
like
a
lot
of
jargon,
but
simply
put,
it's
a
multi-agent
orchestration
that

again

simplifies

a

lot

of

the

interactions

that

patients

have

with

the

provider

organization,

be

it

for

scheduling,

be

it

for

onboarding,

registration,

eligibility

verification,

prior

authorization,

all

of

these

things

create

over-the-counter

collections
for
the
patient
responsibility
in
that
overall
reimbursement
that
the
provider
is
going
to
get,
because
part
of
the
payment
comes
from
the
patient,
part
of
it
comes
from
the
insurer
or

the
payer .
So,
this
is
a
really ,
really
very,
very
significant
AI-driven
advancement
that
we've
been
able
to
do
with
the
ThinkDTM
that's
getting
some
tremendous
traction
in
the
marketplace.

And
then
last

but
not
the
least,
obviously ,
continuing
to
advance
our
AI-driven
revenue
cycle,
as
well
as
our
autonomous
coding
engines.
And
now
both
of
these,
the
autonomous
coding
and
revenue
cycle
engine,
and
MyCare

Hub

are

now

actually

available

in

Epic

Connection

Hub,

which

becomes

a

really

important

aspect

of

being

able

to

penetrate

these

products

in

the

large

health

system

environment

because

the

system

of

record

in
the
large
health
system
environment
tends
to
be
Epic.
So,
again,
some
very
strong
progress
on
our
AI
initiatives
over
these
last
few
months.
And
then,
last
but
not
the
least,
the

comment

I'd

like

to

make,

if

you

can

move

to

the

next

slide,

Saransh,

is

when

you

look

at

the

16

features

of

our

platform,

16-odd

features

of

our

platform.

As

you

can

imagine,

at

a

very

individual

feature

level,

we

are

tracking

and

driving

a

path

to

as

much

autonomy

as

is

practically

possible

driven

by

agentic

AI

in

each

of

those

features.

Now ,

why

do

I

say

as

much

autonomy

as

practically

possible?

Well,

first,

because

of

the

fact

that

it's

healthcare

like

I

said,

for

each

of

these

features,

almost

no

matter

how

good

the

AI

gets,
there
will
always
be
some
edge
cases
that
will
need
human-in-the-loop.

So
I
just
want
to
clarify
that
even
100%
autonomy
as
per
this
tracking
doesn't
mean
that
you've
totally
eliminated
human-in-the-loop.

Like

I

just

gave

you

the

example

of

the

Scribble

Select

product,

even

though

there

is

a

Scribble

Now

100%

autonomous

ambient

AI

scribing

tool,

in

order

to

improve

the

utilization

of

that

Scribble
product
and
totally
eliminating
the
physician's
burden,
we're
having
to
create
variants
with
human-in-the-loop
so
that
the
actual
task
can
be
completed
effectively
even
though
it's
heavily
enabled
by
AI.
So
autonomy

here

might

not

always

mean

100%

elimination

of

human-in-the-loop,

but

it

means

largely

elimination

of

human-in-the-loop,

right?

And

I

just

wanted

to

leave

two

concepts

with

everybody .

One,

we're

tracking

this

by

feature

and

our

AI

endeavors

are

built

by

feature.

I

just

want

to

just

say

there's

two

considerations

as

to

how

much

autonomous

each

of

these

tasks

or

features

can

get.

One

is

whether

the

outcome

from

that

task

is

a

deterministic

outcome

or

a

non-deterministic

outcome.

What

does

that

mean?

Basically ,

generative

AI

is

most

successful

when

there

are

non-deterministic

outcomes

for

the

task.

For

example,
the
outcome
from
ambient
AI
scribing
is
this
large
narrative
clinical
documentation.

That's
not
a
deterministic
outcome.

It's
not
a
specific
outcome.

It's
a
narrative.

Language
generation
is
one
of
the
key

strengths

of

generative

AI.

So

generative

AI

can

be

very,

very

useful

there.

But

when

the

outcome

is

purely

deterministic,

the

generative

AI

has

to

be

constrained

or

overlaid

by

let's

call

it

symbolic

AI

models.

So,

the

neuro

AI,

which

is

the

traditional

generative

AI,

then

needs

to

be

combined

with

symbolic

AI

to

which

then

relies

on

reasoning

based

on

things

like

knowledge

graphs

and

ontologies

and

specific

rules

and

logic.

And

for

deterministic

outcomes,

you

have

to

use

a

combination

of

generative

AI

and

the

symbolic

AI

that

together

can

drive

higher

autonomy .

So

remember

deterministic

versus

non-deterministic

is

a

key

driver

of

autonomy .

And

then

the

second

piece

is

for

many

of

these

tasks,

you're

interacting

with

entities

outside

of

the

provider's

organization.

And

your

ability

to

drive

autonomy

is
also
subject
to
how
technology
ready
that
other
enterprise
is
with
which
you're
interacting,
i.e.
the
payers.
It
could
be
the
payers,
it
could
be
the
clearinghouses,
it
could
be
other
providers

where
you're
dealing
with
tasks
like
referral
management.

So,
I
just
wanted
to
clarify
that
those
are
the
things
that
go
into
defining
how
much
autonomy
can
be
achieved.

And
it's
those
types

of
aspects
that
are
very
thoughtfully
being
put
into
which
of
these
features
get
autonomous
over
a
period
of
time
as
we
go
down
this
path.
So
that's
a
quick
thought
process
as

it
relates
to
the
various
features
of
our
platform.

Let's
now
pivot
into
the
Q4
financial
performance.

I'm
happy
to
report
that
we
had
a
very
strong
quarter
of
financial
performance,
perhaps

our
sixth
consecutive
quarter
of
relatively
strong
financial
performance.

Revenue
grew
18.5%
odd
year-on-year

coming
in
at
about

INR857
crores.

In
constant
currency
terms,

that
was

about
13%
revenue

growth,
which

is
in

USD

terms.

In

INR,

it

was

about

18%,

18.5%

growth.

We

now

sit

with

about

450

odd

large

enterprise

customers.

Like

I

said,

there

are

600

odd

customers.

Of

that,

about

450

are

the
large
enterprise
level
customers,
which
is
where
we
see
the
maximum
growth
coming
from.
Our
revenue
from
top
10
customers
was
about
INR452
crores
in
that
INR857
crores.
And
like
I've
said,

the
vintage
for
our
top
10
and
top
5
customers
continues
to
be
north
of
5
plus
years.

That
revenue
of
INR857
crores
was
delivered
using
13,331
employees.
So
like
I
was
saying

earlier ,
at
the
same
time
last
year,
we
had
12,661
employees.
So
our
growth
in
employee
headcount
has
been
about
5.3%.
And
I
want
to
put
this
in
perspective.
For
about
a
13%

growth

in

revenue

in

constant

currency ,

our

headcount

growth

is

only

about

5.3%.

So

that

constant

non-linearity

between

revenue

growth

and

people

growth

is

very

much

evident.

Also

important

to

note

that

if

you
really
look
at
the
December
2025
quarter ,
which
is
the
previous
quarter ,
our
headcount
was
actually
higher
than
13,331.
So,
Saransh,
if
you
were
to
move
to
the
next
slide,
essentially ,
for

the
5.2%
quarter -on-quarter
growth
in
revenue
that
we've
demonstrated,
we've
actually
declined
net
headcount
from
Q3
of
FY26
to
Q4
of
FY26.

So
like
I
was
saying,
18.5%
year-on-year
growth
in
revenue,

5.2%

quarter -on-quarter .

That

has

resulted

in

an

EBITDA

of

about

INR300

crores,

which

is

about

35%

EBITDA.

And

again,

on

an

18.5%

year-on-year

revenue

growth,

our

EBITDA

growth

is

nearly

33%

year-on-year .

And

on

a

5%

quarter -on-quarter

growth,

the

EBITDA

growth

is

6.6%.

So

again,

that

non-linearity

between

revenue

growth

and

people

growth

and

revenue

growth

and

margin

growth

is

easily

witnessable

here.

And

that

also

has
resulted
in
a
further
non-linearity
between
revenue
growth
and
profit
after
tax,
where
our
profit
after
tax
for
the
INR857
crores
came
in
at
about
INR206
crores,
which
is
about
24%.
And

that
was
essentially
a
growth
of
39%
year-on-year
and
12
odd
percent
quarter -on-quarter .
Now
obviously
there's
non-linearity
between
EBITDA
growth
and
PAT
growth
also
because
over
the
course
of
the
year,
we've
been

constantly

paying

down

debt

and

the

interest

costs

are

coming

down

as

a

result

of

that.

And

so

that's

why

the

PAT

growth

is

even

more

significant

than

the

EBITDA

growth.

But

the

non-linearity

between
revenue
and
margin
growth
continues
to
be
witnessed
here
through
the
financial
performance
of
this
quarter .

And
Saransh,
if
you
were
to
go
back
one
slide,
happy
to
announce
these
three

top

relationships

through

the

quarter

that

are

of

most

significance.

I

had

mentioned

a

mid-sized

health

system

from

the

legacy

AQuity

customer

base

where

we

were

able

to

cross-sell

the

entire

IKS

platform,

Holyoke
Medical
Center .
Very
proud
to
announce
them
as
a
customer
and
very
excited
about
this
implementation.
Also
happy
to
note
that
in
this
case,
we
have
not
incentivised
them
in
any
way

from

an

outcomes

perspective

to

adopt

the

full

platform.

So

obviously

that

sort

of

demonstration

of

the

value

of

the

full

platform

is

starting

to

come

into

play

in

these

deals.

Mission

Community

is
an
existing
customer
of
ours
where
we
are
advancing
the
relationship
from
the
full
manifest
of
our
platform
to
a
very
unique
AI-driven
relationship
that
will
start
to
drive
other
efficiencies
in

the
hospital
as
it
relates
to
predicting
ICU
utilization,
operation
theatre
utilization,
nurse
utilization,
etcetera.
And
from
that,
be
able
to
use
those
resources
or
sweat
those
resources
much
more
effectively .
So
it's

a
very
unique
relationship
that
actually
will
allow
us
to
build
even
more
AI-based
capabilities
from
a
prediction
perspective
that
can
drive
significant
efficiency
in
hospital
operations.
And
then
last
but
not
the

least,
like
I
was
saying,
a
top
five
health
system
in
the
AQuity
customer
base
where
we
very
significantly
expanded
the
revenue
cycle
relationship
and
the
value-based
care
relationship.
And
this
is
the

evidence

of

that

sort

of

land

and

expand

approach

that

we

intend

to

continue

to

take

with

these

large

health

systems.

Obviously ,

all

that

resulted

in

a

strong

growth

in

earnings

per

share,

which

is

about

39%

year-on-year

and

12.3%

quarter -on-quarter .

Saransh,

if

you

go

to

Slide

11,

please.

And

also

continues

to

produce

healthy

ROE

results,

which

are

north

of

30%

and

in

this

case

31

odd

percent.

The

decline

year-on-year

in

the

ROE

percentage

is

nothing

but

based

on

some

revaluation

of

the

base

where,

for

example,

our

equity

holding

in

a

bridge

system,

which

is

a

technology

company ,

got

revalued

based

on

their

own

valuation.

And

based

on

also

the

revaluation

of

some

of

the

assets

based

on

the

depreciation

of

the

rupee

to

the

dollar ,

the

base

got

widened

and

hence

that

shows

a

minor

decline

in

the

ROE

percentage.

Otherwise,

very

healthy

ROE

numbers.

And

all

this

obviously ,

fourth

quarter

strong

performance

resulted

in

a

pretty

strong

performance

for

the

fiscal

year

'26

as

well.

Saransh,

if

you

can

go

to

the

next

slide

where

revenue

came

in

Saransh,

you

can

just

jump

to

Slide

13

actually .

Where

revenue

came

in

at

INR3,193

crores,
which
is
nearly
a
20%
year-on-year
growth.
And
again,
the
non-linearity
being
demonstrated
where
at
a
20%
year-on-year
growth,
we
have
a
38%
year-on-year
growth
in
EBITDA.
EBITDA
coming

in
at

INR1,091

crores

or

34

odd

percent.

And

a

48%

year-on-year

growth

in

PAT,

where

PAT

came

in

at

about

INR721

crores

and

about

22.6%.

So,

all

in

all,

I

think

it

was

a

very
strong
performance.
And
obviously
that
resulted
in
strong
performance
even
on
the
EPS
and
ROE
parameters
where
EPS
for
the
year
came
in
at
43%,
which
was
nearly
48%
higher
over
fiscal

25.

Healthy

ROE

metrics

as

demonstrated

in

Slide

14,

Saransh.

And

then

based

on

all

of

that,

eventually ,

as

you

would

note,

all

that

has

to

result

in

cash

flows.

And

very

happy

to

note

that

this

also

resulted

in

very

strong

operating

and

free

cash

flows.

Our

operating

cash

flow

growth

over

the

year-on-year

basis

was

nearly

99%,

with

operating

cash

flow

coming

in

at

INR863

crores

and

free

cash

flows

on

that

INR721

crores

PAT

coming

in

at

about

INR612

crores.

So

very,

very

healthy

metrics

as

it

relates

to

operating

free

cash

flow

generation.

I

want

to

call
out
that
the
improvement
in
operating
and
free
cash
flow
ratios,
the
ratio
we
track
are
really
OCF
to
EBITDA
and
free
cash
flow
to
PAT.
In
FY25,
our
operating
cash
flow

to
EBITDA
was
about
55%,
whereas
in
FY26
it
came
in
at
79%.
And
free
cash
flow
in
FY25
to
PAT
was
about
56%,
and
in
FY26
it
came
out
at
85%.
So

really
healthy
numbers
as
it
relates
to
free
cash
flow
generation,
which
then
has
enabled
us
to
reduce
our
debt
over
these
last
couple
of
years
from
a
peak
of
about
INR850
crores

to
now
standing
as
of
March
31st,
2026,
at
INR251
crores.

So,
all
in
all,
I
think
a
strong
quarter
of
performance
and
a
strong
year
of
performance.
But
more
importantly ,
perhaps

or
equally
importantly ,
sets
us
up
for
a
future
based
on
all
the
initiatives
that
we
are
taking
on
really
becoming
an
AI-native
agentic
platform
of
interconnected
workflows.
And
perhaps
the
only
platform

system
of
action
that
exists
in
the
industry
today
that
is
able
to
align
its
outcomes
with
provider's
outcomes
to
drive
transformative
value.
I
will
pause
there
for
a
second
and
Nithya,
maybe

let
you
comment
on
some
of
the
key
financial
metrics
that
we
look
at
in
addition
to
what
I
have
covered
and
then
we'll
talk
a
little
bit
about
a
couple
of
other

things.

Nithya

Balasubramanian:

Thank

you,

Sachin.

Saransh,

if

you

can

go

on

to

the

next

slide

.

Just

a

few

additional

nuances

on

the

Q4

numbers.

Revenue

came

in

at

\$95

million.

We

did

see
some
currency
support.
On
the
face
of
the
P&L,
you're
seeing
a
forex
gain
of
INR35
crores,
but
please
note
that
there
was
also
a
hedge
loss
of
INR12
crores.
So,
the

net
currency
benefit
was
for
us
about
INR23
crores.
Employee
benefit
expenses
Q3,
Q4
compared
to
Q3
was
rather
flattish
in
line
with
the
headcount.
Other
expenses
did
increase
by
almost
INR38
crores

between

the

two

quarters.

The

majority

of

the

delta

is

actually

attributable

to

one-time

due

diligence

and

legal

fees

that

are

related

to

the

TruBridge

transaction.

The

regulatory

approvals

are

still

work

in

progress,
so
some
additional
transaction
expenses
are
to
be
expected
in
Q1
as
well.

If
you
look
at
finance
costs,
that
has
come
down
sharply
compared
to
Q3.
Q3
does
have
a

one-of f
write-of f
of
the
debt
issuance
cost
when
we
refinanced
the
loan.
So
our
interest
rates
have
come
down
and
so
has
the
finance
cost.
D&A
and
interest
income
are
broadly
in
line

with

Q3.

Our

PBT

was

INR258

crores

and

PAT

was

INR206

crores.

We

did

report

the

Western

Washington

MSO

associate

loss

at

about

INR5

crores.

So

Q4

or

rather

Q1

calendar

te

nds

to

be

seasonally

the

weakest

quarter

for

US

providers.

Patient

footfalls

tend

to

be

very

low

because

of

the

reset

of

the

insurance

and

the

need

to

work

through

the

deductible.

Plus

it

was

a

fairly

intense

winter

as

well.

So

patient

footfalls

were

pretty

weak.

We

do

expect

this

to

improve

in

the

coming

quarters.

ETR

was

about

19%

for

the

quarter

and

20%

for

the

full
year.
We
expect
ETR
in
the
range
of
about
22%
for
FY27.

Saransh,
if
you
can
go
to
the
next
slide.
Our
EBITDA
per
employee
stands
at
8.8
lakhs
in
FY26,
which

is
in
line
with
the
healthy
EBITDA
growth
that
we
have
seen.
Our
top
10
customers
and
top
5
customers
tend
to
grow
pretty
strongly .
Top
10
customers
grew
at
a
very
healthy

28%

and

top

5

at

18%.

Vintage

of

our

top

customers

also

tends

to

be

very

healthy

and

it

remains

at

the

5

to

6

year

mark.

Sachin's

already

talked

about

the

FCF

yield.

In

terms

of

clients

with

revenue,

we

have

seen

a

slight

decline

and

that's

simply

because

some

of

the

AQuity

customers

that

were

at

more

than

1

million

saw

a

little

bit

of
shrinkage
as
we
offshored
those
customers
and
offered
some
discounts.
I'll
stop
my
remarks
here
and
hand
it
over
back
to
Sachin.
Saransh
Mundra:
Sorry
to
interrupt
in
between.
Sachin
sir,
if

you're
speaking,
you're
on
mute.
Sachin
Gupta:
Oh,
I'm
sorry .
I
was
on
mute.
I
apologize.
Thank
you.
So
quickly ,
I
want
to
just
touch
upon
our
proposed
acquisition
of
TruBridge,
which
we've

spoken

extensively

about,

but

just

to

give

everybody

a

quick

recap.

We

are

awaiting

the

close

of

that

transaction

even

after

we

signed

the

agreement.

And

the

idea

there

is

to

build

the

absolute
leader ,
the
only
integrated
system
of
record
and
system
of
action
for
the
rural
healthcare
market,
which
is
a
\$162
billion
market
with
2,200
plus
hospitals.
TruBridge
has
their
EHR
in
700

to
800
hospitals,
which
includes
patient
data
worth
15
million
plus
patients.
And
the
reality
is
for
most
effective
agentic
AI
orchestration
of
tasks
in
the
native
workflow ,
the
real
data,
the
real

moat
is
not
the
agentic
AI
technology ,
but
it
is
this
AI
training
corpus
that
can
be
built
if
you
have
the
patient
data.
Not
just
transactional
access,
asynchronous
access,
partial
access
to

data,
but
true
ownership
of
that
data
because
then
you
can
convert
that
data
into
a
truly
longitudinal
labeled
action-aware
dataset
where
you
can
link
the
clinical
context
to
the
actions
taken
and

to
the
outcomes
achieved
from
it.
And
that
enables
truly
the
most
effective
agentic
orchestration.

And
then
given
that
we
would
be
in
the
EHR
and
we
own
the
EHR,
we
can
actually

orchestrate

all

of

those

tasks

in

the

native

workflow

synchronously .

And

that

really

becomes

a

very,

very

compelling

value

proposition

for

the

rural

client.

And

so

we're

very

excited

continually

about

closing

the

TruBridge
transaction
and
creating
that
additional
vector
of
growth.
And
the
other
byproduct
of
that,
that
is
useful
for
our
core
physician
group
market,
remember ,
is
that
60%
to
70%
of
care
delivered

in
rural
hospitals
is
still
outpatient
care.
So
a
lot
of
the
models
that
we
will
build,
including
the
SLMs
that
we
are
going
to
build
for
rural
healthcare
and
maybe
multiple
SLMs

for
different
tasks
within
rural
healthcare,
will
actually
apply
in
our
large
physician
group
market
as
well
because
like
I
said,
th
e
nature
of
the
service
provided
in
rural
hospitals
is
largely

outpatient

in

nature,

which

is

what

our

physician

group

customers

provide

in

our

traditional

market.

And

as

you

know ,

we've

put

out

a

True

North

vision

of

taking

our

business

at

the

confluence

of
executing
as
the
only
platform
system
of
action
in
our
traditional
large
physician
group
market
and
a
truly
integrated
system
of
record
and
platform
system
of
action
in
the
rural
healthcare
market.

We
put
a
True
North
strategy
out
there.
We
believe
that
it
will
take
our
EBITDA
from
that
INR1,000
crore
odd
mark
LTM
December
2025
to
about
INR3,000
crores
in
FY30
and
take

us
back
close
to
zero
net
debt
that
we
were
getting
close
to
this
year.
So
that's
sort
of
our
vision
of
the
True
North.
And
again,
a
big
enabler
of
that
is

going

to

be

this

AI

training

corpus

and

the

moat

that

we

will

create

around

it

of

being

able

to

orchestrate

agentically

features

in

the

native

workflow

and

the

SLMs

that

we

will

build,
that
allow
us
to
actually
utilize
the
other
foundational
LLM
models
a
lot
lesser
and
build
our
own
proprietary
moat
in
these
SLMs.

And
in
order
to
actually
accelerate
that
journey
of

being
able
to
build
our
own
SLMs,
we're
also
very
excited
to
announce
this
additional
acqui-hire
that
we
are
doing
today ,
or
rather
did
yesterday ,
a
company
called
ARAI,
which
has
been
founded

by
two
outstanding
scientists.
Dr.
Roland
Haas,
a
gentleman
of
German
origin
that
came
to
India
25
odd
years
ago,
fell
in
love
and
never
left.
I
think
Roland
would
be
happy
with

that
description.
And
Dr.
Asoke
Talukdar ,
both
associated
with
highly
revered
institutes
over
the
course
of
their
career .
They
built
some
very,
very
fine
IP
that
is
very,
very
relevant
to
our
AI

journey .

As

I

was

mentioning

earlier ,

the

way

to

solve

for

autonomy

across

the

tasks

in

our

16

odd

feature

set

is

through

neuro-symbolic

models.

Generative

AI

obviously

relies

more

on

the

neuro

models,

the

deep

learning

models,

and

hence

is

more

suitable

for

non-deterministic

outcomes.

But

because

some

of

these

tasks

will

have

deterministic

outcomes,

you

need

neuro-symbolic

models

where

you

can

then

constrain

the

outcomes
of
the
generative
AI
through
the
reasoning
that
is
available
in
the
symbolic
models
that
requires
the
usage
of
things
like
ontologies,
neuro-symbolic
models
where
you
can
then
constrain
the
outcomes
of

the
generative
AI
through
the
reasoning
that
is
available
in
the
symbolic
models
that
requires
the
usage
of
things
like
ontologies.

Saransh
if
you
would
just
move
to
the
next
slide.
Ontologies
and

knowledge

graphs,

which

by

the

way,

the

team

at

ARAI

has

already

built

a

bunch

of

knowledge

graphs

and

the

IP

associated

with

them

for

clinical

reasoning.

They've

built

clinical

decision

support

systems.

And
really
with
this,
we
are
going
to
be
introducing
the
construct
of
what
I
call
glass
box
AI.
What
is
the
construct
of
glass
box
AI?
It's
actually
pretty
straightforward.
The

idea

really

is

that

in

healthcare,

traditional

AI

doesn't

really

work

because

not

only

do

you

need

to

do

the

task,

but

you

need

to

do

the

task

in

a

transparent,

traceable,

and

auditable

manner

because

if

the

outcome

of

the

task

is

anything

less

than

accurate,

it's

very

important

to

understand

how

the

AI

generated

the

outcome

that

it

was

suggesting.

And

so

I'll

give

you
an
example
of,
you
know ,
take
medical
coding
as
an
example,
right?
In
a
traditional
AI
setup,
a
model
might
infer
the
codes
directly
from
the
clinical
notes,
the
medical
codes.
And

those
codes
might
be
close
to
accurate,
but
they
might
be
missing
some
subtle
critical
details.
In
a
glass
box
AI
system,
the
model
instead
maps
the
documentation
to
the
standardized
ontologies.
It

validates
the
relationships
against
the
known
medical
knowledge.

It
also
flags
ambiguity
when
the
confidence
that
the

AI
is
producing
in
the
code
is
low,
which
means
that
will
get
then
naturally
routed

to
a
human
reviewer
when
necessary
if
the
confidence
score
is
less.
And
so
as
a
result
of
that,
you
get
higher
accuracy
in
coding,
lower
denials,
and
a
clear
audit
trail.
And

I
think
this
sort
of
glass
box
AI
construct
is
going
to
be
the
real
winner
in
the
healthcare
operating
environment.

So
I
think
really
excited
that
the
ARAI
team
advances
our

journey

towards

this

transparent,

traceable,

and

auditable

AI

with

their

proprietary

knowledge

graphs

that

they've

already

built.

And

they

really

enable

us

to

have

a

very

scalable

R&D

talent

pipeline.

You

know ,

they

are

basically

talent

magnets

themselves.

They

are

hands-on

even

as

their

CEO

and

CAIO,

they

write

code

today .

So,

you

know ,

they've

had

all

of

these

research

students

around

them

that

make

for

a

very

interesting

recruitable

talent

base.

And

so

very

excited

about

what

they

will

be

able

to

do

to

accelerate

our

journey .

And

if

you

would

move

to

slide

25.

In

summary ,

we

are
paying
about
\$1.2
million
in
upfront
cash
for
acquiring
the
IP.
Saransh
you
can
move
it
to
the
next
slide.
And
obviously ,
you
know ,
as
they
become
employees
of
IKS,
they'll
have

some
ESOP-based
incentives
over
the
years.
You
know ,
and
they,
like
I
was
saying,
they've
already
knowledge
graphed
several
features
in
the
IKS
feature
portfolio,
if
you
would,
the
autonomous
medical
coding,
the

denials
prediction
and
prevention
within
revenue
cycle,
and
several
other
clinical
decision
support
related
knowledge
graphs.

So
it
should
lead
to
a
significant
acceleration
of
what
we
were
trying
to
build
from

scratch
and
propagate
this
construct
of
this
sort
of
glass
box
AI,
if
you
would.
And
then,
oh,
by
the
way,
they
become
even
more
valuable
assuming
that
we
are
able
to
close

the
TruBridge
transaction
in
a
few
months
from
now
because
then
that
data
flywheel
that
I
was
talking
about,
that
AI
training
corpus,
that
AI
training
corpus
is
what
they'll
be
able
to

accelerate

our

journey

to

that

dramatically

and

help

us,

you

know ,

build

these

SLMs

that

I'm

talking

about

that

reduce

our

reliance.

If

you

could

move

to

the

next

slide

Saransh.

That

reduces

our
reliance
on
the
more
commercially
externally
available
foundational
models
because
over
a
period
of
time,
that
cost
of
compute
is
going
to
be
important.
And
then
the
proprietary
knowledge
that
we
will

build

in

our

SLMs

will

become

our

strategic

moat

over

a

period

of

time.

So

super

excited

to

announce

the

acquisition

and

integration

of

ARAI

as

well.

And

I'll

just

spend

a

last

couple
of
minutes
on
something
that
is
very
dear
to
our
hearts,
might
not
have
immediate
financial
consequences
for
you
all
as
investors,
but
I'll
take
a
minute,
is
the
IKS
Cares
Foundation,

which
was
a
construct
launched
a
few
years
ago
because
as
you
know ,
we
employ
about
1,900
plus
clinically
trained
staff,
most
of
which
are
doctors.
And
we
said,
what
is
a
way

for
us
to
do
CSR
in
a
way
where
these
doctors,
in
addition
to
enabling
US-based
physicians,
can
also
get
connected
back
to
the
practice
of
medicine
and
in
the
process
actually
earn

more
and
we
can
do
something
good
for
society .

And
so
we've
actually
launched
the
IKS
Cares
Foundation,
which
leverages
our
own
internal
clinical
talent
on
weekends
and
holidays
to
run
medical

camps
in
financially
underprivileged
sections
of
society
predominantly
in
Maharashtra
and
Telangana.

And
we've
taken
three
major
causes
that
we
are
solving
for
through
these
camps.
One
is
anemia
in
women
and
children,
which

we've
been
shocked
to
learn
about
the
incidence
of,
high
incidence
of
anemia
in
women
and
children.
Second
is
of
course
diabetes,
which
is
a
national
pandemic.
And
the
third
is
just
oral

hygiene
for
underprivileged
sections
of
society .

And
some
of
the
numbers

that
we've
been
able

to
achieve

are
really
stunning
with

15,000
plus
screenings

that
we've
done

in
our
first

year
and
a

half
of
existence,
which
is
a
massive,
massive
unmet
need.

We've
done
this
over,
Saransh
if
you
move
to
slide
31.

194
medical
camps
where
overall
45,000
plus
people
from
underprivileged
sections

of
society
have
been
screened.
15,000
plus
of
them
were
actually
diagnosed
as
anemic
and
42%
of
those
were
transformed
from
anemic
to
non-anemic
levels.
And
then
there's
110
odd
follow-up
camps
where

medications

have

been

provided.

So

in

addition

to

that,

recently

we

started

these

dental

camps

where

we've

done

15

odd

camps

and

nearly

1,800

dental

screenings

and

900

plus

dental

treatments.

So

a

very
important
cause
that
is
very
near
and
dear
to
us
that
allows
us
to
do
good
in
India,
Indian
healthcare,
even
as
we
are
enabling
a
lot
of
efficiencies
in
US
healthcare.

So
with
that,
thank
you
for
your
time.
Again,
I'll
round
up
by
saying
a
strong
quarter
and
a
year
of
performance
and
a
lot
of
exciting
things
to
look
forward
to.

Moderator:

Sir,
shall
we
open
the
floor
for
question
and
answer?

Sachin

Gupta:

Yes,
please.

Moderator:

Thank
you
very
much.

We
will
now
begin
the
question-and-answer
session.

Ladies
and
gentlemen,
we
will
wait
for

a
moment
while
the
question
queue
assembles.

We
will
take
the
first
question

from
the
line
of

Anil
Nahata
from

PARAMI
Financial.

Please
go
ahead.

Anil
Nahata:
Good
morning,
Sachin
and
team.
First

of
all,
congratulations
for
a
great
acquisition
of
TruBridge.
The
way
the
deal
is,
I
mean,
equal
size
deal
completely
financed
by
debt,
I
guess
which
will
take
around
three,
four
years
to

pare
down
the
debt
while
we
can
do
anything
else.
I
guess
we
are
all
into
that
and
as
they
say,
as
when
you
are
all
in,
you
better
get
it
right.
So

all
the
best
for
that.

My
first
question
is
on
the
opportunity
which

I
believe
may
be
coming

in
the
US
market,
which

is
CommonSpirit
Health.

I
mean,
they
have
terminated
their

deal
with
Tenet
and
they
are
the
one
of
the
largest
healthcare
providers
there
and
have
multiple
Epic
instances.

At
the
same
time,
I
see
that
we
have
been
very
active
on
the

Epic
marketplace
where
we
have
now
sort
of
seven
listings
against
a
couple
which
was
a
couple
of
quarters
back.
So
how
are
we
gearing
up
for
these
kinds
of
opportunities
and
is

there

a

huge

play

for

us?

Sachin

Gupta:

Anil,

thank

you

for

your

kind

words

first

of

all.

Appreciate

it.

Lots

of

good

work

ahead

of

us.

You

know ,

it's

hard

to

comment

on

individual

opportunities

as

you

can

imagine

based

on

the

confidential

nature

of

some

of

these

pursuits.

But

happy

to

note

that

CommonSpirit

is

very

much

on

our

radar

and

they

are

in

the
process
of
figuring
out
the
specifics
of
their
own
strategy
as
they
have
bolted
out
of
the
Tenet
arrangement.

And
you
know ,
in
fact,
we
have
some
past
history
of
working
with

several,
call
them
ministries
within
CommonSpirit.

And
so
as
they,
as
their
own
strategy
becomes
clearer ,
it'll
be
interesting
to
see
how
that
plays
out
for
us.
But
I
think
your
observation
is

very

appropriate

and

timely

that

it

could

present

a

pretty

large

opportunity .

Anil

Nahata:

Thank

you

for

that,

Sachin.

My

second

question

is,

if

I

go

back

to

your

last

quarter's

con-call,

you

did
mention
that
you
are
creating
a
parallel
system
just
like
we
have
the
platform
in
the
ambulatory
space
for
the
acute
space.
And
then
came
the
announcement
for
TruBridge,
which
is
basically

in
the
acute
space.
This
sort
of
development
you
were
mentioning
will
take
us
two
years
to
complete
to
where
we
are
in
the
ambulatory
space.
So
how
will
the
product
development,
the

tech

stack

now

happen

once

TruBridge

and

IKS

are

integrated?

Would

IKS

stack

be

taken

there

or

how

is

it

going

to

happen?

Just

if

you

can

do

a

slight

deep

dive

into

that?

Thank

you

so

much.

Sachin

Gupta:

Sure,

Anil.

Happy

to

do

that

obviously .

So

the

way

to

think

about

it

is,

remember

I

was

saying

that

60%

to

70%

of

the

care

delivered

in

rural

hospitals

is

outpatient

care.

And

what

IKS

has

done

in

its

traditional

large

physician

group

market

is

predominantly

outpatient

care.

So

a

lot

of

the

technology

that

we

have

built

in
our
platform
system
of
action
for
all
of
these
tasks
actually
becomes
naturally
applicable
to
that
rural
healthcare
market
because
of
the
service
mix
that
they
have
because
they
don't
have
secondary ,

tertiary ,
quaternary
capabilities
like
sophisticated
hospitals.
So
yes,
I
think
our
platform
system
of
action
from
our
physician
group
markets
will
be
very
relevant
in
the
rural
healthcare
market
even
though
there
is

zero

actual

customer

overlap.

In

the

world

that

we

are

going

to,

if

we

are

able

to

close

the

TruBridge

transaction

successfully ,

and

as

we

modernize

their

EHR

from

a

legacy

COBOL

to

a
PostgreS,
what
we'll
be
able
to
do
is
now
integrate
our
system
of
action
deeply
into
the
electronic
health
record
with
the
access
to
data,
convert
that
data
into
the
AI
training

corpus
and
using
that
AI
training
corpus,
which
is
necessary
for
this
agentic
orchestration
to
be
able
to
do
that
agentic
orchestration
synchronously
in
the
native
workflows
of
the
EHR.
So
think
of

it
as
a
good,
still
a
good
two-year
type
product
journey .
Now ,
some
of
the
features
of
our
platform
system
of
action
will
be
able
to
integrate
faster
into
the
TruBridge
EHR.
Others

might

take

longer

depending

on

the

specifics

of

that

feature,

but

I

think

you're

exactly

right

that

the

TruBridge

acquisition

puts

us

in

a

position

where

we

are

able

to

orchestrate

several

features

of
our
traditional
platform
system
of
action
in
the
rural
healthcare
market,
such
that
we
are
the
only
integrated
platform
system
of
action,
system
of
records.
Moderator:
Thank
you.
We
will
take
the

next

question

from

the

line

of

Nitin

Jain

from

FairV alue

Equity

Advisory .

Please

go

ahead.

Nitin

Jain:

Yes,

thank

you

for

the

opportunity .

And

what

I

would

like

to

know

from

the

management

is
what
type
of
risks
do
you
envisage
you
might

face
two,
three
years
down
the
line
when
AI
becomes
more
prevalent
and
coding
becomes
even
more
cheaper?
And
how
are
we
preparing

for

that?

Sachin

Gupta:

Nitin,

obviously

the

better

part

of

the

presentation

and

the

dialogues

were

actually

geared

around

that,

obviously .

Thank

you

for

your

question.

But

sufficed

to

say

that

obviously

we

are
building
our
own
AI.
So
the
fact
that
AI
makes
coding
easier
or
cheaper
is
awesome
for
us
because
we
don't
get
paid
to
write
code
for
others.
We
write
our
own
code

and
our
own
technology
orchestrates
these
actions.
So
as
AI
makes
code
generation
cheaper
and
more
reliable,
that's
a
huge
tailwind
for
us.
That's
not
a
headwind.
And
then
as
it
relates
to

how

are

we

leveraging

the

AI

that

we're

building,

that

is

what

I

was

talking

about

where

because

now

with

the

TruBridge

acquisition

we

will

have

access

to

this

data

mode

and

the

AI
training
corpus
that
will
be
built
on
it,
all
the
AI
that
we
are
building
will
actually
become
a
lot
more
relevant
and
usable
much
faster
in
the
native
workflows
of
the

electronic

output.

Nitin

Jain:

Hello.

Hello.

Moderator:

Yes,

Nitin,

you're

audible.

Please

proceed.

Nitin

Jain:

Yes,

thank

you.

Thank

you

for

the

clarification.

My

next

question

is,

the

management

mentioned

that

we

have
been
growing
very
well
for
the
last
almost
five
to
six
quarters
now.
And
given
that
we
have
a
high
base
going
into
the
next
year,
at
what
point
in
time
do

we
think
this
base
will
catch
up
and
our
growth
might
taper
down
a
bit?
Sachin
Gupta:
Nitin,
as
you
know
that
I've
said
this
all
through
the
time
we
went
public,
we

are
not
giving
guidance
and
we
won't
give
guidance.

You
know ,
this
is
not
a
linear
journey .

Not
only
do
we
have
a
track
record
of
six
quarters,
but
if
Saransh
you
can

put
that
slide
up
there,
we
have
a
track
record
of
10
years
that
we
can
demonstrate
in
our
financial
performance.
But
that
is
nothing
to
say
that
history
is
not
always
a

predictor

of

the

future.

I

can,

all

I

can

tell

you

is

there's

a

very

large

stamp,

a

very

small

outsourced

stamp

relative

to

that

large

stamp

that

is

growing

at

12%.

And

I
think
we've
built
a
business
that
has
the
right
to
win
in
this
market.
And
yet
I'm
not
in
a
position,
Nitin,
to
predict
exactly
what
our
growth
rate
is
going
to

be
quarter -on-quarter .
Having
said
that,
we've
given
a
true
north
target
to
ourselves
of
tripling
our
EBITDA
from
trading
12
months
calendar
2025
or
about
INR1,000
crores
to
about
INR3,000
crores
in
FY30.

I

wish

I

had

a

better

ability

to

predict

exactly

what's

going

to

happen

every

quarter ,

quarter

after

quarter

and

when

the

growth

will

accelerate

or

slow

down.

I

don't,

unfortunately ,

but

I

think
at
the
confluence
of
our
past
growth
track
record
and
the
defensibility
of
the
model
that
we've
built,
hopefully ,
you
know ,
we're
putting
ourselves
in
a
position
that
over
a
medium
to

long-term

time

frame,

we

can

emer ge

as

a

leader

in

this

space.

Moderator:

Thank

you.

We

will

take

the

next

question

from

the

line

of

Rohit

Thorat

from

Axis

Capital.

Please

go

ahead.

Rohit

Thorat:

Yes,

thank

you

for

the

opportunity .

So

in

your

opening

comments,

you

highlighted

that

your

revenue

growth

is

growing

faster

than

your

headcount

growth.

So

aren't

you

seeing

any

rising

pressures

from
clients
to
actually
pass
some
of
the
productivity
back
to
them?
Because
a
lot
of
IT
services
companies
and
BPO
people
have
also
highlighted
the
fact
that
they
are
seeing
AI-led
productivity

deflation

of

around

2%

to

3%

in

FY27.

So

are

you

witnessing

any

similar

pressures

or

you

haven't

encountered

any

such

pressures

yet?

Sachin

Gupta:

So

thank

you

for

the

question.

And,

you

know ,
as
I've
been
always
saying,
our
model
is
slightly
different
from
or
maybe
fundamentally
different
from
traditional
IT
services
and
BPO
services
where
they
are
getting
paid
based
on
the
number
of

people
they
deploy
or
the
number
of
transactions
they
process.
Our
pricing
model
has
always
been
outcome-based
and
a
percentage
of
the
customer's
revenue
is
how
we
get
paid.
And
so
obviously
in

that
model,
sir,
you'll
appreciate
that
it
gives
us
a
very
differentiated
ability
to
be
able
to
capture
some
of
the
efficiencies
in
our
margins.
Having
said
that,
over
a
period
of
time,

naturally ,
even
on
a
percentage
of
revenue
basis,
can
we
expect
a
minor
deflation
in
the
percentage
of
revenue
pricing
that
we
get?
Yes.
Have
we
seen
that
deflation
already
in
some
of

our

features

that

large

health

system-owned

groups

buy

individually?

Absolutely ,

yes.

For

example,

like

I

was

mentioning

our

Ambient

AI

scribing

product,

we

have

seen

deflation

in

pricing.

And

I

would

say

that

we

will

see

more

deflation

in

pricing

of

features

that

are

more

easily

autonomizable

and

less

deflation

of

pricing

in

features

that

are

not

easily

autonomizable,

right?.

And

at

the

confluence

of

that,

that
is
also
one
of
the
reasons
that
the
winner
over
a
period
of
time
is
going
to
be
a
platform
system
of
action
versus
what
we
call
the
point
solution
system
of

action.

Because

when

you

have

a

point

solution

system

of

action,

the

proclivity

of

that

pricing

getting

commoditized

is

much

higher

than

when

you

have

a

consolidated

platform

pricing

for

the

customer ,

which

is
why
that
platform
approach
is
very,
very
important
to
our
future.
Because
one,
the
platform
makes
life
a
lot
easier
for
the
customer .
And
second,
it
has
enhanced
benefits
for
the
entity

that's
providing
the
platform
as
well.
So
I
think
fundamentally
different
paradigms.
But
of
course,
I
mean,
the
history
of
any
product,
sir,
as
you
know ,
is
that
as
products
keep
getting
better ,

costs

come

down,

or

prices

come

down.

And

the

winners

in

the

market

are

able

to

mitigate

that

pricing

through

both

intelligent

pricing

mechanisms,

and

managing

their

own

costs

more

effectively .

And

so

far,
we've
been
able
to
and
we
feel
like
we're
continuing
to
build
a
model
where
we
will
have
a
significant
advantage
over
traditional
IT
services
and
BPO
models.
Rohit
Thorat:
Thank
you.

Thank
you
for
answering
that.
Just
a
couple
of
more
bookkeeping
questions
from
my
side.
First
is
on
the
ESOP
cost,
which
has
seen
a
sharp
increase
in
the
last
couple
of
quarters.

So

what

should

be

the

steady

quarterly

run

rate,

which

we

should

see

going

forward?

And

the

second

up

question

is

regarding

ETR.

So,

Nitya

mentioned

that

ETR

sh

ould

be

22%

in

FY27.

Is
that
just
for
the
core
IKS
business?

Or
would
it
be
on
a
consolidated
basis
for
IKS
and
TruBridge
businesses
combined
whenever
the
transaction
completes?

So
Yes.

Sachin

Gupta:

So
I'll

let

Nithya

respond

to

the

second

one.

Maybe

Nithya,

you

can

answer

both?

Nithya

Balasubramanian:

Yes,

sure.

So

on

ETR,

the

guidance

is

only

for

IKS.

It's

not

for

the

Profoma,

IKS,

and

TruBridge

entity .

We'll

talk

to

you

about

it

when

we

do

the

consolidation.

And

on

ESOP

cost,

our

ESOP

cost

will

trend

in

line

with

our

aspiration

to

continue

to

expand

our

technology

team

as
well
as
our
leadership
team.
And
ESOP
is
a
tool
that
we
continue
to
use
to
be
able
to
align
our
employees'
incentives
with
shareholder
value
creation.
Sachin,
if
you
want
to

add

on

the

ESOP

question?

Sachin

Gupta:

Yes,

just

our

only

thing

I'll

add

is

that

just

like

any

other

financial

parameters,

we

don't

give

guidance

on

any

one

parameter .

But

like

Nithya

said,

it

is

a

very

important

tool.

It

is

a

competitive

market.

As

a

leader

in

the

market,

we

need

to

make

sure

that

the

most

important

contributors

to

our

business,

even

in

an
AI
world,
which
is
our
people,
are
tucked
in
and
incentivized
to
continue
to
drive
our
leadership
position.
And
so
ESOP
will
continue
to
be
a
reality .
As
we
close
and
integrate

TruBridge,
we
will
obviously
be
incentivizing
a
lot
of
the
key
leadership
that
is
coming
from
TruBridge
as
well.
But
we
try
not
to
give
any
guidance
around
that
specifically .
Rohit
Thorat:
Thank

you.

Thank

you

for

answering

my

questions.

Moderator:

Thank

you.

We

will

take

the

next

question

from

the

line

of

Dev

Thacker

from

ithought

PMS.

Please

go

ahead.

Dev

Thacker:

Hello,

thank

you

for
the
opportunity
and
congratulations

for
the
great
set
of
numbers.

So
my
question
was
regarding
the
TruBridge
acquisition,
where
COBOL

to
SQL
is
listed
as
the
most
important
item.

So
could
you

please

expand

on

that?

Like

what

challenges

are

we

seeing?

And

of

the

700

clients,

any

rough

number ,

how

many

of

them

are

on

the

COBOL

and

how

important

it

is

for

the

RCM

expansion?

Sachin

Gupta:

Great,

thank

you

for

the

question.

So

first

of

all,

obviously ,

you

know ,

the

transaction

isn't

closed

now.

So

until

the

transaction

closes,

we

can't

really

affect

anything.

But

the
strategy ,
which
by
the
way,
they
have
already
started,
is
the
migration
from
COBOL
to
PostgreS.
And
that
is
critical
to
move
them
from
the
legacy
system
to
eventually
a
cloud-native
AI-first

system.

And

so

in

a

traditional

world,

the

COBOL

to

PostgreS

migration

would

have

been

a

real

challenge,

because

as

you

know ,

in

the

COBOL

world,

a

lot

of

the

business

logic

is

embedded

in

the

database

layer .

A

lot

of

it

is

embedded

in

the

UI

layer

as

well.

And

reverse

engineering

that

business

logic

out

of

those

layers

becomes

a

huge

challenge.

But

in

this
case,
as
AI
has
become
much
more
prevalent,
AI
does
a
lot
of
that,
the
Neuro
AI
does
a
lot
of
that
reverse
engineering
of
the
business
logic,
which
makes
the
migration

remarkably

easier

than

it

used

to

be

in

a

traditional

world.

Having

said

that,

it

is

still

a

significant

migration.

And

it

will

take

us

a

while

to

get

our

arms

around

it

and
do
that
completely .

The
good
news
is,
we're
doing
it
in
a
modular
way.

So
there's
several
modules
of
the
EHR
that
might
be
in
a
PostgreS
world
before
the
others
are.

And

that

will

accelerate

our

progress.

Having

said

all

that,

to

your

question

of

the

RCM

opportunity ,

the

RCM

opportunity

is

kind

of

independent

of

that

migration.

Now ,

once

that

migration

happens,

and
we
start
converting
the
data
to
an
AI
training
corpus,
obviously ,
that
will
have
impacts
on
our
ability
to
make
the
RCM
more
efficient
through
agentic
orchestration
of
several
tasks.
But
today ,
the

world
that
we're
living
in,
the
reality
of
that
is
that
there
is
a
lot
of
RCM
that
is
still
happening
manually
and
hospitals
are
struggling
with
it,
which
is
why
prior
to

IKS,

their

RCM

business

across

their

700

plus

hospitals

on

a

gross

level

is

growing

at

15%,

16%.

At

a

net

level,

it's

only

growing

at

10%

the

achievable

business

because

they're

not

able

to

execute

efficiently ,

leveraging

technology ,

leveraging

the

India

model.

And

that's

where

IKSs

core

expertise

will

come

in,

transforming

that

RCM

piece.

In

parallel,

the

COBOL

to

PostgreS

migration

will

happen

and

the
creation
of
the
data
filing
will
start
to
happen,
which
will
then
allow
us
to
get
agentic
in
several
of
the
tasks
within
the
RCM
system
of
action.
So,
this
is
a

good
18
to
24
month
process
before
we
can
be
in
a
fully
integrated
system
of
action
and
system
of
record
that
allows
for
agentic
orchestration
in
the
workforce.
But
that
doesn't
impede

us
from
continuing
to
do
what
needs
to
be
done
to
improve
performance
in
RCM,
as
well
as
proliferating
that
RCM
into
beyond
the
250,
300
of
their
customers
that
are
already
on

RCM,
the
EHR
customers,
proliferating
the
RCM
model
to
the
other
customers.

Dev

Thacker:

Got

it.

Thank

you

so

much

for

the

detailed

clarification.

Next

question

could

be

a

little

bit

elementary .

But

when
we
look
at
the
contribution
of
revenues
from
top
10
clients
on
a
quarterly
basis,
the
percentage
contribution
is
much
higher
as
opposed
to
seen
on
an
annual
basis.
So,
like
how

should

we

see

this?

Because

what

I

wanted

to

understand

was

the

overall

growth

trajectory .

Like

you

mentioned,

equity

cross-selling

strategy

has

been

revamped.

So,

going

forward,

should

we

see

the

overall

growth

among

all

set

of

clients?

Sachin

Gupta:

Nithya,

you

want

to

take

that?

Nithya

Balasubramanian:

Yes,

I

think

it's

best

if

you

look

at

it

more

on

a

year-on-year

basis

that

gives

a

more
truer
reflection
of
the
underlying
business
growth.
There
will
always
be
quarterly
variances
and
there
is
some
seasonality
to
some
parts
of
the
business
as
well.
So,
please
look
at
it
on

a

year-on-year

basis.

Moderator:

Thank

you.

We

will

take

the

next

question

from

the

line

of

Madhuchanda

Dey

from

MC

Pro.

Please

go

ahead.

Madhuchanda

Dey:

Hi,

I

have

three

questions.

The

first

question

is,

you

have

grown

at

a

rate

of

15%

in

your

three

terms.

So,

is

it

possible

to

break

up

that

growth

between

volume

and

value?

Hello?

Sachin

Gupta:

Yes,

we

can

hear

you,

ma'am.

Your

question

is

to

break

up

growth

between

volume

and

value.

We

tend

not

to

do

that.

Obviously ,

that

changes

on

a

very,

very

frequent

basis

and

so

it's

very

hard

to

do

that.

Ma'am.

Madhuchanda

Dey:

Okay .

I

mean,

we

just

wanted

to

understand

what

kind

of

value

growth

happens

annually ,

I

mean,

in

an

existing

contract

of

yours?

Sachin

Gupta:

Like

I
said,
that
's
not
a
natural
linear
number ,
ma'am,
because
volume
growth
is
a
function
of
many
factors.

Like
let's
say
you
take
a
large
physician
group
that
you're
working
with,
there

will
be
years
in
which
they
will
have
the
same
number
of
physicians
year-on-year
and
the
volume
growth
is
just
driven
by
the
patient
footfall.

There'll
be
other
years
where
the
number
of

physicians

itself

has

grown

and

hence

the

volume

growth

is

driven

by

that.

There

are

customers

in

which

there

might

be

a

thousand

physicians,

but

only

500

of

their

physicians

are

using

our

product.

And

so

now

in

a

year,

the

500

could

grow

to

750

and

that

drives

a

different

type

of

volume

growth.

There

could

also

be

an

increase

in

the

number

of

features
that
the
physicians
are
using
of
our
platform,
the
same
number
of
physicians.

And
that
could
lead
to
a
different
type
of
volume
growth.

So
that's
why,
ma'am,
it's
very
hard
to

isolate

the

specifics

of

perhaps

what

you're

asking,

if

I

understand

it

correctly .

Madhuchanda

Dey:

Okay ,

got

it.

I

have

another

housekeeping

question

before

I

move

to

my

third

question,

which

is,

you

know ,
in
your
latest
presentation,
the
contribution
from
top
10
customers
is
37%
for
the
fiscal
'26,
whereas
for
the,
if
I
look
at
the
quarterly
numbers,
they
are
43%
in
Q1,
45%

in
Q2,
and
48%
in
Q3.
If
you
could
help
us
understand
this
number
a
little
properly .
Nithya
Balasubramanian:
The
gentleman
before
this
had
the
same
question
and
I
would
recommend
investors
to

look

at

these

numbers

on

an

annual

basis.

They

always

tend

to

be

quarterly

variances

in

terms

of,

in

terms

of

our

customers.

As

I

mentioned

before,

there

also

tends

to

be

seasonality

in
particularly
the
revenue
optimization
or
the
RCM
business.

So
that
tends
to
contribute
to
the
quarterly
variances.

So
I
think
the
best
right
number
to
look
at
would
be
on
an
annual

basis,

where

as

you

can

appreciate,

both

our

top

10

and

top

5

have

grown,

have

grown

strongly .

Moderator:

Thank

you.

We

will

take

the

next

question

from

the

line

of

Krish

Jain

from

NAF A

Asset

Managers.

Please

go

ahead.

Krish

Jain:

Hi,

I

hope

I'm

audible.

So

first

of

all,

congratulations

on

a

great

year

and

I

hope

this

growth

continues.

So

my

question

is

more
about
the
industry
as
a
whole.
So
just
like
a
few
days
back,
Carlyle
Group
announced
they've
bought
two
companies
which
are
in
the
RCM
space.
And
this
is
not
the
first

time

I've

heard

a

PE

deal

happening.

Two

or

three

other

big

PE

firms

have

bought

Indian

RCM

players,

you

know ,

one,

one

and

a

half

years

back.

So

I

wanted

to

understand

if

my

understanding

of,

you

know ,

consolidation

happening

in

the

industry

and

more

and

more

PE

firms

getting

interested

in

this

space

is

correct?

And

what

is

your

comment

about,

you

know ,

the
competitive
landscape
in
the
medium
and
future,
medium
and
long
term
because
of
these
acquisitions
happening?
Sachin
Gupta:
Hi,
Krish.
Thank
you
for
the
question.
And
you
and
I
both
hope
that

the
growth
continues
even
as
we
work
on
it.
But
real
quick
on
competition,
like
I
was
saying,
you
know ,
Krish,
when
you
have
such
a
large
TAM
and
the
outsourced
TAM
is

so
small
relative
to
the
TAM
and
growing
so
significantly ,
obviously
you
have
to
expect
the
competitive
intens
ity
to
increase.

And
you
have
to
expect
that
there
will
be
more
and
more

capital
that
will
enter
this
space
chasing
the
opportunity .
So
none
of
that
is
a
surprise.
There
is
a
lot
of
private
equity
capital.
In
fact,
the
statistics
are
order
of
magnitude.
There

has
been
about
US\$50
billion
invested
in
healthcare
IT
alone,
which
is
all
of
this
space,
over
the
last
four
years.
So
there
is
a
lot
of
capital
across
the
globe
chasing
this
opportunity .

And
this
US\$50
billion
that
I'm
talking
about
is
predominantly
just
for
the
US
healthcare
market.

Having
said
that,
the
way
to
think
about
competition,
like
I
had
said
at
the
beginning

of
the
conversation,
is
there's
three
genres
of
competitors.

There's
a
finite
set
of
systems
of
record
companies,
the
EHR
vendors.

There
is
a
massively
growing
set
of
point
solution
systems
of
action,

which
is
the
easiest
way
to
enter
the
market
is
to
build
one
feature
or
one
action.
And
then
there
is
this,
call
them
rare
beachfront
properties,
perhaps,
although
that
sounds
a
bit

self-serving
like
ours,
where,
you
know ,
we
are
building
a
platform,
comprehensive
system
of
action
that
takes
all
the
features
that
the
provider
should
not
be
straddled
with,
right?
And
so
I
think

the
fastest
growing
category
of
competitors
is
in
that
point
solution
systems
of
action.
Because
again,
the
entry
barriers
in
that
genre
of
competition
are
the
smallest,
the
entry
barriers.
And
that's
where

you
see
that,
you
know ,
the
deal
you
referenced
with
Carlyle,
it's
a
point
solution
company
with
pure
focus
on
RCM.
That
doesn't
make
them
a
good
or
a
bad
company .
I'm
not

commenting
on
the
quality
of
the
competition.

I'm
just
saying
that
is
the
genre
that
will
see
the
maximum
competitive
intensity .

Now ,
we
could
be
right
or
we
could
be
wrong,
but
our

contention

is,

I

call

that

world

point

solution

hell

because

imagine

if

a

large

health

system

has

to

hire

15,

20

different

vendors

or

even

10

different

vendors

across

these

15,

16

tasks

to
do
them
well.
In
a
scenario
like
that,
how
can
they
possibly
hold
any
one
vendor
accountable?

And
they
are
left
with
the
baggage
of
integrating
everything,
right?
And
so
for

me,
very
early
in
our
journey ,
we
had
determined
that
we
don't
want
to
be
a
point
solution
vendor .
We
want
to
be
a
full
platform
system
of
action.
And
that's
what
we're

betting

on.

But

that

is

not

to

take

away

from

the

fact

that

there

will

be

a

constant

increase

in

competitive

intensity

in

that

point

solution

system

of

action

space.

And

yes,

there

will
also
be
continued
consolidation
in
that
space
because
as
that
space
gets
more
and
more
commoditized,
which
is
inevitable,
then
your
only
way
to
fight
that
commoditization
is
to
scale
and
consolidate.

And
that's
why
one
is
continuing
to
see
that.
Hope
this
helps.
Krish
Jain:
Yes,
sir.
Thank
you
for
the
detailed
answer .
That
really
helps
a
lot.
And
forgive
me,
but
as
I

was

late

to

the

call,

so

maybe

this

question

has

been

answered.

But

what

is

going

to

be

our

strategy

in

the

future

for

acquisitions?

Are

we

going

to

see

more

TruBridge

as

acquisitions,

which

is

basically

doubling

our

revenue,

or

are

you

looking

at,

you

know ,

very

focused

AI

plays

that

you

want

to

buy,

small

focused

players

like

the

recent

acquisition?

Sachin

Gupta:

So

good

question.

Thank

you
again.

No,

I
mean,

I
don't

think

we

are

looking

for

additional

TruBridges.

As

you

know ,

if

we

are

able

to

consummate

the

TruBridge

transaction,

we

will

end

up

with

about

3x

leverage

of

EBITDA

at

the

time

of

close.

And

so

obviously

our

focus

will

be

to

deleverage

the

balance

sheet

through

the

internal

accruals

and

the

strong

cash

flows

that
we
anticipate
generating
through
the
combination.

Having

said

that,

I

think

there

will

be

some

continued

opportunities

for

tuck-in

type

deals,

some

much

smaller

in

nature

like

the

deal

we

announced

today

with

ARAI
that
fundamentally
accelerate
our
capability
from
an
AI
perspective.
Or
there
might
also
be
some
tuck-in
type
opportunities
as
it
relates
to
certain
point
solution
vendors
like
I
was
discussing
earlier
that

will

end

up

having

found

sort

of

a

space

where

they

have

hit

their

ceiling

because

of

the

commoditization

of

the

point

solutions.

And

those

might

create

some

of

those

opportunities.

But

a

transformative

acquisition

like

TruBridge,

first

of

all,

comes

very

rarely .

And

second,

obviously

we

want

to

continue

to

stay

disciplined

about

not

over-leveraging

the

balance

sheet

and

or

not

unnecessarily

diluting

shareholder

equity

through

raising

equity .

So

generally ,

I

don't

anticipate

more

transformative

acquisitions

anytime

soon.

Small

tuck-ins

that

are

highly

accretive,

certainly

we'd

be

open

to

those.

Moderator:

Thank

you.

We

will

take

the

next

question

from

the

line

of

Dev

Thacker

from

ithought

PMS.

Please

go

ahead.

Dev

you

may

proceed

with

your

question.

Dev

Thacker:

Hello.

Thank

you

for

the

opportunity

again,

sir.

Sir,

I

had

just
one
request.
So
the
metrics
and
the
presentation
are
very
detailed,
but
if
possible,
could
you
also
provide
breakup
of
revenue
in
terms
of
single
solutions
and
multi-solutions?
Because
that
would
be

very
helpful.
Sachin
Gupta:
Yes,
we've
never
done
that.
Remember
the
basic
thesis
of
our
business
is
that
the
winners
are
the
platform
constructs,
right?
And
so
over
a
period
of
time,
we

anticipate
the
bulk
of
our
revenue
migrating
to
platform
constructs,
even
in
cases
of
large
health
systems
where
we
might
start
and
land
with
point
solutions
and
expand
to
the
platform.
So
we

ourselves

are

resisting

the

temptation

of

getting

too

caught

up

in

revenue

contributions

for

individual

solutions

because

that

is

at

odds

with

the

identity

of

the

organization

in

of

itself.

So

I

apologize,

my

friend.

Dev

Thacker:

Got

it,

sir.

Thank

you.

Moderator:

Thank

you.

We

will

take

the

next

question

from

the

line

of

Chirag

Kachhadiya

from

Motilal

Oswal

Financial

Services.

Please

go

ahead.

Chirag

Kachhadiya:

Hello.

Am

I

audible?

Moderator:

Yes,

you're

audible.

Please

proceed.

Chirag

Kachhadiya:

Sachin,

you

mentioned

about

the

electronic

health

records

that

all

these

systems

in

the

US

started

in

the

early

2000s.

And
they
remain,
you
know ,
just
a
database
collector
of
the
patient
information
and
all.

Sachin
Gupta:
Chirag

I
am
sorry
you
are
very
faint.

Can
you
speak
a
bit
louder?
Sorry
Chirag.

Chirag

Kachhadiya:

Yes.

So

my

question

is

on

the

electronic

health

records.

As

you

mentioned

most

of

the

players

in

the

US

started

in

the

early

2000

or

before

that.

And

they

remain,

you
know ,
just
a
collector
of
their
patient
data
base
and
all.
So
the
entity
which
we
are
acquiring,
they
also
have
an
EHR
practice,
right?
And
if
we
look
at
their
past

historical
performance,
that
is
largely
stagnant,
okay?
So
how
are
we
going
to,
you
know ,
increase
the
growth
trajectory
of
that
vertical?
And
in
our
existing
IKS
offering,
what
percentage
is
coming
from

EHR?

And

is

there

any

overlap

in

the

services

of

this

entity

and

TruBridge?

And

how

are

we

going

to,

you

know ,

increase

the

growth,

revenue

growth

for

this

EHR?

Sachin

Gupta:

I

apologize

Chirag,

I

really

struggled

to

catch

the

question.

Nithya

did

you

catch

it

clearly?

Nithya

Balasubramanian:

Bits

and

pieces.

I

think

his

question

is

around

the

TruBridges

EHR

we

are

acquiring,

and

the
fact
that
that
business
has
been
stagnant.

And

I
think,
what
do
we
intend
to
do
about
it
once
it's
in
our
hands.

Sachin

Gupta:

So,
thank
you
for
the
question.
Yes.

So,
you
know ,
like
we've
said,
the
EHR
market
is
a
mature
market
where
in
most
of
the
EHR
sub-segments,
like
rural
healthcare
is
a
US\$162
billion
sub-segment,
you
know ,
they've
already
reached

a
maturity
where
there
it's
a
two
or
three
vendor
market
and
there
is
not
much
fluctuation.

So
for
example,
in
the
rural
EHR
space,
sorry ,
rural
hospital
space,
there
are
2,200
hospitals

and
TruBridge's
EHR
is
already
in
700
of
those
hospitals,
right?
So
nearly
35%
market
share
already
exists.
So
what
one
should
not
anticipate
is
some
dramatic
growth
coming
from
the
EHR
segment

in
of
itself.
Having
said
that,
in
the
case
of
rural
healthcare,
the
two
vendors
that
are
most
prevalent
are
TruBridge
and
a
vendor
called
MEDITECH.

There's
also
a
third
vendor
called

MEDHOST ,

but

TruBridge

and

MEDITECH

are

the

most

prevalent.

And

so

the

EHR

that

is

able

to

modernize

itself

most

rapidly

and

is

able

to

build

an

integrated

system

of

action

eventually

might

have

some

market

share

opportunity

as

well.

That

will

not

be

our

focus.

Our

focus

will

not

be

to

capture

greater

EHR

market

share

in

the

next

two

to

three

years.

Our

focus

will
be
to
modernize
the
EHR,
integrate
our
system
of
action
in
it,
and
the
real
growth
will
be
to
cross-sell
the
system
of
action
into
the
EHR
customer
base.
Remember
I
said

only
250
odd
of
the
EHR
customers
that
TruBridge
has,
only
250
odd
have
the
RCM
offering.
So
first
you
have
a
450
plus
customer
market
just
for
the
RCM
system
of
action.

Then
there
are
other
components
of
our
system
of
action
because
we
have
16
odd
features.
RCM
is
just
six
or
seven
of
them,
right?
So
we'll
also
cross-sell
all
of
those
features

into
the
TruBridge
EHR
install
base.
So
think
of
the
growth
vectors
really
as
the
cross-sell
of
the
system
of
action
in
the
EHR
install
base.
And
then
as
we
modernize
the
EHR

and
we've
integrated
the
system
of
action,
could
we
ignite
another
vector
of
growth
where
we
actually
capture
additional
EHR
market
share?
Perhaps,
but
I
wouldn't
be
banking
on
that
as
the
thesis

here.

Chirag

Kachhadiya:

Okay ,

thank

you.

Moderator:

Thank

you.

Thank

you

very

much.

As

there

are

no

further

questions

from

the

participants,

I

now

hand

the

conference

over

to

Mr.

Saransh

Mundra

for

closing

comments.

Thank

you

and

over

to

you,

sir.

Saransh

Mundra:

Thank

you.

Thank

you,

everyone.

Please

reach

out

if

you

have

any

further

questions.

Thank

you

so

much

for

attending.

Sachin

Gupta:

Thank

you,

everyone.

Nithya

Balasubramanian:

Thank

you.

Moderator:

Thank

you

members

of

the

management.

On

behalf

of

ICICI

Securities,

that

concludes

this

conference.

Thank

you

for

joining

us

today ,

and

you

may

now

disconnect

your

lines.

Thank

you.

Please

note

that

this

transcript

has

been

edited

for

readability .